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Patient Support Services Procedures Manual



Patient Support Services Procedures Manual - combined

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GENERAL INFORMATION A - Z

Chemicals general - Cleaner

PRODUCT	APPLICATION	DIRECTIONS	SAFETY	PPE
 ASSET J-FILL	Multi-purpose cleaner - concentrate	Always use dispenser to correctly dilute from cold water tap and turn off tap once dispensed Do not mix or decant. Thumb trigger to operate Yellow lever to connect/disconnect trigger mechanism	Diluted - NOT HAZARDOUS For prolonged contact, protection of the skin is recommended.  Asset J-FILL Diluted Exp 12-2027.pdf	
 CONVACARE	Adhesive Remover Wipes	Open sachet and wipe over surfaces to remove residue from adhesive tapes, dressings, acrylic adhesives.	FLAMMABLE Avoid eye contact. Use with adequate ventilation. Do not eat or drink before washing hands. Avoid breathing vapours.  Convacare Exp 02-2027.pdf 	PPE not required under normal conditions of use
 CREAM R7	Bathroom Cream Cleanser	1. Shake bottle 2. Apply with damp cloth or non-abrasive sponge/pad and clean surface. 3. Rinse with clean water and wipe dry	SERIOUS EYE IRRITANT Wash face, hands and any exposed skin thoroughly after handling. Avoid direct contact and/or splashes where possible. For prolonged contact, protection of the skin is recommended.  Cream Cleanser R7 Exp 2-2028.pdf	

PRODUCT	APPLICATION	DIRECTIONS	SAFETY	PPE
 GO GETTER	Toilet & Urinal Cleaner	1. Toilet Bowls – Flush toilet. Direct Go Getter under the rim of the bowl. Allow solution to run down to the water. Use this solution to scrub all areas in and around the bowl using a toilet brush. Flush toilet. 2. Urinals – Flush Urinal. Ensure Go Getter covers corners and areas where there is uric acid build up. Scrub area with toilet brush. Flush urinal.	NOT HAZARDOUS Avoid contact with skin and eyes. Wear disposable gloves.  Go Getter Exp 11-2026.pdf 	
 KWIKMASTER DISHWASHING DETERGENT	Dishwashing Concentrate For Dishwashing Only	Wipe / rinse excess food from surfaces to be cleaned. Squirt a small amount of Kwikmaster dishwashing detergent into sink and add water. 3 to 5 mls of detergent per litre of water Rinse washed items thoroughly with clean water.	NOT HAZARDOUS EYE IRRITANT Avoid skin and eye contact. Individuals with sensitive skin should consider wearing rubber or PVC gloves.  Kwikmaster Manual Dishwash Detergent	PPE not required under normal conditions of use
 OXIVIR Tb WIPES	One step Cleaner and Disinfectant Wipes	Use wipe straight after dispensing while still moist. Use on small re-usable equipment and surfaces such as re-usable goggles, patient care equipment and dressing trollies *Keep lid closed when not in use	NOT HAZARDOUS Avoid contact with skin and eyes. Protect against physical damage. Make certain containers are well sealed when not in use. With prolonged use, wear: neoprene or nitrile gloves.  Oxivir Tb Wipes Exp 07-2029.pdf	PPE not required under normal conditions of use
	Promed Antibacterial Wipes	Use wipe straight after dispensing while still moist. Use on small equipment surfaces: stainless steel surfaces, telephones, computers, televisions, equipment surfaces. *Keep lid closed when not in use	NOT HAZARDOUS Avoid contact with skin and eyes. Protect against physical damage. Make certain containers are well sealed when not in use. With prolonged use, wear: neoprene or nitrile gloves.	PPE not required under normal conditions of use

PRODUCT	APPLICATION	DIRECTIONS	SAFETY	PPE
PROMED 70% ALCOHOL WIPES			 Promed antibacterial wipes !  	
 ROOM CARE R2L	Alkaline Cleaning Agent – Hard Surface Cleaner including <u>toilet bowls</u> and <u>urinals</u>	1. Squirt a small amount of Room Care R2L onto a damp cloth or paper towel to clean the surface. 2. Wipe or rinse with fresh water 3. Buff with a dry cloth or paper towel for extra shine. 4. Squirt a small amount into toilet bowl or urinal	SERIOUS EYE IRRITANT Safety glasses are recommended. Avoid contact with skin and eyes.  Room Care R2L Exp 09-2028.pdf 	 
 SODIUM HYPOCHLORITE 0.125%	General purpose surface disinfectant	Damp wipe all surfaces with undiluted product. Wipe off the excess.	NOT HAZARDOUS Safety glasses are recommended. Approved respiratory apparatus should be worn if a mist vapour or dust is generated  Sodium Hypochlorite 0.125%	 
 TUFFIE	Detergent Wipes – Alcohol Free Multi Surface General Cleaning Wipes for small surfaces	Use wipe straight after dispensing while still moist. For use on small surfaces and single pieces of non-electrical equipment. *Keep lid closed when not in use	NOT HAZARDOUS Avoid eye contact and prolonged skin contact. Wash hands thoroughly with water after use.  Tuffie Wipes Exp 10-2026.pdf	PPE not required under normal conditions of use
	<u>Glass and Mirror (spot) cleaner</u>	Spray directly from bottle to glass/mirror from approximately	NOT HAZARDOUS	PPE not required under normal

PRODUCT	APPLICATION	DIRECTIONS	SAFETY	PPE
 WINDEX		15 – 20 centimetres (cm) and wipe with clean cloth	Avoid eye contact. Avoid prolonged skin contact. Avoid breathing vapours.  Windex expires 01-2028.pdf	conditions of use

Chemicals general - PCA

PRODUCT	APPLICATION	DIRECTIONS	SAFETY	PPE
 ASSET J-FILL	Multi-purpose cleaner - concentrate	Always use dispenser to correctly dilute from cold water tap and turn off tap once dispensed Do not mix or decant. Thumb trigger to operate Yellow lever to connect/disconnect trigger mechanism	Diluted - NOT HAZARDOUS For prolonged contact, protection of the skin is recommended.  Asset J-FILL Diluted Exp 12-2027.pdf	
 CONVACARE	Adhesive Remover Wipes	Open sachet and wipe over surfaces to remove residue from adhesive tapes, dressings, acrylic adhesives.	FLAMMABLE Avoid eye contact. Use with adequate ventilation. Do not eat or drink before washing hands. Avoid breathing vapours.  Convacare Exp 02-2027.pdf 	PPE not required under normal conditions of use
 CREAM R7	Bathroom Cream Cleanser	1. Shake bottle 2. Apply with damp cloth or non-abrasive sponge/pad and clean surface. 3. Rinse with clean water and wipe dry	SERIOUS EYE IRRITANT Wash face, hands and any exposed skin thoroughly after handling. Avoid direct contact and/or splashes where possible. For prolonged contact, protection of the skin is recommended.  Cream Cleanser R7 Exp 2-2028.pdf	

PRODUCT	APPLICATION	DIRECTIONS	SAFETY	PPE
 GO GETTER	Toilet & Urinal Cleaner	1. Toilet Bowls – Flush toilet. Direct Go Getter under the rim of the bowl. Allow solution to run down to the water. Use this solution to scrub all areas in and around the bowl using a toilet brush. Flush toilet. 2. Urinals – Flush Urinal. Ensure Go Getter covers corners and areas where there is uric acid build up. Scrub area with toilet brush. Flush urinal.	NOT HAZARDOUS Avoid contact with skin and eyes. Wear disposable gloves.  Go Getter Exp 11-2026.pdf 	
 KWIKMASTER DISHWASHING DETERGENT	Dishwashing Concentrate For Dishwashing Only	Wipe / rinse excess food from surfaces to be cleaned. Squirt a small amount of Kwikmaster dishwashing detergent into sink and add water. 3 to 5 mls of detergent per litre of water Rinse washed items thoroughly with clean water.	NOT HAZARDOUS EYE IRRITANT Avoid skin and eye contact. Individuals with sensitive skin should consider wearing rubber or PVC gloves.  Kwikmaster Manual Dishwash Detergent	PPE not required under normal conditions of use
 OXIVIR Tb WIPES	One step Cleaner and Disinfectant Wipes	Use wipe straight after dispensing while still moist. Use on small re-usable equipment and surfaces such as re-usable goggles, patient care equipment and dressing trollies *Keep lid closed when not in use	NOT HAZARDOUS Avoid contact with skin and eyes. Protect against physical damage. Make certain containers are well sealed when not in use. With prolonged use, wear: neoprene or nitrile gloves.  Oxivir Tb Wipes Exp 07-2029.pdf	PPE not required under normal conditions of use
	Promed Antibacterial Wipes	Use wipe straight after dispensing while still moist. Use on small equipment surfaces: stainless steel surfaces, telephones, computers, televisions, equipment surfaces. *Keep lid closed when not in use	NOT HAZARDOUS Avoid contact with skin and eyes. Protect against physical damage. Make certain containers are well sealed when not in use. With prolonged use, wear: neoprene or nitrile gloves.	PPE not required under normal conditions of use

PRODUCT	APPLICATION	DIRECTIONS	SAFETY	PPE
PROMED 70% ALCOHOL WIPES			 Promed antibacterial wipes !  	
 ROOM CARE R2L	Alkaline Cleaning Agent – Hard Surface Cleaner including <u>toilet bowls and urinals</u>	1. Squirt a small amount of Room Care R2L onto a damp cloth or paper towel to clean the surface. 2. Wipe or rinse with fresh water 3. Buff with a dry cloth or paper towel for extra shine. 4. Squirt a small amount into toilet bowl or urinal	SERIOUS EYE IRRITANT Safety glasses are recommended. Avoid contact with skin and eyes.  Room Care R2L Exp 09-2028.pdf 	 
 SODIUM HYPOCHLORITE 0.125%	General purpose surface disinfectant	Damp wipe all surfaces with undiluted product. Wipe off the excess.	NOT HAZARDOUS Safety glasses are recommended. Approved respiratory apparatus should be worn if a mist vapour or dust is generated  Sodium Hypochlorite 0.125%	 
 STRIDE CITRUS HC J-FILL	Neutral cleaner for <u>Water Jugs</u> For use in Catering Pantry only. For use by PCAs only	Refer to PSS Patient Care Assistant Manual	SERIOUS EYE IRRITANT Wash face, hands and any exposed skin thoroughly after handling  Stride Citrus HC J-Fill Exp 3-2028.pdf  	

PRODUCT	APPLICATION	DIRECTIONS	SAFETY	PPE
 TUFFIE	Detergent Wipes – Alcohol Free Multi Surface General Cleaning Wipes for small surfaces	Use wipe straight after dispensing while still moist. For use on small surfaces and single pieces of non-electrical equipment. *Keep lid closed when not in use	NOT HAZARDOUS Avoid eye contact and prolonged skin contact. Wash hands thoroughly with water after use.  Tuffie Wipes Exp 10-2026.pdf	PPE not required under normal conditions of use
 WINDEX	<u>Glass and Mirror (spot) cleaner</u>	Spray directly from bottle to glass/mirror from approximately 15 – 20 centimetres (cm) and wipe with clean cloth	NOT HAZARDOUS Avoid eye contact. Avoid prolonged skin contact. Avoid breathing vapours.  Windex expires 01-2028.pdf	PPE not required under normal conditions of use

Chemicals specialised - Cleaner

PRODUCT	APPLICATION	DIRECTIONS	SAFETY	PPE
 <u>INOX D7.1 POLISH</u>	Stainless Steel Polish For use on stainless steel handrails in <u>lifts</u> only	1. Squirt onto a clean cloth 2. Apply on a clean and dry surface and polish	NOT HAZARDOUS Ready to use. Do not mix with water Use only on non-food contact areas  Inox D7.1 Exp 02-2029.pdf	PPE not required under normal conditions of use
 <u>NOCOLYSE</u>	Disinfectant Solution for use with the <u>Nocospray system</u>	<u>Refer to PSS Cleaners Manual</u>	MILD SKIN IRRITANT EYE IRRITANT Safety: Before use carefully read product label. Use safe work practices to avoid eye and skin contact and inhalation.  Nocolyse Exp 06-2023.pdf 	
 <u>REVEAL</u>	Heavy Duty Floor Cleaner	<u>Refer to PSS Cleaners Manual</u> Reveal, 1:100 ratio Asset JF is generally used. Only use Reveal for heavy duty soilage if directed to by a Supervisor	SKIN IRRITANT SERIOUS EYE IRRITANT Safety: Before use carefully read product label. Use of safe work practices are recommended to avoid eye or skin contact and inhalation  Reveal SDS Exp 01-2029.pdf 	

PRODUCT	APPLICATION	DIRECTIONS	SAFETY	PPE
 <u>SAFESORB</u>	<u>Absorbent Powder for General Spills or Body Fluids</u>	Pour Safe-Sorb over and around entire area of blood, body or any other fluid spill. When the spill has been absorbed, use a 'bluey' to gather material and place into waste disposal.	NOT HAZARDOUS Safety: Avoid eye contact. Avoid prolonged skin contact. Avoid breathing vapours.  Safe Sorb SDS Exp 09-2026.pdf	
 <u>SNAPBACK</u>	Floor Buffer	<u>Refer to PSS Cleaners Manual</u>	SKIN IRRITANT SERIOUS EYE IRRITANT Safety: Before use carefully read product label. Use of safe work practices are recommended to avoid eye or skin contact and inhalation.  Snapback Exp expires 02-2024.pdf 	
 <u>STRIP EZE</u>	Floor Stripper	<u>Refer to PSS Cleaners Manual</u>	SEVERE SKIN BURNS & EYE DAMAGE EYE IRRITANT Safety: Before use carefully read product label. Use of safe work practices are recommended to avoid eye or skin contact and inhalation  Strip Eze SDS Exp 11-2028.pdf 	
	Shampoo Cleaner soft carpet	<u>Refer to PSS Cleaners Manual</u>	NOT HAZARDOUS Safety: Before use carefully read product label.	

PRODUCT	APPLICATION	DIRECTIONS	SAFETY	PPE
 <u>TAPI EXTRACT</u>			Use of safe work practices are recommended to avoid eye or skin contact and inhalation.  Tapi Extract Exp 08-2029.pdf	
 <u>UNBELIEVABLE CARPET SPOT & STAIN REMOVER</u>	<u>Carpet Spot & Stain Remover</u>	1. Spray carpet spotter onto clean cloth, then apply to stain. 2. Leave for a few seconds. 3. Using a clean wet cloth, wipe gently working from edge to centre of the stain. 4. Rinse the cloth and wipe carpet to remove all stain residue.	MILD SKIN IRRITANT Safety: Before use carefully read product label. Use of safe work practices are recommended to avoid eye or skin contact and inhalation  Unbeiveable Exp 09-2026.pdf 	
 <u>VECTRA</u>	Floor Sealer	<u>Refer to PSS Cleaners Manual</u>	SKIN & EYE IRRITANT SERIOUS EYE DAMAGE Safety: Before use carefully read product label. Use of safe work practices are recommended to avoid eye or skin contact and inhalation.  Vectra Floor Sealer SDS expires 07-2025 	
	Antimicrobial Disinfectant	For use in Pharmacy Aseptic Suite ONLY	SKIN DRYNESS OR CRACKING	PPE required for entry into aseptic suite:

PRODUCT	APPLICATION	DIRECTIONS	SAFETY	PPE
 VIRACLEAN		1. Empty half a 5 litre bottle (2.5L) of Viraclean into bucket. 2. Mop floors with Super Swish flat mop and disposable mop pads. 3. Discard mop pad following each clean. 4. Clean walls weekly using a disposable wall wipe and half filled bucket of Viraclean.	Safety: Avoid contact with skin and eyes.  Viraclean _SDS expires 04-2025.pdf	  Polyester coverall and overshoes  Gloves  Mask  Hairnet to be worn at all times

Chemicals specialised - PCA

PRODUCT	APPLICATION	DIRECTIONS	SAFETY	PPE
 <u>ASEPTI-CLEAN</u>	Detergent Miele dishwasher	<u>Refer to PSS Patient Care Assistant Manual</u>	NOT HAZARDOUS Safety: Avoid eye contact. Avoid prolonged skin contact. Avoid breathing vapours.  Asepti Clean Exp 02-2026.pdf	 
 <u>ASEPTI-RINSE CLEAR</u>	Rinse Additive Miele dishwasher	<u>Refer to PSS Patient Care Assistant Manual</u>	NOT HAZARDOUS Safety: Avoid eye contact. Avoid prolonged skin contact. Avoid breathing vapours.  Asepti Rinse Clear Exp 11-2025.pdf 	 
 <u>SAFESORB</u>	<u>Absorbent Powder for General Spills or Body Fluids</u>	Pour Safe-Sorb over and around entire area of blood, body or any other fluid spill. When the spill has been absorbed, use a 'bluey' to gather material and place into waste disposal.	NOT HAZARDOUS Safety: Avoid eye contact. Avoid prolonged skin contact. Avoid breathing vapours.  Safe Sorb SDS Exp 09-2026.pdf	 

Colour coded cleaning system

The colour coded cleaning system is visually effective, and is used to reduce cross contamination.

BLUE
GENERAL AREAS

RED
BATHROOMS

YELLOW
ISOLATION ROOMS & SPILLS

GREEN
KITCHENS & TEA ROOMS

WHITE
THEATRES

Duty Statement BHS All Purpose Orderly - A block

0630-1500

*Times are indicative only

Time	Duty Description
06:30	Collect page from night orderly.
	Check reception, and escort patients and their file to the appropriate floors.
	Leave patient in TV room and advise them that a nurse will be with them shortly (PM only).
	Patient's file to be left with reception on appropriate floor. Notify nursing staff that the patient has arrived.
	Collect third floor linen trolley from linen room and change over.
	Return old trolley back to linen room.
	Collect first floor linen trolley from clean linen room and change over.
	Return old trolley back to the linen room.
	Take nursery trolley to the second floor.
	Stack linen neatly on shelves in first and second nursery.
	Take theatre trolley up to the first floor and leave outside lift.
	Collect empty trolley later and return to linen room.
	Sweep outside front entrance and clean windows– daily. Pick up leaves.
	Sweep outside back door and clean windows.
09:30	Collect coding trolley from back of reception area and take to F Block area.
	Return trolley back to A Block reception area.
	Pick up sandwiches from kitchen and delivery to Ward 3.
10:00	Escort PM patients to Ward 3 and Day Surgery
11:00	Between 11:00 – 14:00 check reception half-hourly for theatre list and deliver to Kitchen, Day Surgery, Pathology, x-ray, Physiotherapy and CSSD.
	Check for flowers at reception and deliver to wards 1, 2 & 3.
	Take food bin from washup area out and replace with an empty bin.
11:30	Lunch
12:00	Check reception for parcel to go for F Block and leave at F Block reception.
13:00	Assist the 1st and 3rd floor staff member to make bed bundles and carbolise beds.
	When the linen attendant pages to advise the CSSD linen today has arrived, the trolley is to be collected from the linen room and taken to third floor. Leave outside the CSSD door in corridor.
13:45	Go to F Block to check files.
14:15	Assist with Ward 1,2 & Ward 3 Bed Bundles
14:45	Give pagers to PM orderly.
15:00	End of shift

Additional Duties	When required, assist with bed moves (AM only when patients stay overnight). Help Cleaners with Air Mattress if requested.
Mornings	Daily - Check third floor bed list. If any beds require carbolising, this is to be done in between taking patients to the floors.
	Take patients to Radiology from W1 and W3 when required.
Monday and Friday	Clean windows in front foyer – more often if required.
	Make sure wheelchairs are always on floors.
	Clean and check tyres weekly.
	Additional duties as required. Transportation Services • Transports patients between wards and departments when requested. • Escorts patients on discharge. • Carries out urgent errands or messages of a clinical nature at the request of the ward shift coordinator. • Collects urgent pharmacy supplies as required for ward supply or patient discharge. • Delivers all urgent specimens to appropriate departments and laboratory receptions. • Transports clinical equipment between wards and departments.
	Cleaning Services • Cleans clinical, non-clinical and administration areas of Bentley Health Service in accordance with duty statements, infection prevention requirements, and hospital cleaning protocols and procedures. • Collects and removes soiled linen and rubbish from designated areas in accordance with duty statements and hospital policies and procedures. • Replenishes consumables including hand towels, hand soap, toilet rolls and other related cleaning consumables. • Maintain cleaning rooms, cleaning equipment and cleaning equipment storerooms in a clean, safe, and tidy condition. • Disposes of full, sealed sharps containers and replaces with empty sharps containers. • Manual Handling Assistance – under direct supervision of Nurses • Assists with sitting patients out of bed and returning patients to bed. • Carries out chair to chair transfer of patients under nursing supervision. • Moves patients from bed to trolley and vice versa. • Assists with lifting, turning, and re-positioning of patients using appropriate lifting aids and equipment. • Demonstrates knowledge and competency in manual handling techniques, including patient handling.

	Emergency Response Functions • Performs duties as assigned under the emergency response procedures of Bentley Health Service • Responds to all designated emergency alarms and carries out tasks (e.g. blood collection, specimen delivery) as directed.
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To be used as a guideline

Duty Statement Ward PCA - Morning

Indicative Time	Task/Sequencing
06:00	Write name on whiteboard
	Report to shift Coordinator
	Restock detergent/disinfectant wipes , hand wash/lotions, gloves and cupboards above sinks in every room/hallway
Before Breakfast	Collects, cleans and distribute patient water jugs
	Remove commode chairs from patient rooms and place in storage area. Full clean of commode chairs prior to storage.
	Clean and reprocess appropriately used bowls, pans and bottles in pan room from night duty
	Distribute patient chairs into rooms from store area as required
	Remove soiled linen from patient areas to linen skips
	Check ward fridges and remove any expired hospital food items including: milk, dietary supplements, sandwiches, desserts
	Assist patients to prepare for breakfast (position patients, ensure heart tables clear and accessible, sit patients out of bed as required)
Morning tea	Assist patients to prepare for morning tea (position patients, ensure heart tables clear and accessible, sit patients out of bed as required)
10:00	Start PCA tea breaks (15 min) – Update whiteboard - Notify coordinator
Before lunch	Assist with patient care under direction and supervision of nursing staff including: • Make beds • Assist with patient hygiene (bed wash, shower) • Assist with toileting • Assisting patients with meals (full assist, partial assist, prompt) • Patient mobility (including transfer to chair/bed, pressure area care) • Provide clinical equipment to bed areas (IV poles, bowls etc)
	Clean shower trolley and shower chairs between patient use
	Assist patients to prepare for lunch (position patients, ensure heart tables clear and accessible, sit patients out of bed as required)
	Collect any leftover meals trays (catering staff to distribute and collect meal trays)
13:00	Lunch breaks (30 min) – Update whiteboard - Notify coordinator
During shift	Cleans patient bed area including: • Clean bed • Tidy and clean locker • Tidy and clean heart tables • Clean patient chairs • Change bedside rubbish bag once a shift and as required • Clean high touch points: PCA and Cleaner work together. The below are not mutually exclusive and PSS staff communicate collaboratively to ensure all

	high touch points are wiped at least once, every 24 hours. Generally PCAs will wipe: ○ Chair ○ Call bell/button ○ Bedside locker ○ Patient tray table ○ Television remote ○ Telephone ○ Bed rail ○ Overhead examination light ○ Suction canister See PCA cleaning duties below for weekly tasks
During shift	Cleans patient bed area including: • Clean bed • Tidy and clean locker • Tidy and clean heart tables • Clean patient chairs • Change bedside rubbish bag once a shift and as required Clean high touch points, including patient call bell, TV remote and telephone See PCA cleaning duties below for weekly tasks
	Cleans pan room, preparation room, treatment room. • Includes bench tops, shelving and pan sanitisers once a shift and as required See PCA cleaning duties below for weekly tasks
	Remove rubbish and linen bags to designated collection area
	Clean and reprocess appropriately pans, bowls and bottles throughout shift (check hourly)
09:00-13:00	Restock pan room, treatment room and preparation room
	Restock clinical trolleys (IV trolley, ward specific trolleys)
As required	Assist nursing staff with patient handling including but not limited to inter and intraward transfers
	Cleans discharge beds and cleaning of discharge bed area (as per hospital infection control standards)
	Prepares bed for new admissions and for transfers between areas (including theatres and special care areas) and around ward
	Carries out urgent messages or errands of a clinical nature at the request of ward shift coordinator
	Escort patients on discharge
	Transfer patient property from the ward to Security Office/ Patient Trust upon request from Clinical staff. When taking patient property ensure you have completed paperwork from the clinical staff.
13:00 to 14:00	Patient rest period – assist nurses to settle patients as required. Turn off patient ward area lights and close ward doors at 13:00 and turn on lights and open ward doors at 14:00

14:00	Clean and restock PCA (Lesley) Trolley Hand over to PM PCAs and notify of any unfinished duties
14:30	Report off duty to shift Coordinator

Weekly Tasks

Monday	Change curtains/screens as required* and oxygen equipment bags • Ladder should be used • Refer to schedule
Tuesday	Damp dust linen cupboards
Wednesday	Damp dust lotion room cupboards • Remove stock from containers, clean container and repack stock • Remove containers/stock from shelves, clean shelves and replace stock/containers
Thursday	Scrub commodes and shower chairs • Thoroughly check underside of seats and lids
Friday	Clean linen skips and trolleys • Also check wheels etc are in good condition
Saturday	Clean store room shelves • Remove stock from containers, clean container and repack stock • Remove containers/stock from shelves, clean shelves and replace stock/containers <i>Complete approximately one quarter of the stock weekly - equates to all stock monthly – refer to schedule</i>
Sunday	Clean ward and staff fridges • Defrost freezer unit

*Curtains/screens as required:

- Change any curtain if visibly soiled or frayed as needed any day/time
- Antimicrobial curtains - Record hang date & next exchange date per Nurse Unit Manager's schedule
- Cloth curtains – On Mondays change a portion of curtains so that all are exchanged monthly

This is a guideline only:

Weekly Duties should be carried out during quieter periods of Ward activity and form part of the handover

Refer to PSS Patient Care Assistant Manual for further information and instructions

Duty Statement Ward PCA - Afternoon

Indicative Time	Task/Sequencing
13:00	Write name on whiteboard
	Report to shift coordinator
	Clean and reprocess appropriately used bowls, pans and bottles in pan room
Afternoon Tea	Assist patients to prepare for afternoon tea (position patients, ensure heart tables clear and accessible, sit patients out of bed as required)
15:00	Exchange patient water jugs
15:00-16:00	Commence PCA tea breaks (15 min). Update whiteboard - Notify Coordinator
Before evening meal	Assist with patient care under direction and supervision of nursing staff including: • Assist with patient hygiene • Assist with toileting • Assisting patients with meals (full assist, partial assist, prompt) • Patient mobility (including transfer to chair/bed, pressure area care) • Provide clinical equipment to bed areas (IV poles, bowls etc)
	Assist patients to prepare for evening meal (position patients, ensure heart tables clear and accessible, sit patients out of bed as required)
18:00	Meal breaks commence
During shift	Cleans patient bed area including: • Clean bed • Tidy and clean locker • Tidy and clean heart tables • Clean patient chairs • Change bedside rubbish bag once a shift and as required • Clean high touch points: PCA and Cleaner work together. The below are not mutually exclusive and PSS staff communicate collaboratively to ensure all high touch points are wiped at least once, every 24 hours. Generally PCAs will wipe: ○ Chair ○ Call bell/button ○ Bedside locker ○ Patient tray table ○ Television remote ○ Telephone ○ Bed rail ○ Overhead examination light ○ Suction canister See PCA cleaning duties below for weekly tasks

	Check patient bathrooms and clean/tidy as required between routine cleaning rounds
	Spot clean patient bed area as required, including: • Damp wipe back of bed • Damp wipe ledge behind bed • Damp wipe suction bottles, tubing etc • Damp wipe IV pumps and equipment (including hoists) • Damp wipe knobs and monitors
	Cleans pan room, preparation room, treatment room. • Includes bench tops, shelving and pan sanitisers once a shift and as required
	Remove rubbish and linen bags to designated collection area
	Check ward fridges and remove any expired hospital food items including: milk, dietary supplements, sandwiches, desserts
	Restock bedside dispensers (ie disposable gloves, wash lotion)
	Dispose of full, sealed sharps containers and replaces with empty container
	Transports patients between wards and departments as necessary (clinically urgent situations or where orderly not available)
As required	Assist nursing staff with patient handling including but not limited to inter and intraward transfers
	Cleans discharge beds and cleaning/preparation of discharge bed area (as per hospital infection control standards) within an hour of bed becoming vacant
	Prepares bed and assists with new admissions and for transfers between areas (including theatres and special care areas)
	Carries out urgent messages or errands of a clinical nature at the request of ward shift coordinator (routine/non-urgent messages conducted by courier service at Wellington St Campus)
	Collect supplies from pharmacy for ward supply or patient discharge
	Escort patients on discharge
	Check linen trolley and contact linen room if extra linen required
	Transfer patient property from the ward to Security Office/ Patient Trust upon request from Clinical staff. When taking patient property ensure you have completed paperwork from the clinical staff.
20:00	Collect patient chairs from rooms, full clean and place in store area
	Place patient commodes (with bowls) and urinals by beds as instructed by nursing staff

	Ward tidy and spot clean as required including: • Check sharps containers and replace as required • Clear furniture and tidy patient ward areas • Check patient rubbish bags and replace as required • Check and exchange water jugs • Tidy pan room • Clean and reprocess appropriately any bowls, pans, bottles • Tidy treatment room
	Clean and restock PCA (Lesley) Trolley
21:30	Report off duty to shift coordinator

Weekly Tasks

Monday	Change curtains/screens as required* and oxygen equipment bags • Ladder should be used • Refer to schedule
Tuesday	Damp dust linen cupboards
Wednesday	Damp dust lotion room cupboards • Remove stock from containers, clean container and repack stock • Remove containers/stock from shelves, clean shelves and replace stock/containers
Thursday	Scrub commodes and shower chairs • Thoroughly check underside of seats and lids
Friday	Clean linen skips and trolleys • Also check wheels etc are in good condition
Saturday	Clean store room shelves • Remove stock from containers, clean container and repack stock • Remove containers/stock from shelves, clean shelves and replace stock/containers <i>Complete approximately one quarter of the stock weekly - equates to all stock monthly – refer to schedule</i>
Sunday	Clean ward and staff fridges • Defrost freezer unit

*Curtains/screens as required:

- Change any curtain if visibly soiled or frayed as needed any day/time
- Antimicrobial curtains - Record hang date & next exchange date per Nurse Unit Manager's schedule
- Cloth curtains – On Mondays change a portion of curtains so that all are exchanged monthly

This is a guideline only:

Weekly Duties should be carried out during quieter periods of Ward activity and form part of the handover

Refer to PSS Patient Care Assistant Manual for further information and instructions

Duty Statement Ward Cleaner

Time	Task/Sequencing
Shift Commence	Vacuum and mop floors and corridors
Office Areas	Damp wipe high touch points Damp wipe desks, ledges, and furniture Empty rubbish bins and replace bin liners Wipe over computers with Isowipes - not keyboard and screens Run taps for one minute
Ward Areas	Damp wipe high touch points Remove rubbish and replace bin liners Spot clean and damp wipe walls and furniture Vacuum or static sweep floor Mop floor using grey charging bucket, blue mop handle and blue mop pads Dust horizontal surfaces other than patient fixtures and equipment High dust curtain rails and window ledges Damp dust television screens Clean any drink fountains Run taps for one minute
Patient Rooms	Damp wipe high touch points Clean patient room and surrounds, including basins, paper towel holders and soap dispensers Sweep and mop floor Run taps for one minute
Discharge cleans	Completed with Sodium Hypochlorite 0.125% Mop with Asset JF See PSS Cleaner Procedure Manual for details
Toilets and Showers	Damp wipe high touch points Mop floor using grey charging bucket, red mop handle and blue mop pads Clean tiles, floors, and toilet bowls, back of toilet bowls and pipes Wipe over fittings and door frames Wipe mirrors and walls Replenish consumables, including paper towels, toilet paper and hand soap Run taps for one minute

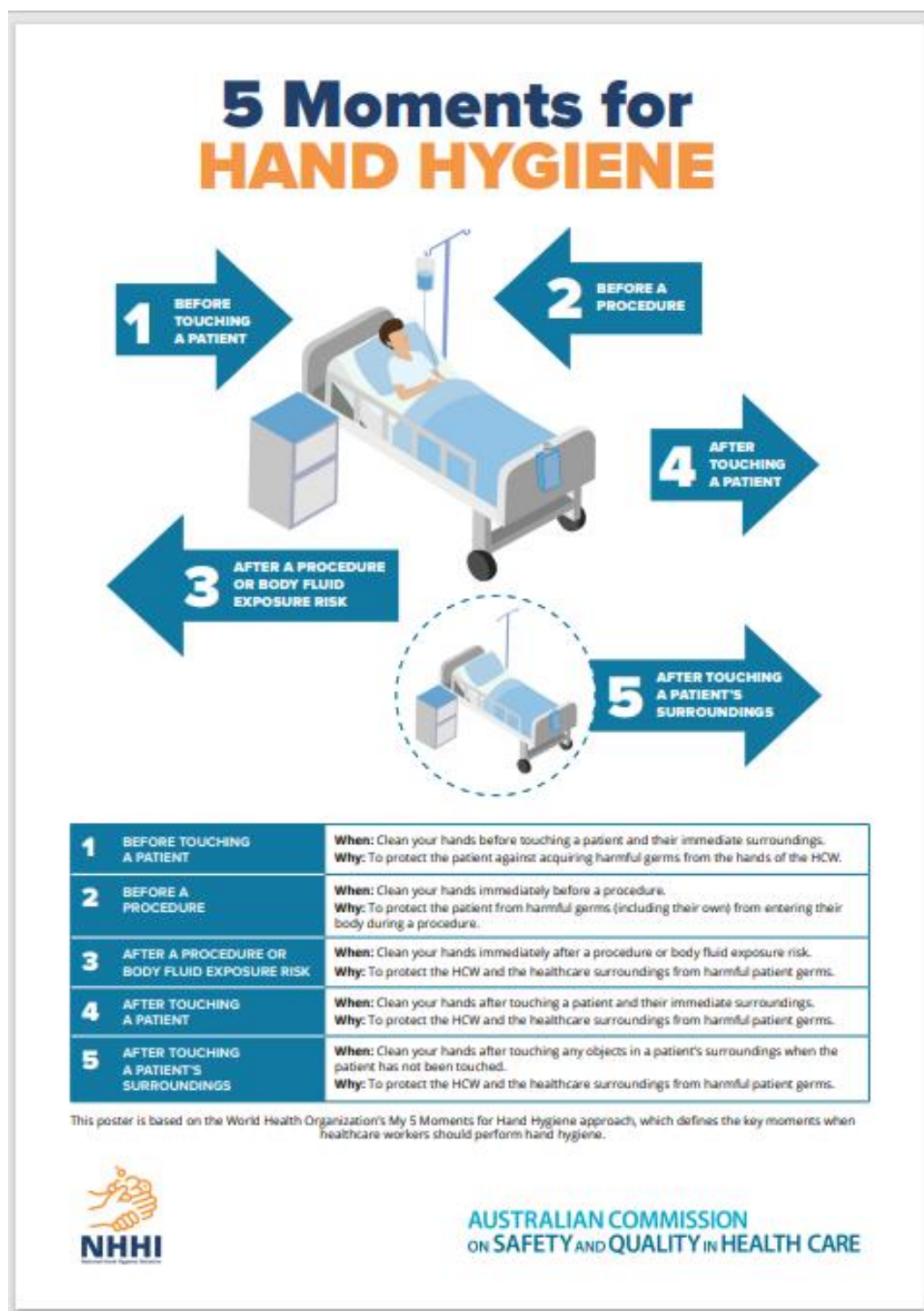
Pan Room	Damp wipe high touch points Sweep and mop vinyl floors Empty waste bins and replace bin liners Run taps for one minute
Storeroom	Damp wipe high touch points Sweep and mop floors using general purpose detergent
Break Times	Breaks are generally taken at the times below noting that operational requirements of the ward and hospital are priority Nurse Coordinator must be informed before break is taken and name written on whiteboard Cleaners working on the same ward must not take breaks at the same time Morning tea break: • First cleaner 1000 – 1015 • Second cleaner if rostered 1015 – 1030 Lunch break: • First cleaner 1200 – 1230 • Second cleaner if rostered 1230 – 1300
Tea Room	Damp wipe high touch points Clean sinks/basins and surrounds Sweep and mop floors using general purpose detergent. Run taps for one minute
Shift End	Clean and restock Cleaner's trolley
Weekly Tasks	Machine scrub floors Buff floors Dust ceiling and wall vents - outside of vent only Other duties as directed by PSS Supervisor

This is a guideline only and is not an exhaustive list of duties.

Refer to PSS Cleaners Manual for further information and instructions.

Five “5” Moments of Hand Hygiene

Always practice hand hygiene in accordance with the 5 moments.



Personal Protective Equipment (PPE) sequence for donning

As required by the Isolation Room card

The type of PPE required is determined by the type of precautions (i.e. Contact, Droplet and/or Airborne).
Follow the order specified below, applying the appropriate PPE for the level of precautions.

PPE is applied at Donning Station BEFORE entering the room

1 Perform hand hygiene



2 Gown

- › Fully cover torso from neck to knees, arms to end of wrists, and overlap around the back.
- › Fasten ties at back of neck and side of waist.
- › If two gowns are required, the first gown is to have the opening at the front and the second gown is to have the opening at the back.



3 Surgical mask (with or without visor attached) or N95/P2 respirator

- › Choose appropriate mask for type of precautions.
If N95/P2 worn ensure mask is the correct size for your face.
- › Secure ties / elastic bands at back of head and neck.
- › Fit flexible band to nose bridge.
- › Fit snug to face and below chin.
- › For N95/P2 perform a 'fit check' to ensure there is a seal between the mask and your face.



4 Protective eye wear / face shield

- › Place protective eye wear over your eyes, (and over prescription glasses if worn) and adjust to fit comfortably and securely.
- or
- › Place face shield over face and adjust to fit comfortably and securely.



5 Perform hand hygiene



6 Gloves

- › Extend to cover wrist of isolation gown.



Personal Protective Equipment (PPE) sequence for doffing

As required by the Isolation Room card

Remove INSIDE the isolation room

1 Gloves

Outside of gloves are contaminated

- Using a gloved hand, grasp the palm area of the other gloved hand and peel off first glove.
- Hold removed glove in gloved hand.
- Slide fingers of ungloved hand under remaining glove at wrist and peel off second glove over first glove.
- Discard gloves in rubbish bin.



2 Perform hand hygiene



3 Gown

Gown front and sleeves are the most contaminated

- Unfasten gown ties (top then waist).
- Peel gown away from neck and shoulders, touching inside of gown without touching clothes or skin.
- Carefully remove arms from sleeves.
- Gently roll exposed side of gown inward and away from the body, into a small bundle.
- Discard gown in rubbish bin.



4 Perform hand hygiene



Remove at Doffing Station OUTSIDE the isolation room

5 Protective eye wear/face shield

Outside of protective eye wear/face shield are contaminated

- Remove face shield from the back by lifting head band.
- Remove goggles by ear pieces.
- Single use items are discarded in rubbish bin.
- If the item is reusable, ensure it is cleaned appropriately at Doffing Station.



6 Perform hand hygiene



7 Surgical mask or N95/P2 respirator

Outside of mask/respirator is contaminated

- Grasp bottom elastics of the N95/P2 respirator, then the ones at the top, and remove without touching the outside of mask.
- or
- Undo bottom ties of surgical mask, then top ties, and remove mask.
- Discard in rubbish bin.



8 Perform hand hygiene



Minimum requirements for Contact Precautions (Orange Isolation Room Card)



Gloves are required for substantial contact with the patient or the patient's environment, and as per standard precautions i.e., exposure to blood/body fluids.

A sleeveless disposable apron is the minimum protection required for clothing, on entry to an Orange Carded room.

Gloves

Examples of patient care interactions that may not be considered substantial include:

- dispensing of medications
- collection of water jugs or rubbish bags
- patient observations including blood pressure monitoring.

If substantial contact with the patient or environment is not anticipated, gloves are not necessary for healthcare workers upon entry to the room. Perform hand hygiene before entry and on exiting the room, and as per the 5 Moments for Hand Hygiene.

Gloves should be made available in the patient's room to facilitate their use when required, and to support changing of gloves as recommended within the 5 Moments for Hand Hygiene

Aprons

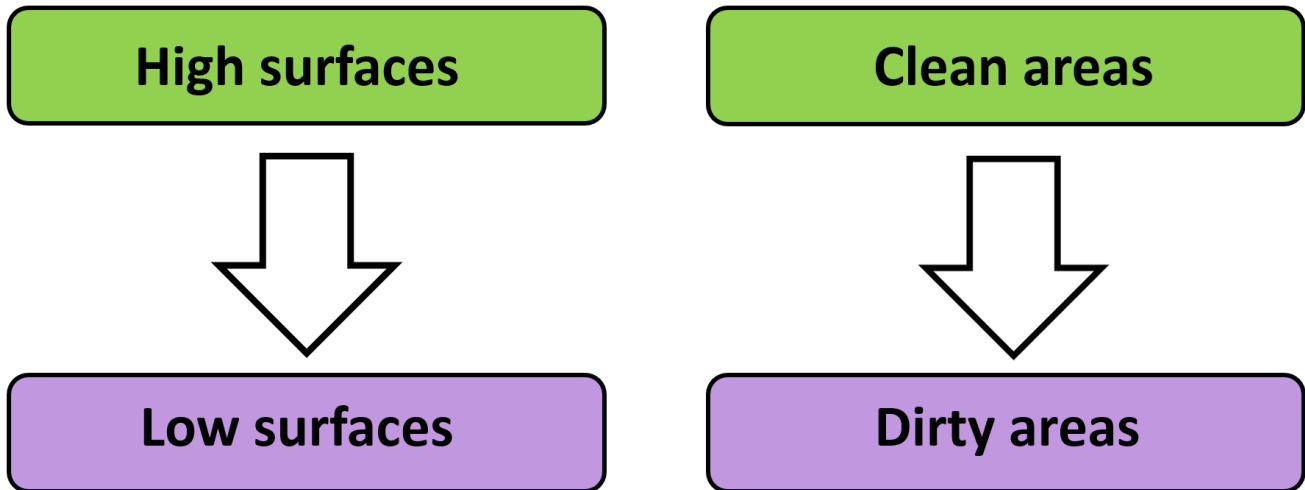
Aprons, at a minimum, should be worn by ALL healthcare workers entering an Orange Carded room (except catering staff). These should be removed before exiting the room, and hand hygiene performed.

For substantial patient contact, where more complete coverage of HCW uniform and arms is required, disposable long-sleeved gowns should be donned prior to entering the room, and removed prior to exiting, and hand hygiene performed.

Principles of cleaning “Golden rules”

Cleaning workflow must be from ‘high’ surfaces to ‘low’ surfaces and from clean areas to dirty areas. For example:

- ‘Clean’ areas include over bed lamps or lights, patient lockers, heart tables
- ‘Dirty’ areas include low surfaces of bed, toilets, sluice



A fresh clean solution must be made up for each room.

When making up a cleaning solution: follow written instructions, check expiry date, prepare just before use and change solution frequently.

Change the cleaning detergent and rinse water frequently (dependent on the extent of soiling) during cleaning of a patient area. This aims at keeping microbial load to a minimum in the cleaning and rinse solution.

Use damp cloths, not dripping wet cloths. Do not put more fluid on surfaces than necessary. See Microfibre Ratios

Remove or mop up any excess fluid.

Always dispose of cleaning cloth appropriately when you have finished cleaning each individual patient area. Blue and red cloths are launderable, yellow disposed

Always wipe in an “S” shape not circular motion

Exchange gloves and perform hand hygiene in-between cloths

When a cloth has been used to wipe surfaces in a bed unit area, do not return the cloth into cleaning solution; otherwise the cleaning solution will need to be changed prior to cleaning the next patient bed area. i.e. do not “double dip”

Dispose of cleaning cloths if they become visibly contaminated. Blue and red cloths are launderable, yellow disposed

Cleaning cloths must not be left sitting in buckets of solution. Empty bucket and discard cloths immediately after use.

Cleaning equipment must be appropriately cleaned and decontaminated immediately after use and stored in a manner that prevents contamination.

At the end of the shift, dispose of any left-over mop pads, rinse the Microfibre charging bucket thoroughly with water and detergent. Turn the Microfibre charging bucket upside down to dry. Blue mop pads are launderable, yellow disposed

Wipe down the mop handle after every use

Launderable (blue) mop pads and (blue) cleaning cloths placed in clear plastic bag. Clear plastic bag placed in blue linen bag. Send for laundering at the end of each shift

Avoid high dusting during invasive patient care activities for example suctioning, wound dressings and cannula insertions. Discuss with nursing staff an appropriate time for high dusting to occur.

Dispose of cleaning solutions in the sluice, never down a hand basin.

Close the lids of wipes containers to stop the wipes drying out.

Always clean in accordance with the procedures outlined in PSS Cleaner & PCA Manual.

BATHROOM

Key safety principles:

When cleaning bathrooms and shower areas, it's important to prioritise musculoskeletal injury prevention by using proper body mechanics and tools. Start by ensuring your posture is correct; avoid bending or twisting at the waist, which can strain your lower back. Instead, squat or kneel to reach low areas and use your legs for lifting. Make sure to position yourself at a comfortable height, and avoid excessive reaching overhead or bending for long periods. To reduce strain on your shoulders and wrists, use long-handled cleaning tools such as a mop or duster, especially when cleaning high or hard-to-reach areas. Additionally, take frequent breaks to prevent fatigue and allow your body to recover.

Another key aspect of injury prevention is managing the amount of force used during cleaning tasks. For scrubbing, opt for tools with ergonomic handles that allow you to maintain a neutral wrist position, reducing the risk of repetitive strain injuries. If you need to move heavy items, or equipment use proper lifting techniques, such as bending at the knees and keeping the item close to your body. Maintain a steady pace to avoid overexertion, and always ensure the floor is dry and free from slips or tripping hazards to prevent falls. By staying mindful of your posture and using supportive tools, you can reduce the risk of musculoskeletal injuries while effectively cleaning the bathroom.



Basin - hand

Daily and when visibly soiled

Equipment:

- Gloves
- Cleaning cloth (red)
- Paper towels and key
- Go Getter
- Hazard / wet floor signs
- Liquid soap dispenser refill
- Scouring pad
- 4.8L bucket (red)

Procedure / Steps	Safety Points
<i>When cleaning bathroom areas work from cleaner areas toward more dirty areas - mirror, then shelf, then paper towel dispenser then soap dispenser then hand basin.</i> 1. Perform hand hygiene 2. Apply gloves 3. Damp wipe over mirror then shelf, then paper towel dispenser – use a paper towel to dry. See Glass and mirror procedure 4. Wipe soap dispenser (including the hand dispenser lever) 5. Using cleaning solution Cream R7 and a damp, not wet, red cleaning cloth, wipe over edges of the basin 6. See Microfibre cloths 8 sided fold method and Damp dusting procedures 7. Clean the basin surrounds and the taps (including pipes and underneath the basin)   8. Clean the sides and then the base of basin, working towards the plug hole 9. If required use scourer to remove dirt or calcium build up from the base of the basin and around the plug hole.	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 

10. Using clean cold water rinse all surfaces to remove residue from cleaning solution
11. Turn on the tap/s and let water run for one minute
12. Turn off the tap/s
13. With a dry red cleaning cloth wipe over all surfaces to remove water build-up / marks
14. Check the floor for any water spills, mop if necessary. See [Damp mopping](#) procedure
15. Once finished, dispose of scourer. Place red cleaning cloths in laundering bag
16. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
17. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry
18. Remove gloves
19. Perform hand hygiene
20. Open paper towel dispenser with key and restock if necessary. Push closed for automatic locking of the dispenser
21. Open soap dispenser check level – replace if necessary and check the dispenser is working correctly by priming the hand lever
22. Return cleaning equipment to cupboard
23. Perform hand hygiene



Go Getter



When removing water jugs and cups etc. fluids are not be disposed of in patients sink

Cleaning solutions are not to be disposed of in a patient's sink

Dusting (Dolly) mops are not be washed in a patient's sink

Exchange gloves and perform hand hygiene in-between cloths



Government of Western Australia
East Metropolitan Health Service



Royal Perth
Hospital

Daily running of taps

To remove contaminants, and ensure the cleanest water possible for our patients, all taps must be run for one minute

Single handle taps run for one minute, handle in the centre



Double handle taps, run each side cold and hot, for one minute each



Although it may seem like a waste of water, it is important that all pipes have clean water running through them every day

Please see your Supervisor with any questions.

rph.health.wa.gov.au > healthywa.wa.gov.au

Where tradition plus innovation equals excellence

Glass and mirror

As required

Equipment:

- Paper towels
- 4.8L bucket (red)
- Cleaning cloth (red)
- Windex may be used

Procedure / Steps	Safety Points
<i>When cleaning bathroom areas work from cleaner areas toward more dirty areas - mirror, then shelf, then paper towel dispenser then soap dispenser then hand basin.</i> 1. Perform hand hygiene 2. Apply gloves 3. With a damp, not wet, red cleaning cloth, wipe over glass  4. See Microfibre cloths 8 sided fold method and Damp dusting procedures 5. Wipe over with paper towel until glass is clean and dry 6. Remove gloves and perform hand hygiene 7. Once finished, dispose of cleaning cloths appropriately. Blue and red cloths are launderable, yellow disposed 8. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture  Exchange gloves and perform hand hygiene in-between cloths

9. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry
10. Perform hand hygiene

For hard to remove marks on glass and mirrors, Windex may be used.

Spray a small amount directly from bottle to glass/mirror onto mark from approximately 15 – 20 centimetres (cm) and wipe with clean cloth



Asset JF



WINDEX may be used

Shower recess

Daily or as directed

Equipment:

- Gloves
- Asset JF
- Wet floor / hazard signs
- 4.8L bucket (red)
- Charge bucket (grey)
- Mop pad (red)
- Grout brush
- Cleaning cloth (red)
- Scouring pad
- Cream R7

Procedure / Steps	Safety Points
Check bathroom area for any pools of water, and mop prior to commencing task. See Damp mopping procedure Plastics ward shower recess is cleaned after each patient use. Additional cleaning may be requested in event a shower recess is left visibly soiled after patient use. 1. Perform hand hygiene 2. Apply gloves 3. Remove patient equipment from the shower recess 4. Place wet floor / hazard signs in highly visible areas 11. Using a damp, not wet, red cleaning cloth, start wiping. From the top working in a downward motion, clean all wall tiles surrounds & taps in the shower. Working from high to low. See Microfibre cloths 8 sided fold method, Wall and Damp dusting procedures 	 Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture

5. Ensure all soap bars that are left on the soap holder are disposed of and then clean soap holder ensuring it is free of all residue
6. Use scourer to remove build up from the base of shower or around the plug hole
7. Use Cream R7 and grout brush to remove build up on tiles, grout and corners. Check grout brush and replace as required.
8. Rinse shower from top to bottom with clean water using the shower head
9. With a clean dry (red) cloth wipe surface area of all walls, surrounds and tap fittings to remove any excess water.
10. Mop floor and surrounding areas ensuring, it is semi dry and no pools of water are left. When mopping in a shower use grey charging bucket with a red mop handle. [See Damp mopping](#) procedure
11. Once finished, dispose of scourer and cleaning cloths appropriately. Blue and red cloths are launderable, yellow disposed
12. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
13. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry.
14. Return cleaning equipment to cupboard
15. Remove and dispose of gloves
16. Perform hand hygiene
17. When floor is dry remove wet floor signs and return to storage area
18. Perform hand hygiene



Asset JF



Cream R7

Exchange gloves and perform hand hygiene in-between cloths



Plastics ward shower recess is cleaned after each patient use

Additional cleaning may be requested in event a shower recess is left visibly soiled after patient use

Toilet

Twice daily & as required

Equipment:

- Gloves
- Cleaning cloth (red)
- Goggles
- 4.8L bucket (red)
- Room Care R2L
- Toilet brush
- Asset JF
- Grey charging bucket
- Mop pad (Blue)
- Wet floor/hazard signs

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply gloves and other PPE such as goggles due to splash risk when cleaning with toilet brush. 3. If toilet brush appears worn, discard and replace 4. Flush toilet 5. With a damp, not wet, red cleaning cloth, work from cleanest area to dirtiest. See Microfibre cloths 8 sided fold method and Damp dusting procedures 6. If there is no cistern, wipe over wall area behind the toilet. See Wall procedure 7. Wipe the sides of the cistern, paying attention to the flush button / lever 8. Wipe toilet paper dispenser – paying particular attention to wiping the underside 9. Wipe the outside of the pan and 'S' bend 10. Wipe lid of toilet, seat and underneath the seat 11. Direct Go Getter under the rim of the bowl and allow to run down to the water 12. Use this solution and the brush to scrub all areas in and around the bowl	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 



13. Flush the toilet, rinsing the toilet brush at the same time
14. Wipe over rim of toilet
15. Wipe over the toilet brush holder with a damp paper towel
16. Dispose of cleaning cloths appropriately. Blue and red cloths are launderable, yellow disposed
17. Mop the area around the toilet. Mop floor using grey charging bucket, red mop handle & blue mop pads. See [Damp mopping](#) procedure
18. Send mop pad for laundering
19. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets and cleaning mop handle
20. Empty bucket/s and clean with Asset JF and turn bucket/s upside down to dry.
21. Clean mop handle using Tuffie Wipe.
22. Remove and dispose of gloves
23. Perform hand hygiene
24. With clean hands, refill toilet paper dispenser



Go Getter

If toilet brush appears worn, discard and replace



Asset JF



Exchange gloves and perform hand hygiene in-between cloths

Urinal

Daily

Equipment:

- Gloves
- 4.8L bucket (red)
- Wet floor / hazard signs
- Cleaning cloth (red)
- Grey charge bucket
- Room Care R2L
- Asset JF
- Mop pad (blue)
- Goggles

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply gloves and goggles 3. Display wet floor/hazard signs 4. Flush urinal and using toilet brush scrub all internal surfaces with Go Getter  5. Flush and rinse away 6. With a damp, not wet, red cleaning cloth, wipe all external surfaces of the urinal and spot clean walls as required. See Wall, Microfibre cloths 8 sided fold method and Damp dusting procedures 7. Mop the area around the toilet. Mop floor using grey charging bucket, red mop handle & blue mop pads. See Damp mopping procedure 8. Send mop pad for laundering 9. Dispose of cleaning cloths appropriately. Blue and red cloths are launderable, yellow disposed 10. Remove and dispose of gloves, then remove goggles and perform hand hygiene.	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture   

11. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
12. Empty bucket/s, clean with Asset JF and turn bucket/s upside down to dry.
13. Remove and dispose of gloves
14. Perform hand hygiene



Asset JF



Go Getter

Exchange gloves and perform hand hygiene in-between cloths

EQUIPMENT

Key safety principles:

Avoiding musculoskeletal injuries when handling patients and hospital equipment is crucial for healthcare workers who regularly engage in physically demanding tasks. The first step is to always use proper body mechanics when handling or moving patients. Bend at the hips and knees rather than the waist, keeping your back straight. This helps to engage your leg muscles, which are stronger and more capable of handling the strain. When handling or transferring a patient, make sure to keep the load close to your body to reduce the strain on your back, shoulders, and arms. Use assistive devices like hoists, air assisted equipment and sliding sheets whenever possible, to minimise the physical effort required. Additionally, always ensure that the area around the patient is clear of obstacles to reduce the risk of sudden movements or awkward positions during the lift.

For hospital equipment, it is important to recognize the weight and size of the items you're handling. Avoid pushing or pulling heavy equipment over long distances. Instead, use carts or transport devices that are designed for easy maneuverability, and make sure they are properly maintained to ensure smooth operation. When lifting or moving equipment, ensure that the item is at an appropriate height, so you're not forced to bend down or reach too high, both of which can strain the back and shoulders. If the equipment is bulky or difficult to move, ask for assistance from a colleague, as teamwork can help distribute the load and reduce the risk of injury.

Another important aspect of injury prevention is maintaining awareness of your surroundings and patient needs throughout the process. Before moving a patient, assess their weight, condition, and mobility level to determine whether any additional help or tools, like transfer boards or hoists, are needed. When using hospital equipment, ensure that it's set up to be easily accessible and that you're working within a safe, ergonomically-friendly space. Regularly rotating tasks with colleagues can also help to prevent fatigue and overuse injuries. Taking breaks, stretching, and focusing on posture throughout the day can further support musculoskeletal health, allowing healthcare workers to perform their roles more safely and effectively



General principles for cleaning patient care equipment:

The first step in reprocessing reusable equipment after each use must be thorough cleaning. All shared non-disposable equipment that contacts the patient must be cleaned (and disinfected, if required) and allowed to dry:

- Between each patient use
- After contamination with blood and body fluids
- At regular intervals as part of the cleaning schedules

Manual cleaning should be performed in a methodical manner that includes all surfaces. Including the underside of items that may be heavily soiled, such as commode chairs. Wipe in a “figure 8” or “S” shape pattern to avoid retracing the area already cleaned and prevent recontamination.

The manufacturer’s instructions should be used to assess the appropriate cleaning method and solution for each piece of equipment.

Equipment that is damaged, has broken non-intact surfaces or no longer functions as intended or cannot be properly cleaned, should be repaired or discarded.

Items to be returned to the manufacturer must be cleaned to remove organic material prior to return. Contact the manufacturer for specific instructions on cleaning and transporting items.

Items to be sent to the Medical Technology and Physics Department or Facilities Management must be cleaned first. Contact these departments for specific instructions on cleaning and transporting items.

Patients on transmission-based precautions require dedicated equipment during their admission (e.g. tourniquet, commode, sphygmomanometer cuff). This equipment requires cleaning and disinfecting or discarding upon discharge.

Alcohol impregnated wipes are not designed for cleaning and should only be used to clean items in which alcohol is the manufacturer’s preferred cleaning agent.

Following cleaning, items should be checked to assess the effectiveness of the cleaning

Air mattress (Talley Quattro rental)

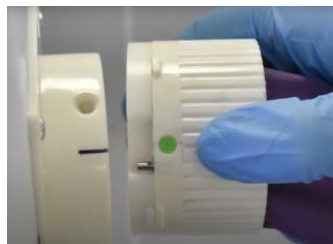
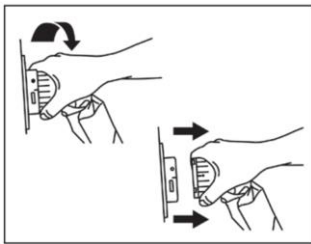
When a patient is discharged

Equipment:

- Gloves
- Cleaning cloth (blue)
- 4.8L bucket (blue)
- Air mattress
- Asset JF

Procedure / Steps	Safety Points
  Training video – overview Talley Group QUATTRO Acute mattress system, reducing pressure ulcer incidence Training video – set up Setting up a Quattro Plus Mattress System (youtube.com) Training video – button functions Quattro Plus Button Functions (youtube.com) Training video – fault finding Quattro Plus Fault Finding (youtube.com) Training video – deflation, disconnect & pack for collection Quattro Plus Mattress Deflation (youtube.com) This procedure is generally completed by: • PCAs in patient rooms that were not isolation rooms • Cleaners in patient rooms that were isolation rooms	 Seek assistance to help with manual handling of air mattress. Two handlers required Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body

1. Perform hand hygiene
2. Apply gloves
3. Remove any remaining linen from mattress
4. Unplug electrical connection by gently turning Turn anti-clockwise and gently pull out. This allows the airlines in the mattress to deflate. The deflation time is typically 10-15 minutes. It is preferred that the mattress is deflated prior to rolling. **This is to reduce the amount of force required by the handlers to fold the mattress up.**



5. **Prepare mattress for bagging/removal - Seek assistance to help with manual handling of air mattress. Two handlers required.**

See [Air mattress cleaning and returning SOP](#) and [Air mattress quick reference guide](#)

6. Using a damp, not wet, blue cleaning cloth, clean the mattress. Start by wiping the top and deflated sides, then clean the underside of the mattress while rolling up
7. Using a damp, not wet, blue cleaning cloth wipe down the pump
8. See [Microfibre cloths 8 sided fold method](#) and [Damp dusting](#) procedures
9. Using small rolls, roll from the foot end first, keeping the tubing between the fold
 - The deflation time after the electrical connection is unplugged, is typically 10-15 minutes. It is preferred that the mattress is deflated prior to folding. This is to reduce the amount of force required by the handlers to fold the mattress up



Avoid stooped posturing

Keep neck in neutral posture



Asset JF

Exchange gloves and perform hand hygiene in-between cloths

For noting:

This procedure is generally, completed by:

- PCAs in patient rooms that were not isolation rooms
- Cleaners in patient rooms that were isolation rooms

10. Red bag:

Fully Launderable

The entire mattress can be thermally decontaminated to Department of Health Circular HSG(95) 18 standards for effective infection control.



Non-Isolation Rooms - place air mattress in single red bag and secure with cable tie for collection (supply for red bags kept on ward/unit and refilled by mattress supplier)

Isolation Rooms - place mattress in a red bag and cable tie, and then place this in a second red bag ("double bag") and secure with another cable tie



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.



11. Once bagged, place mattress on a trolley and wheel to the usual designated area on ward/unit using safe manual handling principles. Pump and power cable do not get placed in bag, but left outside of bag



12. Continue cleaning bed and surrounds as per [Discharge/transfer](#) procedure
13. Once finished, dispose of cleaning cloth/s appropriately. Blue and red cloths are launderable, yellow disposed
14. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
15. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry.
16. Remove and dispose of gloves
17. Perform hand hygiene
18. Return a standard mattress to the bed

19. Inform the ward coordinator that the air mattress is now ready for return to the hire/rental company for appropriate decontamination before reuse	
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Air mattress quick reference guide

QUICK REFERENCE GUIDE

QUATTRO ACUTE MATTRESS REPLACEMENT SYSTEM (MRS)

Weight Capacity 0 – 250kg

CONNECTION: MATTRESS TO POWER UNIT

1. ATTACH the air supply hose to the power unit
2. ALIGN the black line on the connector and power unit and pushing together.
3. ROTATE the mattress hose connector clockwise until the green indicator fills the indicator hole on the power unit connect.



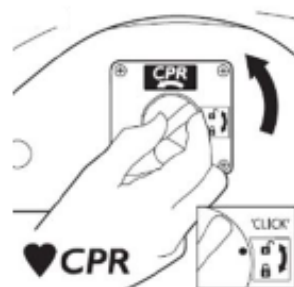
4. CONNECT the power lead to the power unit (left hand side)
5. PLUG IN to mains supply
6. SWITCH ON the power unit
7. The system will display STARTING
8. Then INITIALISING whilst the mattress inflates (inflation time 10-15 minutes)
9. DO NOT turn the power off during this stage
10. If this is done, the power unit must be SWITCHED OFF. Press the MUTE button to silence audible alarm. RE-CONNECT power, and SWITCH ON the power unit.
11. Once inflated the mattress will automatically default to "ACTIVE MEDIUM".

CPR FACILITY

The CPR release is situated at the HEAD END, on the RIGHT HAND SIDE of the mattress (viewed from the foot end).

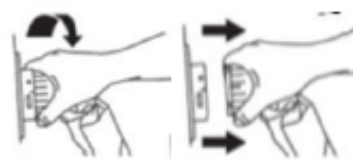
PLEASE NOTE; THE YELLOW CPR TAG IS AN INDICATOR – NOT THE CPR RELEASE

1. For rapid deflation, ROTATE the dial of the CPR device ANTI-CLOCKWISE to open.
2. If RE-INFLATING the mattress, ensure CPR device is ROTATED CLOCKWISE until it clicks closed position.
3. FASTER DEFLATION disconnect hose from pump and open. (Normally closes for transport mode)



TRANSPORT MODE

1. Press MAX INFLATE button to fully inflate the mattress.
2. When maximum pressure is achieved, DISCONNECT the hose by ROTATING the connector ANTI-CLOCKWISE until the black line aligns.
3. Place hose along the foot end of the mattress.
4. TRANSPORT MODE 12-15 HRS (weight dependent).



OPERATIONAL BUTONS

WA HEAD OFFICE
 21 Tulloch Way, Canning Vale WA 6155
 P (08) 9456 1661 F (08) 9456 0017

QUICK REFERENCE GUIDE

(Please note that instructions are also on the back and side of the pumps)	
THERAPY MODE BUTTON: 1. Located on the LEFT hand side of power unit. 2. TOGGLES between ACTIVE 1 IN 4 and CONTINUOUS LOW PRESSURE (CLP) modes. 3. HOLD down THERAPY MODE button for 2 SECONDS until the audible tone is heard. 4. The system will then display 'PLEASE WAIT' and requires approximately 2 MINUTES to initialise.	 THERAPY MODE
WEIGHT / COMFORT CONTROL: 1. DEFAULT setting is ACTIVE MEDIUM. 2. INCREASE or DECREASE the setting by UNLOCKING the pump and use UP and DOWN arrow buttons 3. Settings SOFT / MEDIUM / FIRM / SEATED Comfort settings is based on client preference or clinical reasoning.	  COMFORT CONTROL
DATA: This button will not need to be used on the ward and does not affect mode of operation.	 DATA
MUTE / UNLOCK: 1. Automatic LOCK is activated 2 MINUTES after the last operation 2. The LOCK ICON is on the bottom left of the display 3. PRESS and HOLD the MUTE/UNLOCK button for 4 SECONDS to unlock the display NB: After power failure/switching the power off, pressing MUTE cancels the system's previous settings. If the power is switched off for greater than 12 seconds, the power unit will default back to ACTIVE medium. If the power returns before a period of 12 seconds and the MUTE button has not been pressed, the systems will return to the previous mode of operation.	 MUTE / UNLOCK
MAX INFLATE: • Necessary for some nursing procedures. • MAX INFLATE mode inflates the mattress to maximum static pressure for a period of 15 minutes • After 15 minutes the system automatically returns to the previous mode of operation.	 MAX INFLATE

Air mattress cleaning and returning SOP

1. Air mattresses and pumps should be detergent cleaned as per normal hospital standards.
2. Ensure air mattress is in a sufficiently dried condition
3. Switch off the pump. There will be an audible beep indicating that the pump has lost power. Press the mute button to stop the beeping



4. Disconnect the power cable from the pump and from the wall socket. – Leave the power cable attached to the mattress.

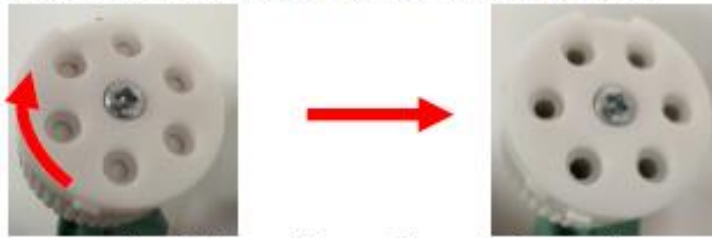


5. Deflate the mattress by rotating the dial of the CPR device anti-clockwise so that the black dot moves from the locked position  and 'clicks' into the open position .



NOTE: The CPR valve is situated at the head end on the right hand side of the mattress (viewed from foot end).

6. Disconnect umbilical hose from pump and open the valve by twisting the top plastic cap at the end of the cord. This helps with the deflation process.



7. Place the hose across the bottom of the mattress and place the power cord on top of the mattress ensuring the plug is facing outwards to prevent damage to the mattress cover.



Plug facing
outwards to
prevent damage
to the cover

8. Roll the mattress, foot end first, keeping the hose in the middle of the roll with the valve end protruding out the side (you will need access to the valve at step 10 below) and press down on the mattress to assist with the deflation process. **Make sure to fold the four black straps inside the roll as they interfere with the bagging process if left hanging.**



**Black
straps
folded
inwards**



Valve protruding
outwards. This
helps with the
deflation and
gives access to
close the valve
at step 10.

9. Repeat step 8 again if necessary to remove more air. The more air that is removed the easier it will be to place the mattress in the bag.
10. Once satisfied with the deflation process, close the CPR valve and the valve at end of the umbilical cord. This prevents the mattress from filling back up with air as you try to put it in the bag.
11. Place the rolled mattress on the edge of the bed with one end hanging off.



12. Slide the red bag over the end of the mattress and tip the mattress over so it falls into the bag.



13. Secure the bag with a cable tie for collection. It is helpful to remove any excess air from the red bag before tying. A supply of red bags is kept on ward/ unit and refilled by HospEquip staff.
14. Write the patient name, ward and pump number (Located on the front of the pump and begins with QAS...) on the bag with a texter.

15. Rotate the brackets on the back of the pump to sit flat. Do this by pressing down while rotating the brackets inwards.



16. Do not place pump in the red bag. Keep beside the bagged mattress.
17. Place bagged mattress and pump in the in "usual" area on ward/ unit
18. **Inform** shift coordinator who will advise HospEquip team electronically that hire has been stopped
19. Representative from HospEquip will collect mattress and pump as soon as practicable.

Bed

Safety checklist for bed Inspection SOP

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Hospital Wide	Nursing (including AINs), Patient Care Assistants, Orderlies, Technical Services, Facilities Management

Policy statement

The purpose of this checklist is to provide a guide across all clinical areas in the organisation to ensure all hospital beds are checked on a regular basis to identify faults and have these actioned appropriately.

Checking / inspection process

During the cleaning process of a discharge bed, the Patient Care Assistants (PCAs) shall perform the following checks:

- **Power cable and plug:** Unplug the power cable and check for damage to the orange power cable, taking special note of the plug top to identify any missing earth pins. See picture as per below.



- **Battery:** While the bed is unplugged, check the handset/remote control functions to ensure that it works. Check for any exposed wires connected to the handset/remote control.
- **Bed frame:** Perform a visual inspection on the entire bed for any damage, e.g., whole frame, side rails, headboard, footboard, brakes, and castors.
- **Once all checks are complete, please ensure the bed is plugged into mains power and switched ON.**

Note: Wards may store additional beds that are not in use. The PCAs should ensure all beds are plugged into mains power, and power turned on, at all times.

Reporting process

If beds are identified as "faulty", PCA shall report to the Shift Coordinator at the time. It is then the Shift Coordinator's responsibility to perform the following steps.

RPH only

- Complete a Technical Services 'Equipment Service Request' form and attach to the faulty bed. Do not use EMPAC or Engineering requisition form.
- Initiate a CARPS request for the removal of the faulty bed to the boiler room.



TECHNICAL SERVICES
MEDICAL ENGINEERING & PHYSICS DEPT.
 MEDICAL AND LABORATORY
EQUIPMENT SERVICE REQUEST

ROYAL PERTH HOSPITAL
 Request No.

137000

Contact Name _____ Tel. No. _____ Date _____
 Equipment Location (for return) _____
 Work Request (include equipment description) _____

 Equipment Control Number (from blue label) _____
 Please tick priority: ☐ 0. None ☐ 1. Now ☐ 2. A day ☐ 3. A week ☐ 4. A month

TO REQUEST URGENT REPAIRS, PHONE:
 TECHNICAL SERVICES 9224 2500

**CLEAN AND DECONTAMINATE ALL EQUIPMENT
BEFORE SENDING TO TECHNICAL SERVICES**

BHS only

- Complete EMPAC Request.
- Attach a copy of EMPAC to the bed.

Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.



Cleaner trolley

Daily

Equipment:

- Cleaning cloth (yellow)
- 4.8L bucket (yellow)
- Scourer
- Asset JF
- Rubbish bag
- Gloves

Procedure / Steps	Safety Points
 HIGH-SECURITY CLEANING CART   High security cleaning cart training video Resource Centre Rubbermaid Commercial Rubbermaid Commerc 1. Perform hand hygiene 2. Apply gloves 3. Prepare Microfibre cleaning cloths. See Microfibre cloth preparation, Microfibre cloths 8 sided fold method and Damp dusting procedures 4. Remove equipment from trolley 5. Empty rubbish bag	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 

6. With a damp, not wet, yellow cleaning cloth, wipe over cleaning trolley and remove all dirt and dust
7. If soiled use scourer to remove dirt
8. Once trolley is clean wipe over with cloth to dry
9. Once finished, dispose of yellow cleaning cloth/s
10. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
11. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry.
12. Remove and dispose of gloves
13. Perform hand hygiene
14. Place all equipment back onto trolley neatly
 - Integrated hooks for the hanging of signs



15. Perform hand hygiene

The cart should remain locked at all times



Asset JF



Do not line trolley with cloths or towels as this is an infection risk

Exchange gloves and perform hand hygiene in-between cloths



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Commode

After each patient use

Equipment:

- 4.8L bucket (red if commode not visibly soiled, yellow if soiled)
- Cleaning cloth (red if commode not visibly soiled, yellow if soiled)
- Sodium Hypochlorite 0.125% (if commode soiled)
- Asset JF (if commode not soiled)
- Wet floor / hazard signs
- Gloves
- Scourer

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply gloves 3. Assess level of contamination of commode, ensuring underneath of the seat is also inspected. If the commode is visibly soiled: 4. Take commode to shower recess. 5. Remove gloves and perform hand hygiene prior to collecting cleaning supplies 6. Place wet floor / hazard signs in highly visible locations when mopping and put on personal protective equipment 7. Collect cleaning supplies 8. Perform hand hygiene and apply gloves 9. Remove gross soilage with a detergent wipe prior to cleaning commode with a cloth, wiping from least to most contaminated area ● Cleaning cloth (red if commode not visibly soiled, yellow if soiled) 10. If necessary, use a scourer to remove heavy soiling 11. Always inspect and ensure underneath the seat is cleaned 12. Rinse commode in shower with clean water 13. Perform second clean with a clean yellow cloth using sodium hypochlorite 0.125% 14. Dispose of cleaning cloths and scourer 15. Remove and dispose of gloves and perform hand hygiene.	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture   

16. Wipe over commode with dry cloth
17. Mop over shower floor to remove excess water (see Damp mopping procedure)
18. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
19. Once finished, dispose of cleaning cloths, empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry.
20. Remove and dispose of gloves
21. Perform hand hygiene
22. When floor is dry remove wet floor signs and perform hand hygiene

If the commode is not visibly soiled, perform the above however:

Clean with a red cleaning cloth using detergent **ensuring the underside of seat is cleaned**

See [Microfibre cloth preparation](#), [Microfibre cloths 8 sided fold method](#) and [Damp dusting](#) procedures

Always consider correct body postures to reduce risk of injury when wiping, carrying, and handling equipment.

If commode seat is damaged, remove from use and report to Nurse Unit Manager



Asset JF



Sodium Hypochlorite
0.125%

Exchange gloves and perform hand hygiene in-between cloths



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out

IV trolley restock

At least once daily



Government of Western Australia
East Metropolitan Health Service

Royal Perth Bentley Group

IV TROLLEY STOCK REQUIREMENTS

IV trolleys are to be checked and restocked at least daily

DO NOT OVERSTOCK

Top Drawer:

Item	Quantity	Order Code
ProtectIV® Plus Safety IV Catheter:	10 each	
16G		2005L
18G		106604D
20G		105558C
22G		105559D
Extension tubing	4	190194W
Chlorhex swab stick	15	155051N
10mL syringe	15	112410B
20mL syringe	6	2424B
Prefilled 10mL 0.9 % sodium chloride syringe (PosiFlush®)	10	159405R
10mL saline ampoules	15	61003A

Second Drawer:

Item	Quantity	Order Code
Blood collection sets (Butterfly):		
21G	5	135940F
23G	5	136337N
Vacutainer device:		
Blue	5	135207C
Pink	5	141451J
Clear	5	135772H
Green	10	185832F
Assorted blood tubes		
Large alcohol wipes	10	155055W
IV pressure pads	10	201673C
Tourniquets	5	2486B
Adhesive tape	1	10486G
Crepe bandage	2	7228E
Blood gas syringe	5	105692E
IV dressing	15	163757R

Side Baskets:

Item	Quantity	Order Code
Small sharps container	1	16236E
Alcohol hand gel	1	142310Y
Detergent wipes (Tuffies®)	1	156520A
Brown paper bag	10	164678X
Specimen transport bags	15	9167J
Blood culture collection bottles	3	From RPH Lab
PIVC insertion packs	10	168981A
Sterile examination gloves:	3 each	
S		173690A
M		173691B
L		173692C
XL		173693E
Sterile gauze squares	6	125687K

Bottom of Trolley:

Item	Quantity	Order Code
Kidney dishes	10	EQ Library
'Bluey' protective sheets		47G

Rear of Trolley:

Rubbish bag 96117K

Leave trolley surface empty (do not affix stickers to surface)

To align with Cannula Peripheral Intravenous (PIVC) Clinical Guideline



Bactec blood culture bottles:

Note that 1 blood culture set =

- 1x aerobic bottle (grey top) and,
- 1x anaerobic bottle (purple top).



PCA trolley

Daily

Equipment:

- Cleaning cloth (yellow)
- Tuffies or Room Care R2L
- Scourer Pad
- Gloves
- Rubbish bag

Procedure / Steps	Safety Points
The PCA Trolley is cleaned at the end of a shift & re-stocked ready for the next day. 1. Perform hand hygiene 2. Apply gloves 3. Remove equipment from trolley 4. Empty rubbish bag 5. Wipe over cleaning trolley with Tuffies or Roomcare R2L and remove all dirt and dust 6. If soiled, use scourer to remove dirt 7. Once trolley is clean, wipe over with cloth to dry 8. Dispose of cleaning cloth and scourers 9. Remove and dispose of gloves 10. Perform hand hygiene 11. Re-stock the trolley contents for the next day as required. Place all equipment back onto trolley neatly. Do not line trolley with cloths, towels or linen 12. Perform hand hygiene	 Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture  Tuffie



Room Care R2L



Do not line trolley with cloths, towels or linen

Exchange gloves and perform hand hygiene in-between cloths



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Patient equipment - hoist, dressing trolley, IV poles etc

After each patient use

Equipment:

- Gloves
- 4.8L bucket (blue)
- Gloves
- Tuffies (smaller equipment)
- Asset JF (larger equipment)
- Room Care R2L
- Cleaning cloth (blue)

Procedure / Steps	Safety Points
For smaller pieces of equipment - hoist, dressing trolley, IV poles etc 1. Perform hand hygiene 2. Apply gloves 3. Wipe over small equipment using Tuffie wipes and/or R2L 4. Remove and dispose of gloves 5. Perform hand hygiene For large equipment: • use Asset JF or Room Care 2L (or Sodium Hypochlorite 0.125% where applicable) and Microfibre cleaning cloths • not Tuffie wipes Hoists - Wipe the hoist machine itself, not the slings	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 





Asset JF



Exchange gloves and
perform hand hygiene
in-between cloths



Tuffie



Room Care R2L



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

FLOORING

Key safety principles:

To avoid musculoskeletal injury when cleaning floors, it's important to use proper body mechanics and posture. When mopping, sweeping, scrubbing or polishing, always ensure you maintain a neutral spine and avoid hunching over. Stand with your feet shoulder-width apart to maintain stability and balance. When mopping or sweeping, avoid bending at the waist. Instead, bend your knees and use your legs to move and push the mop or broom, keeping the equipment close to your body to reduce strain on your back and shoulders. Additionally, use long-handled mops and brooms to prevent excessive bending or reaching, especially when working in hard-to-reach areas.

Taking regular breaks and using ergonomic tools can also help minimise the risk of injury. Use mops or polishers that are adjustable in height to keep the handle at a comfortable level for your body. If you're mopping or sweeping for an extended period, take short breaks to stretch and relieve muscle tension, particularly in your back, shoulders, and wrists. Changing positions or alternating between tasks can also help reduce the repetitive strain that can lead to musculoskeletal injuries. By practicing good posture, using ergonomic equipment, and incorporating breaks, you can significantly reduce the risk of injury while performing floor cleaning tasks.



Auto scrubbing (floor scrubbing)

As required

Equipment:

- Auto scrubber
- Red scrubbing pad
- Cleaning cloth (blue)
- Reveal (floor cleaner)
- Wet floor / Hazard signs
- Safety Glasses

Procedure / Steps	Safety Points
 T300 training video https://www.youtube.com/watch?v=T90VECUhPF0  T2 training video https://www.youtube.com/watch?v=sNUKrKyEFnY Pre – start checks 1. Perform hand hygiene	 Auto Scrubbing is to be completed by trained staff ONLY Check charging cord for breaks or splits Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 

2. Inspect the floor area: Clear area to be buffed of obstacles, rubbish and furniture that could damage the machine or impede its movement. Sweep or vacuum the floor to remove all loose debris before starting the scrubbing process. See [Static sweeping](#) or [Vacuuming](#) procedures
3. Unplug the Auto Scrubber from the wall charging point and turn the power point off
4. General inspection: Ensure the scrubber's body and components are in good condition, free from cracks, damage, or visible wear. Check for any loose parts or fasteners.
5. Check scrub brushes/pads: Inspect the scrubbing brushes (T2) or pads (T300). Make sure they are securely attached and in good condition. If they're worn out or damaged, replace them before use



6. The base plate magnetises. Fit by hand ensuring the pad is secured via the lugs. The red pad is for general floor maintenance. Only use a green pad if directed by a Supervisor



7. Check squeegees: For both models, check the squeegee blades for damage or excessive wear. Ensure that they are clean and functioning correctly
8. Check water hoses to ensure they are not split
9. Perform hand hygiene and apply goggles

If unable to control the machine release the lever and it will stop immediately



Asset JF is generally used. Only use Reveal for heavy duty soilage if directed to by a Supervisor



Reveal, 1:100 ratio

Asset JF is generally used. Only use Reveal for heavy duty soilage if directed to by a Supervisor



Adjust water spray as required so you don't flood the area

10. Solution tank: Check the solution tank to ensure it is clean and free from debris. Fill the solution tank with warm water or Asset JF to the fill line indicator as marked on the inside of tank and close lid.

- Asset JF is generally used
- Only use Reveal at a 1:100 ratio for heavy duty soilage if directed to by a Supervisor

11. Recovery tank: Inspect the recovery tank for any remaining dirty water from previous use. Empty and clean it if necessary.

12. Check batteries: Verify that the battery is fully charged, check the battery terminals for corrosion and ensure they are properly connected

13. Place wet floor / hazard signs at each end of the area that you are Auto Scrubbing

14. Ensure the surrounding area is clear of people



Report any equipment damage or malfunction to a Supervisor immediately. If unsafe to use the equipment do not use. Tag it out.

Safe operation

15. Position the scrubber at the start of the area to be cleaned, ensuring it is on a stable, flat surface

16. Release levers for base plate and squeegee to ensure they are resting on the floor





17. Press the power button to turn on the machine. Confirm that the battery charge level is sufficient for the task
18. **T300 only.** Allow the machine to self test on start up. Refer to the ProPanel LCD touch screen tutorial on the T300. The T2 does not have a ProPanel LCD touch screen



19. Turn on water switch then turn on scrubbing rotate switch
20. Adjust water spray to “medium” as indicated on the dial on the control panel on top of the machine. Make sure the solution is evenly distributed onto the floor
21. Set the speed of the machine. For beginners, it’s best to use a lower speed and gradually increase it as you become more familiar with the machine.
22. Ensure brushes and pads are rotating correctly
23. Start scrubbing:
 - Hold the scrubber handle firmly with both hands for stability
 - Start moving the scrubber in a forward direction, guiding it in slow, controlled passes

- Move in a consistent, overlapping pattern to ensure the entire floor is scrubbed thoroughly

24. **T300 only.** The machine will automatically adjust the cleaning solution flow and brush pressure based on the selected mode, but you should still ensure it's operating at the proper speed for your task
25. Always keep the scrubber moving, avoid holding it in one place, as this can cause streaking or damage to the floor



26. If unable to control the machine release the lever and it will stop immediately or press the emergency stop button
27. **T300 only.** The volume can be reduced on the T300, not T2
28. When you are nearing a turn or corner, adjust the water spray on the dial to "low" so there is no water build up / pools of water left on the floor
29. Continuously monitor the recovery tank for buildup of dirty water. If the tank becomes full, stop the scrubber, empty the tank, and continue. This ensures the best cleaning performance
30. Inspect the floor periodically to ensure it is being cleaned effectively. Adjust the scrubber settings if necessary, particularly if you are cleaning a highly soiled area
31. Ensure that the squeegee is functioning properly by checking that water is being picked up and not left behind. Never leave standing water. Always ensure the scrubber is functioning correctly with the squeegee to prevent water from pooling on the floor
32. When you are nearing completion turn off water switch, then turn off the scrubbing rotate switch. Adjust levers so that the base plate and squeegee are lifted off the floor

33. Remove goggles and perform hand hygiene

Post use checks and maintenance

34. Once complete mop up any excess water off the floor from underneath the machine

35. Empty the recovery tank: For both models, immediately empty the recovery tank to avoid standing dirty water, which could lead to bacterial growth or unpleasant odours. Rinse the recovery tank to remove any residue and prevent clogs or smells. Avoid overloading the recovery tank. Always empty it promptly to prevent the machine from becoming too heavy or difficult to manoeuvre

36. Check all of the scrubbed area for any spills and damp mop if necessary. See [Damp mopping](#) procedure

37. Clean scrub brushes/pads:

- T2: Remove the scrub brushes and clean them with water. If needed, use a brush cleaner or a hose to clear any accumulated debris or dirt
- T300: Detach the scrub pads and rinse them with water. Ensure that no cleaning solution or dirt is left on the pads to avoid streaking on the next use

38. Clean the solution tank. Rinse out the solution tank thoroughly, removing any leftover cleaning solution. If using a chemical dispenser, flush the system with clean water to prevent buildup of chemicals in the lines

39. Clean the solution filter after tank is drained

40. Wipe clean lids and seals to maintain suction

41. Leave the tank lid open to ventilate. If left closed odours can develop

42. Inspect for damage

- Examine the scrubber for any signs of wear or damage
- Check for worn-out brushes/pads, issues with the solution dispenser, or clogged parts
- Inspect the squeegee for damage or buildup that could prevent effective water pickup
- Clean or replace parts as necessary.

43. Clean the squeegee assembly • Remove the squeegee assembly from the scrubber and clean it with water • Inspect the blades for wear and replace them if necessary • Ensure that all parts of the squeegee assembly are in good condition and reattach it securely to the machine 44. On both models, check the battery and recharge if necessary to ensure it's ready for the next use. Clean the battery terminals to ensure there is no corrosion or buildup. 45. Once clean re - attach base plate and scrubbing pad 46. Perform hand hygiene 47. When the floor is dry remove wet floor / hazard signs 48. Storage and packdown. Return Auto Scrubber to allocated storage area. Store the scrubber in a cool, dry area. Ensure it is not exposed to excessive moisture or temperature extremes 49. Plug into charging socket and turn on power supply 50. Inspect brushes and pads. Scrub brushes or pads should be checked regularly to prevent uneven cleaning and to ensure they are performing optimally. Store any scrub brushes, pads, or cleaning accessories in a designated storage area. Ensure they are dry and clean before storing to prevent mould growth or damage. 51. Perform hand hygiene 52. Record any maintenance performed (such as brush replacement or battery recharging) to ensure future issues are addressed promptly. Keep a maintenance log to track the machine's usage and repairs for future reference	
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Buffing floor

Weekly

Equipment:

- Gloves
- Auto Buffer
- Buffing pad (white)
- Snapback and spray bottle
- Cleaning cloth (blue)
- Safety Glasses

Procedure / Steps	Safety Points
 How to Operate a Polishing/Scrubbing Machine PV25. C25 AND C27 POLIVAC INTERNATIONAL PTY LTD Training video 40cm Suction Polisher PV25 - Polivac Australia Pre – start checks 1. Perform hand hygiene 2. Check floor condition. Sweep or vacuum the floor thoroughly to remove dust, debris, or obstacles that may damage the buffer or interfere with buffing. See Static sweeping or Vacuuming procedures 3. Watch for electrical hazards: Never use the buffer in wet conditions or on damp floors	 Buffing is to be completed by trained staff ONLY Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture If cord is faulty, advise your supervisor immediately and take machine to housekeeping for servicing

4. Unwind power cord on buffing machine and ensure the power cord is intact, with no frays, cuts, or exposed wires. If damaged, replace the cord immediately
5. Examine the body of the buffer for any signs of wear, cracks, or damage to the housing
6. Check the handle for secure attachment and easy maneuverability.
7. Ensure all switches (e.g., power, speed controls) are functioning smoothly without sticking or malfunctioning
8. Lay buffing machine handle down on the floor and attach base plate. Verify the condition of the buffer pad. If worn down, replace it with a new pad. If using a polishing pad, check that it's clean and free of debris. Ensure the pad is properly secured to the machine base, with no loose attachments



9. **Suction model:** Inspect the suction hose for any clogs or damage. Clear any blockages and replace any damaged parts. Make sure the collection bag or container is empty, clean, and securely attached. Ensure that any suction filters are clean and intact
10. Attach white buffing pad ensuring it is secure:
 - Place the brush or pad holder over the clover leaf connection and turn the enclosure clockwise to lock into position.
 - If the connection has a metal clutch, locate the brush or pad holder on the metal drive clutch, by slotting grooves together, then turn brush or pad holder sharply to the left
11. Raise handle of machine back into the upright position
12. Unwind power cord on buffing machine and ensure the power cord is intact, with no frays, cuts, or exposed wires. If damaged, replace the cord immediately
13. Place the buffer on a stable, level surface before starting
14. Plug cord into power point and turn on power at socket

If unable to control the machine release the lever and it will stop immediately



Safety glasses are not normally required. However, their use is recommended in those cases where splashes may occur



Snapback

Use 200ml in
1 litre of water



15. Ensure the red safety light is on. Power supply will be visible via the neon indicator at the power plug and also the red light on the handle
16. **Suction model.** Turn on the suction motor by activating the suction switch (separate from the rotary switch). Ensure the suction hose is positioned in a way that allows easy debris collection
17. The handle height must be adjusted to suit individual operator's body proportions
 - Use the lever at the base of machine to adjust/tilt the handle height so it sits just below your waist
 - Adjust the ergo handle to the correct height by placing your foot or hand on the ratchet lever then positioning the handle
 - The correct height of the handle is around the groin to just below the waist region.
18. Flip the wheels up and secure
19. Perform hand hygiene
20. Safety glasses are not normally required. However, their use is recommended in those cases where splashes may occur
21. Apply gloves
22. Prepare diluted Snapback solution in spray bottle
23. Ensure the surrounding area is clear of people

Safe operation

22. Hold lever in which is located on the left-hand side of the handle to turn machine on. To start the machine, first you must depress the red safety button that is next to the hand trigger, then grasp either of the red triggers, and the motor will engage the brush.
23. Hold the buffer handle securely with both hands, maintaining a firm grip. Gradually pull the trigger to initiate the buffer's movement
 - Do not force the trigger without first releasing the safety button as this will damage the safety release in the trigger mechanism
 - Once the machine is on, the safety button can be released
24. **PV25 only.** To use the vacuum system whilst polishing, switch the vac motor to the on position. The vacuum will be activated upon starting the machine



Report any equipment damage or malfunction to a Supervisor immediately. If unsafe to use the equipment do not use. Tag it out.

25. Slightly lift the handle up and manoeuvre the wheels backwards and up out of the way. The wheels must be off the floor when polishing

- When the handle is set at the correct height, the arms should grip the handle in a comfortable, but loose manner
- If arms are bent at the elbows this is an indication that you are applying too much effort to control the machine. Re-adjust the handle



26. Begin moving the buffer in a slow, controlled manner, covering the floor in an even, overlapping pattern. Start at one end of the room and work towards the exit, ensuring you're not cornered

- When buffing, move the buffing machine from side to side and spray lightly with diluted Snapback
- Turn and control buffer with lunging actions
- To facilitate the correct swinging motion of the machine, tip or lift the handle slightly forward to get the polisher to move right and lower the handle to get the machine to move towards the left. This requires minimal effort
- If you are fighting the machine for control you have the handle set at the wrong height or your pad or cloverleaf locking mechanism is damaged

27. **Suction model.** As you buffer, the suction will collect dust and debris from the floor. Ensure the collection bag or tank is checked regularly to prevent overfilling. Move the machine slowly across the floor, overlapping each pass, ensuring all debris is picked up and the floor is thoroughly cleaned

28. Keep the buffer moving constantly

- Do not let it stay in one place to avoid damaging the floor or creating uneven polishing
- Overlap each pass by about 50% to ensure consistent results
- Adjust the speed if necessary based on the floor type and desired finish
- Maintain a steady, even pressure on the buffer. Avoid pushing too hard, as this can cause damage or wear the pad unevenly
- Avoid overloading: Do not apply excessive pressure on the buffer. Let the machine do the work and only guide it smoothly

29. If unable to control the machine release the lever and it will stop immediately

Post use checks and maintenance

22. Once the area has been sufficiently buffed, lower the wheels back onto the floor. Power off the rotary motor and suction motor (if applicable).

23. Unplug the machine from the power source, ensuring the machine has completely powered down

24. Remove and dispose of gloves and perform hand hygiene. Perform hand hygiene and apply new gloves before cleaning equipment

25. Lay buffing machine handle down on the floor. Remove buffing pad and base plate. Ensure that brushware is removed from the machine after use and before storing machine away. For rotary models, remove the buffing pad from the machine and check for wear or damage

26. Wipe underneath buffing machine with a damp, not wet, blue cleaning cloth

- Wipe down the machine body and handle with a clean cloth to remove dust or any cleaning product residue
- Clean any hard-to-reach areas using a soft brush
- Ensure the power cord is neatly coiled and stored in a safe place

27. Return to the upright position

28. Clean the pad by shaking off loose debris or gently scrubbing it with a damp cloth and or running it under warm, not hot water. If it is too worn or damaged, replace it. Hang to dry 29. Suction model. Detach the suction hose and empty the collection bag or tank. Dispose of any collected debris properly. Check the suction filters for clogs or debris. Clean or replace filters if necessary. Ensure there is no moisture in the collection bag or tank, as this could cause mould or damage to the machine. 30. Hang base plate on the handle of machine 31. Dispose of the diluted Snapback solution in sluice 32. Remove gloves 33. Perform hand hygiene 34. Check cord for breaks or splits and wind cord up, store safely 35. Before storing the buffer, check that all screws, bolts, and parts are securely tightened. If any parts are loose, tighten them as needed 36. Return buffing machine to allocated storage area / cleaning room. The pad holder and pad must be removed prior to storage. Store the buffer in a dry, cool location, away from any direct heat sources or moisture. Store on a stable surface to avoid damage. 37. Perform hand hygiene 38. Maintain a log of any maintenance performed, such as pad replacement, filter cleaning, and any parts replaced. This helps in future servicing and ensures optimal machine performance	
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Carpet extraction

As required

Equipment:

- Nilfisk ES300 Extraction Cleaner
- Vacuum Cleaner
- Carpet broom / rake
- Tapi Extract
- Wet Floor / Hazard Sign
- Goggles
- Gloves
- Carpet Driers
- Unbelievable Stain Remover

Procedure / Steps	Safety Points
 Training video ES300 Use And Care - YouTube Pre – start checks 1. Perform hand hygiene 2. Remove all obstacles and furniture from the area 3. Vacuum carpet thoroughly. See Vacuuming procedure 4. Empty and return vacuum cleaner to allocated storage area or cleaning room 5. Place wet floor / hazard signs at each end of the area that you will be cleaning 6. Visual inspection. Check for visible damage on the machine's exterior, including cracks, loose parts, or exposed wiring. Inspect the scrub deck and brushes for wear and tear. Ensure that the brushes	 Carpet extraction is to be completed by trained staff ONLY Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture

are securely mounted and free from debris. Check the squeegees for signs of damage, excessive wear, or debris buildup

7. Battery check. Inspect the battery level. Ensure the battery is fully charged before use, or plug in the machine to charge it. Check for battery leaks or corrosion around the terminals. Clean any corrosion using a solution of baking soda and water if needed
8. Water and Cleaning Solution. Fill the solution tank with clean water and the appropriate cleaning detergent. Tapi Extract: 200ml for 10L of water (1:50). Check the recovery tank for cleanliness and ensure there are no obstructions that may cause clogging or improper suction
9. Safety Features. Ensure safety shields and covers are in place and properly secured. Check emergency stop button functionality to ensure it is easily accessible and operational
10. Drive and steering. Test the drive mechanism to ensure smooth movement in forward and reverse. Check steering for functionality, ensuring it turns easily without any resistance or unusual noise
11. Controls and settings. Test all operational controls such as the scrubbing speed, water flow, and brush functions. Ensure the vacuum/suction settings are correct for the current cleaning task.
12. Set the desired cleaning mode according to the floor type and area to be cleaned
13. Perform hand hygiene and apply goggles
14. Perform hand hygiene and apply gloves
15. Pre-treat spots/stains with undiluted Unbelievable Carpet Spot & Stain Remover by spraying onto carpet, paying particular attention to spots and traffic lanes
16. Fill carpet extractor with Extraction Rinse according to label directions and rinse carpet (1:230 – light soil, 1:200 – medium soil, 1:160 heavy soil)
17. Agitate with a carpet rake, extractor brush or shampoo brush
18. Adjust the ergonomic adjustable handle for easy operation – adjusts to the height of the operator



Unbelievable Carpet Spot & Stain Remover



Always refer to SDS prior to using chemicals

Carpet Extraction Cleaners are not to be used on Flotex Carpet



Report any equipment damage or malfunction to a Supervisor immediately. If unsafe to use the equipment do not use. Tag it out.



Tapi Extract

200ml for 10L of water
(1:50)

Safe operation

19. Ensure all pre-start checks are completed. Turn the key switch to the ON position to power up the Nilfisk ES300
20. Adjust the settings for scrub speed, water flow, and brush pressure based on the type of floor and level of dirt. Activate the scrubbing brushes and lower the scrub deck to the cleaning position. Engage the vacuum system to start the recovery of water and dirt
21. Always operate the machine in forward mode to ensure optimal scrub performance. During operation:
 - Operate the machine at a steady pace. Avoid excessive speed, as this can result in uneven cleaning and may damage the floor
 - Avoid sharp turns or sudden stops to prevent tipping or damage to the machine or surrounding area
 - Monitor water levels in the solution tank and recovery tank regularly. Refill or empty as necessary





22. Safe practices:

- Ensure the floor area is free from obstructions such as cords, debris, or obstacles that could cause the machine to stop abruptly or cause damage
- Avoid operating the machine on wet, uneven, or slippery surfaces to reduce the risk of slipping or losing control of the machine
- Use caution when reversing the machine to prevent hitting walls or other objects
- Always be aware of your surroundings and ensure that other workers or personnel are at a safe distance while operating the machine

23. In case of an emergency, press the emergency stop button to immediately halt all machine operations

24. In case of a malfunction or if you notice any unusual noise, vibration, or error codes, stop the machine and report it to maintenance personnel for investigation

Post use checks and maintenance

25. After cleaning, re-set pile by brushing in one direction with a clean stiff synthetic broom or carpet rake

26. Remove and dispose of gloves and perform hand hygiene

27. Perform hand hygiene and apply new gloves before cleaning equipment

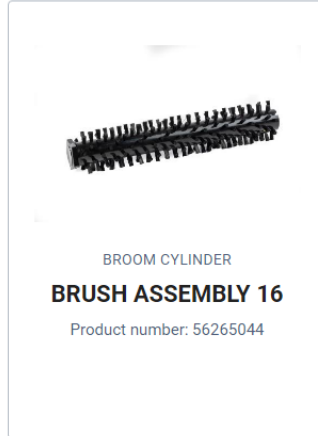
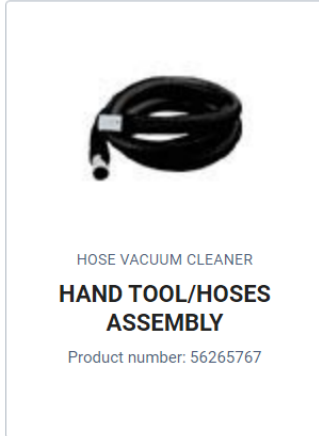
28. Cleaning the machine. Empty the recovery tank by releasing the drain valve and removing any collected dirt or water. Rinse thoroughly to prevent buildup. Clean the scrub deck and brushes with water after each use to remove dirt and detergent residue. Inspect the squeegees for dirt buildup and clean them as necessary to ensure effective water recovery



29. Battery maintenance. Check the battery charge level and ensure the battery is properly connected. Clean battery terminals if necessary, using a dry cloth to prevent corrosion. Charge the battery immediately after use to maintain battery health and prevent premature wear

30. Check brushes and pads. Inspect scrub brushes and pads for wear after each use. Replace worn or damaged brushes and pads promptly to maintain optimal cleaning performance

31. Routine maintenance. Check over equipment and cords for any damage. Tag and report faulty equipment to Supervisor. Check the suction hose for blockages or wear. Inspect the scrub motors and ensure they are functioning correctly. Lubricate moving parts such as wheels and steering mechanisms to ensure smooth operation



32. Store the machine properly. Store the Nilfisk ES300 in a dry, clean area to prevent exposure to moisture or dust that could affect the electrical components. Ensure the machine is turned off and any power cords or batteries are safely stored away. Cover the machine with a protective cover to prevent dirt or damage from external factors

33. Remove and dispose of gloves and perform hand hygiene

34. Remove goggles and perform hand hygiene

35. Set up several carpet dryers having them blow in different directions

36. Remove wet floor/hazard signs

37. Once carpet is completely dry return Carpet Dryers to allocated storage area /cleaning room

38. Perform hand hygiene

39. Maintain a log of any maintenance performed, such as pad replacement, filter cleaning, and any parts replaced. This helps in future servicing and ensures optimal machine performance

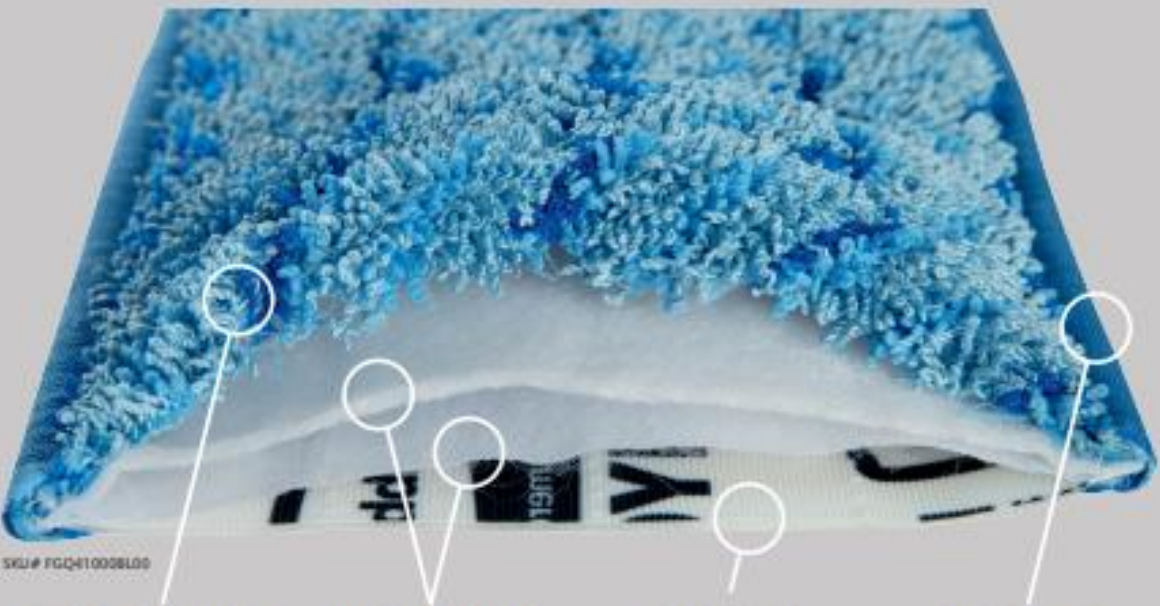
Damp mopping - anatomy of a microfibre mop pad

As required



ANATOMY OF A MICROFIBER MOP PAD

HYGEN™ Microfiber Wet Mop Pads are constructed of premium **split nylon/polyester blend** microfiber that provides **superior wet mopping performance**.



SKU # FGQH1000BL00

FACE MATERIAL	INNER LAYERS	BACKING	EDGE TAPE
- Split microfiber removes 99.9% or more of tested fungi, viruses and bacteria with water alone* - Optimized material construction helps clean effectively while allowing the mop pad to glide across floors with less effort - Bleach resistant to ensure mops can be effectively laundered to help reduce the risk of cross-contamination	Absorbent inner layers help hold enough solution to clean up to 250sq feet	- Industry-leading durable backing secures mop to frame to keep mop pad in place while mopping - Withstands chlorine bleach and extreme heat for superior durability ensuring pad securely attaches to frame throughout the product's life	Durable edge helps retain mop shape with use over time, 200 launderings with bleach and 500 launderings without bleach



*Based on third-party testing on VCT surface with water only | EPA Reg No 91025 WMA-001
 Tested fungi: *Candida auris* (C. auris)
 Tested viruses: *Poliovirus* (PV), SARS-CoV-2 Omicron variant
 Tested bacteria: *Clostridium difficile* (C. diff), *Moraxella morganii*, *Staphylococcus aureus* (MRSA), *Pseudomonas aeruginosa*

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Damp mopping - preparing the charging bucket

Daily

Equipment:

- Asset JF
- Charging bucket (grey)
- Mop pads (blue)
- Gloves
- Measuring jug

Procedure / Steps	Safety Points
  Note: floors that are very scuffed need a scrubber, not Damp mopping. Perform hand hygiene and apply gloves  1. PLACE x5 PADS Place x5 pads in charging bucket with Velcro /HYGEN brand facing up.  2. ADD SOLUTION Using measuring Jug, pour 800 - 900ml of diluted chemical solution SLOWLY and EVENLY over the top of the pads. *Mop pads must be damp NOT wet*  3. PLACE x5 PADS Place another x5 pads in charging bucket with velcro/ HYGEN brand logo facing up. Add another 800 - 900ml of diluted chemical solution pouring SLOWLY and EVENLY over the top pad.  4. ABSORB Allow approx. 10-15 minutes for the diluted chemical solution to absorb evenly into the launderable microfibre pads before use.	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture   Asset JF

5. Remove gloves and dispose

6. Perform hand hygiene

For noting: Always prepare 5 pads at a time only. i.e.

- Place 5 pads then pour 800-900mL
- Place 5 pads then pour 800-900mL
- Place 5 pads then pour 800-900mL

...and so on

See [Microfibre ratios](#)



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Damp mopping

As required

Equipment:

- Wet floor / hazard signs
- 'Charged' mop pads (blue)
- Charging bucket (grey)
- Quick connect mop handle and frame

Procedure / Steps	Safety Points
Note: floors that are very scuffed need a scrubber, not Damp mopping. Place wet floor / hazard signs in highly visible locations, in view of oncoming traffic when mopping.  1. Attach frame to mop lightly • Centre Quick-Connect frame over the charging bucket and push down to attach the mop via the velcro connection 	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 

2. Mopping

- Begin mopping at the far corner of the room, mopping the floor using overlapping figure 8 strokes, while moving backwards toward the doorway/exit



While mopping:

- Maintain an upright posture by adjusting the height of the mop handle a couple of inches below chin level
- Be mindful not to press down too hard on the mop
- Clean long stretches by walking behind cleaning head
- Spot cleaning occasional resistant mark - place foot on mop to add some pressure and use small forward/backward motion to remove the mark. Don't over balance. If unable to remove the mark, contact PSS Supervisor. Consider whether the floor needs scrubbing and buffing
- Corridor: When damp mopping corridor, mop one half at a time to allow for traffic When it is dry move your hazard signs and do the other half
- Mop pads are to be replaced once they start drying out and resistance can be felt when mopping



Mop in a small figure 8 motion to avoid twisting

Keep your back straight

Don't mop for extended, continuous periods – avoid repetitive work

Mop pads are not to be dripping wet

Do not place mop in bucket when filling or lifting

Corridor: When damp mopping corridor, mop one half at a time to allow for traffic When it is dry move your hazard signs and do the other half

- Mop pads are to be replaced before and after cleaning bathrooms

3. Remove soiled mop

- Remove soiled mop by placing one foot on one side of the exposed edge and the other foot on the other end of the exposed mop pad



4. Lift mop

- Lift the soiled mop, using the Quick-Connect frame, positioned at a 90 degree angle to the mop. Pull handle straight up and the mop pad will peel away from the frame base. Pull the soiled mop from the frame and place into bag



5. Perform hand hygiene



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Place wet floor / hazard signs in highly visible locations when mopping

Once floor is dry remove wet floor / hazard signs and return to storage area

Launderable (blue) mop pads and (blue) cleaning cloths placed in clear plastic bag. Clear plastic bag placed in blue linen bag. Send for laundering at the end of each shift.

Hand Hygiene: Ensure hand hygiene is performed after touching the patient's environment and in between patient zones

Change mop pads frequently: Each mop pad will clean approximately 25 square meters

Corridor: When Damp mopping corridor, mop one half at a time to allow for foot traffic. When it is dry, move your hazard signs and do the other half.

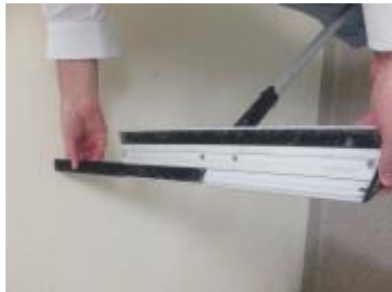
Furniture: Move furniture and mop underneath. Replace when dry.

Showers and Toilets: Mop floor using grey charging bucket, red mop handle & blue mop pads

Clean the charging bucket daily, and or when all mops in charging bucket have been used:

1. Perform hand hygiene
2. Apply gloves
3. Dispose of cleaning solution in sluice
4. Once finished, dispose of mop pads appropriately. Blue are launderable, yellow disposed
5. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
6. Empty and clean buckets with Asset JF. Turn upside down to dry.
7. Remove and dispose of gloves

8. Perform hand hygiene
9. Wipe down mop handle
10. Inspect the mop head. The velcro strips can be replaced. Simply unscrews. Discuss replacement of velcro strips with your Supervisor.



11. Return equipment to cleaner's cupboard
12. Perform hand hygiene
13. Once floor is dry remove wet floor / hazard signs and return to storage area
14. Perform hand hygiene



Static sweeping

Daily

Equipment:

- Static mop
- Static cloth
- Dust pan & brush

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Attach a new clean static cloth to static mop head 3. Inspect the floor to ensure it is free of spills 4. Sweep down edges first. At the end of a corridor, turn using a “U” turn 5. Use the mop in an “S” motion to cover entire floor surface 6. Work way up and down the corridor or room until area is entirely swept	If there is liquid on the floor wipe up before static mopping Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture

7. Ensure the sweeping edge of the mop is in contact with the floor
8. Clean long stretches by walking behind cleaning head
9. Sweep all rubbish and debris into a central point
10. Pick up rubbish and debris with dust pan and brush
11. Remove and dispose of static cloth
12. Perform hand hygiene before cleaning static mop
13. Wipe clean static mop
14. Return equipment to storage area
15. Perform hand hygiene



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Strip and seal of vinyl floor

As required

Equipment:

- Gloves
- Auto scrubber
- Vectra
- Safety Glasses
- Wet & Dry Vacuum
- 4.8L bucket (blue)
- Coveralls
- Strip Eze
- Wet floor / Hazard signs

Procedure / Steps	Safety Points
STRIPPING 1. Remove all obstacles and furniture from the area 2. Sweep the floor to remove all loose debris. See Static sweeping procedure 3. Place wet floor / hazard signs at each end of the area that you are stripping 4. Perform hand hygiene 5. Apply gloves, coveralls and safety glasses 6. Dilute 1 part STRIPEZE with 2-6 parts of cool water (depending on build up) in a mop bucket 7. Apply liberal amounts of diluted solution to the surface to be stripped 8. Let stripper solution soak in for 5-7 minutes ensuring it doesn't dry 9. Agitate surface thoroughly using auto scrubber (preferred) or mop using a stripping pad or brush to break up film. See Auto scrubbing (floor scrubbing) or Damp mopping procedures 10. Manually strip areas that cannot be reached with the machine 11. Pick up all of the solution with a clean mop or wet and dry vacuum. Trail mop as needed if stripper residues are present 12. Rinse floor with clean water 13. If stripper residues are still present clean floor using mop and bucket or auto scrubber. See Auto scrubbing (floor scrubbing) or Damp mopping procedures 14. Let floor dry completely 15. Inspect floor for any areas requiring further stripping 16. Remove all PPE and perform hand hygiene	Strip & sealing of floors to be completed by trained staff ONLY Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture  

17. Perform hand hygiene and apply new gloves before cleaning all equipment
18. Clean all equipment and return to allocated storage area /cleaning room
19. Remove gloves and perform hand hygiene



SEALING

1. Apply one coat of VECTRA using a clean mop
2. Allow coat to dry for 20-30 minutes depending on drying conditions
3. Repeat process 3 to 4 times each time opposite to previous coat
4. Perform hand hygiene
5. When the floor is completely dry remove wet floor / hazard signs
6. Perform hand hygiene



Strip Eze

Always refer to SDS
prior to using chemicals



Vectra





Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Vacuuming

As required

Equipment:

- Pull behind vacuum cleaner or backpack vacuum cleaner

Procedure / Steps	Safety Points
Note: Ensure you check the filter daily and change when wear and tear becomes apparent. 1. Perform hand hygiene 2. Unwind vacuum cleaner cord 3. Check cord for breaks, splits and damage. If the cord is faulty or damaged report it to your supervisor immediately 4. Check vacuum bag and replace if required 5. Plug vacuum cleaner into power point 6. Adjust height of wand to allow to rest against hip of handler 7. Hold vacuum hose lightly behind or around lower back 8. Move the vacuum head in a forwards backwards motion. Ensure you overlap strokes, so the entire floor is covered: • Hold vacuum handle at about hip height • Use forward lunging actions to push sweep over carpet floor • Lean forward with whole bodyweight if firm pressure is required when vacuuming • Shift bodyweight between feet and maintain arms by the side • Adopt staggered bench or knights for vacuuming under surfaces and support on knee  	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture  Check cord for breaks, splits and damage. If the cord is faulty or damaged report it to your supervisor immediately Remove plug from the power point by the plug and not by the cord



9. Vacuum from farthest point to closest point
10. For corners and edges remove vacuum cleaner head and use nozzle
11. Move mobile furniture and vacuum underneath then replace furniture
12. Remove and clean vacuum head to prevent clogging and dirt from building up in the bristles
13. Unplug vacuum cleaner
14. Check vacuum bag and replace if required
15. Check cord and equipment for breaks, splits or damage.
16. Wind cord and store safely and return vacuum cleaner to storeroom
17. Perform hand hygiene
18. Report any torn carpet or stains to PSS Supervisor



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

PATIENT ROOM

Key safety principles:

When cleaning in a patient's hospital room, it's crucial to use proper body mechanics to avoid musculoskeletal injuries. Start by positioning yourself close to the surfaces you're cleaning, whether it's the bed, furniture, or floors. Avoid twisting your body or reaching excessively; instead, bend your knees and use your legs to squat down or lift cleaning equipment. When making the bed or moving heavy items, bend at the knees and keep your back straight to reduce strain on your spine. If you need to reach high areas, use a step stool rather than stretching, which can strain your back and shoulders. Always maintain a neutral spine and avoid bending from the waist, as this can lead to back and neck pain over time.

In addition to proper posture, ensure you use ergonomic tools that reduce strain during cleaning tasks. For example, long-handled cleaning tools, such as mops, dusters, and brushes, can help prevent bending or awkward reaching. When cleaning around the bed or other furniture, slide rather than lift heavy items whenever possible to minimise the load on your muscles. Take regular breaks to stretch and change your position to avoid repetitive motions that can lead to fatigue and injury. By maintaining good body mechanics, using ergonomic tools, and taking care of your body with breaks and stretches, you can help prevent musculoskeletal injuries while cleaning in a hospital setting.



Basin – hand

Daily & when visibly soiled

Equipment:

- Gloves
- Cleaning cloth (red)
- Paper towels and key
- Go Getter
- Hazard / wet floor signs
- Liquid soap dispenser refill
- Scouring pad
- 4.8L bucket (red)

Procedure / Steps	Safety Points
<i>When cleaning bathroom areas work from cleaner areas toward more dirty areas - mirror, then shelf, then paper towel dispenser then soap dispenser then hand basin.</i> 1. Perform hand hygiene 2. Apply gloves 3. Damp wipe over mirror then shelf, then paper towel dispenser – use a paper towel to dry. See Glass and mirror procedure 4. Wipe soap dispenser (including the hand dispenser lever) 5. Using cleaning solution Cream R7 and a damp, not wet, red cleaning cloth, wipe over edges of the basin 6. See Microfibre cloths 8 sided fold method and Damp dusting procedures 7. Clean the basin surrounds and the taps (including pipes and underneath the basin)   8. Clean the sides and then the base of basin, working towards the plug hole 9. If required use scourer to remove dirt or calcium build up from the base of the basin and around the plug hole.	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 

10. Using clean cold water rinse all surfaces to remove residue from cleaning solution
11. Turn on the tap/s and let water run for one minute
12. Turn off the tap/s
13. With a dry red cleaning cloth wipe over all surfaces to remove water build-up / marks
14. Check the floor for any water spills, mop if necessary. See [Damp mopping](#) procedure
15. Once finished, dispose of scourer. Place red cleaning cloths in laundering bag
16. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
17. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry
18. Remove gloves
19. Perform hand hygiene
20. Open paper towel dispenser with key and restock if necessary. Push closed for automatic locking of the dispenser
21. Open soap dispenser check level – replace if necessary and check the dispenser is working correctly by priming the hand lever
22. Return cleaning equipment to cupboard
23. Perform hand hygiene



Go Getter



When removing water jugs and cups etc. fluids are not be disposed of in patients sink

Cleaning solutions are not to be disposed of in a patient's sink

Dusting (Dolly) mops are not be washed in a patient's sink

Exchange gloves and perform hand hygiene in-between cloths



Daily running of taps

To remove contaminants, and ensure the cleanest water possible for our patients, all taps must be run for one minute

Single handle taps run for one minute, handle in the centre



Double handle taps, run each side cold and hot, for one minute each



Although it may seem like a waste of water, it is important that all pipes have clean water running through them every day

Please see your Supervisor with any questions.

Curtains (bed screens)

Refer to exchange frequency below

Equipment:

- Linen skip
- Platform Safety Step Ladder
- Waste Bag (disposable curtains)
- Clean curtains (linen or anti-microbial / disposable)

Procedure / Steps	Safety Points																																																							
<u>Do not change all the screens in succession, alternate your workload. If unsure talk to your shift coordinator or PSS Supervisor.</u> Refer to and sign Curtain Exchange Register (usually located in the dirty utility room) <table><tr><td></td><td>08/08/2022 - 14/08/2022</td><td>05/09/2022 - 11/09/2022</td><td>03/10/2022 - 09/10/2022</td><td>31/10/2022 - 06/11/2022</td></tr><tr><td>ROOM A</td><td></td><td></td><td></td><td></td></tr><tr><td>ROOM B (4)</td><td></td><td></td><td></td><td></td></tr><tr><td>ROOM C</td><td></td><td></td><td></td><td></td></tr><tr><td>ROOM D</td><td></td><td></td><td></td><td></td></tr><tr><td></td><td>15/08/2022 - 21/08/2022</td><td>12/09/2022 - 18/09/2022</td><td>10/10/2022 - 16/10/2022</td><td>07/11/2022 - 13/11/2022</td></tr><tr><td>ROOM E (4)</td><td></td><td></td><td></td><td></td></tr><tr><td>ROOM F</td><td></td><td></td><td></td><td></td></tr><tr><td>ROOM G</td><td></td><td></td><td></td><td></td></tr><tr><td>ROOM H</td><td></td><td></td><td></td><td></td></tr><tr><td></td><td>22/08/2022 - 28/08/2022</td><td>19/09/2022 - 25/09/2022</td><td>17/10/2022 - 23/10/2022</td><td>14/11/2022 - 20/11/2022</td></tr></table> <u>Exchange Frequency</u> All curtains, whether linen or disposable, must be exchanged: • When visibly soiled, frayed, torn or damaged in any way • Following cleaning of a bedspace in an inpatient ward occupied by a patient who was under isolation or at • Following discharge/transfer of a patient who is under isolation • During an intensive (outbreak) ward clean <u>Procedure</u> 1. Use a step ladder to remove curtains. See Ladders – Safety Alert		08/08/2022 - 14/08/2022	05/09/2022 - 11/09/2022	03/10/2022 - 09/10/2022	31/10/2022 - 06/11/2022	ROOM A					ROOM B (4)					ROOM C					ROOM D						15/08/2022 - 21/08/2022	12/09/2022 - 18/09/2022	10/10/2022 - 16/10/2022	07/11/2022 - 13/11/2022	ROOM E (4)					ROOM F					ROOM G					ROOM H						22/08/2022 - 28/08/2022	19/09/2022 - 25/09/2022	17/10/2022 - 23/10/2022	14/11/2022 - 20/11/2022	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture Check platform safety step ladder is stable and safe before use Ensure 3 points of contact at all times 
	08/08/2022 - 14/08/2022	05/09/2022 - 11/09/2022	03/10/2022 - 09/10/2022	31/10/2022 - 06/11/2022																																																				
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2. To move the step ladder push the ladder holding the front side rail to the desired location. Don't lift safety step ladder.
3. As you approach the safety step ladder complete a safety scan. Check it is fully locked in place
4. Collect all equipment required to where task is to be performed
5. As you enter the ladder, ensure 3 points of contact at all times



6. Place linen skip under the curtain to be removed and lower used/dirty linen curtains directly into linen skip while unhooking the curtain
7. Place linen skip with waste bag inside under the curtain to be removed and lower disposable curtains into waste bag while unhooking the curtain
8. Support linen curtains by hanging over the ladder bar while hanging new curtains
9. Hand hygiene should be performed after handling used curtains, prior to handling new curtains
10. Perform hand hygiene
11. Hang new curtains after room has been cleaned
12. Overlap curtains by 4 hooks to ensure privacy
13. Disposable Curtains – write today's date in *CURTAINS FITTED* and the next schedule change date (refer below) in *CURTAINS CHANGE DATE*
14. Wheel linen skip with bag to the central collection area

Change linen bag as required or when maximum $\frac{3}{4}$ full to minimise the weight of bag and risk of a manual handling injury

Change waste bag as required to minimise the weight of bag and prevent back and shoulder injuries

15. Remove bag from linen skip and double tie drawstring
16. Place linen bag on linen trolley
17. Return linen skip to ward area
18. Tie a clean linen bag to the linen skip and leave in designated linen collection area on the ward (if unsure ask your Nurse Coordinator)
19. Perform hand hygiene
20. For Linen curtains: Sign Curtain Exchange Register (usually located in the dirty utility room)

Anti-microbial / Disposable Curtains

Disposable curtains **must be dated upon installation** and changed when:

- Visibly soiled or damaged
- Following cleaning of a bedspace in an inpatient ward occupied by a patient who was under isolation, or at discharge/transfer of a patient who is under isolation
- During an intensive (outbreak) ward clean
- If the surface of the curtain is no longer smooth i.e. frayed
- As per manufacturer's recommendations if criteria 1-4 do not apply (refer to Nurse Unit Manager for exchange frequency in order to date curtains correctly)

Each Nurse Unit Manager must implement a process to ensure these anti-microbial / disposable curtains are used in accordance with the recommendations

Ladders - Safety Alert



Government of Western Australia
East Metropolitan Health Service

Safety Alert

Issued by:
EMHS WHS

Safety Alert 0052
July 2024

Step Ladders and Step Stools

What is happening?

- The domestic rated step ladders and step stools have been deemed unsuitable to use.
- 'Industrial' rated ladders and step stools, are to be supplied by departments instead.
 - An industrial rated ladder that should be used in the workplace, is classified as 'industrial grade' and has a sturdier build. It has a safe working load rating of 120kg (minimum).
 - An industrial rated step stool is a stable and wider stool, that has anti-slip tape reducing the risk of unwanted movements when in use. It has a load capacity of 150kg maximum.
- It's recommended that departments should choose a ladder and step stool that offers the highest level of protection, to ensure the safety of its workers.



Domestic Rated Step Ladder



Industrial Rated Step Ladder-
Bailey 150KG 3 Step Platform
Ladder 0.9m



Domestic Rated Step Stool



Industrial Rated Step Stool

Safety Labels

All ladders should contain the following information:

- Name of Australian manufacturer,
- The working length (metres),
- Compliance with AS/NZS 1892.1:2018,
- Duty rating (industrial),
- The statement 'This ladder is intended to carry a maximum load of ___kg'.
- The statement: 'MISUSE OF A LADDER MAY RESULT IN A FALL, SERIOUS INJURY, PARALYSIS OR DEATH'.



Recommendations

- Staff are reminded to follow safe work practices.
- Choose a ladder or step stool that allows for easy reach, three points of contact, appropriate heights and proper stability.
- Consider the work environment and task being undertaken, number of steps required, storage, heights of workers, and avoiding overreaching and awkward postures.

Further Information

- Speak to your Manager, WHS Department, WHS Committee or your Health and Safety Rep.
- Review [SafeWork Australia: Risk of Falls in the Workplace](#) and [WorkSafe Vic: Using Portable Ladders](#)

Approved: Michelle Anderson, Director WHS, EMHS WHS Department

Damp dusting

Daily

Equipment:

- Extendable Dusting (Dolly) mop
- Cleaning cloth (blue)
- 4.8L bucket (blue)
- Dusting mop
- Asset JF
- Gloves

Procedure / Steps	Safety Points
    Use extendable dusting mop for high and hard to reach surfaces Consider use of Platform Safety Step Ladder – See Ladders – Safety Alert Use cleaning cloth (blue) for all other surfaces Inspect dusting mop head is suitable for use. If visibly soiled, frayed or damaged dispose and replace with new dusting mop head 1. Perform hand hygiene 2. Apply gloves	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture Use a platform safety step ladder for hard to reach and high surfaces  

Dusting mop

3. Half fill bucket from Asset JF dispenser
4. Wet dusting mop and gently squeeze until only damp (almost dry), and no water dripping
5. The dusting mop head must be in contact with surfaces at all times
6. Rinse and gently squeeze out frequently, to remove dirt / dust
7. Wash dusting mop head with Asset JF and rinse after each bed unit area.
8. Inspect dusting mop head is suitable for use.
9. If visibly soiled, frayed or damaged dispose and replace with new dusting mop head.
 - **Dusting (Dolly) mops are not to be washed or wrung out in a patient's sink**
10. In non-isolation rooms, dusting mop head can be rinsed for use up to a maximum of a four bed room
11. In isolation rooms, dispose of dusting mop head and replace with new

Cleaning cloth

4. See [Microfibre cloth preparation](#) procedure and [Microfibre cloth 8 sided fold method](#)
5. With a damp, not wet, blue cleaning cloth, wipe over all surfaces
 - **Dispose of cleaning cloth between bed spaces.**
 - **Once the cloth has been used to wipe surfaces, do not return the cloth into cleaning solution; otherwise the cleaning solution will need to be changed prior to cleaning the next patient bed area. i.e. don't double dip**
6. Dispose of cleaning cloths appropriately. Blue and red cloths are launderable, yellow disposed
7. Remove gloves and perform hand hygiene in between cleaning cloths.
8. Once finished, dispose of cleaning cloths appropriately. Blue and red cloths are launderable, yellow disposed
9. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets

Use long adjustable dusting mop handle for high surfaces to avoid stretching and twisting

Work from clean to dirty and from high to low



Asset JF



Exchange gloves and perform hand hygiene in-between cloths

Dusting (Dolly) mops are not to be washed in a patient's sink

10. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry. 11. Remove and dispose of gloves 12. Perform hand hygiene <i>All surfaces including non-medical equipment and appliances (e.g. Televisions) must be dusted daily. See Television procedure</i>	
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Discharge/transfer

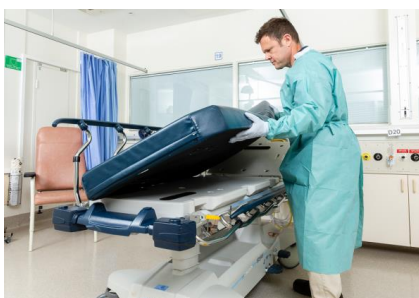
When a patient is discharged/transferred

Equipment:

- Gloves
- Wet floor / hazard signs
- 70% Alcohol Wipes
- Asset JF for mopping
- Mop pad (blue)
- Sodium Hypochlorite 0.125%
- Cleaning cloth (blue)
- Charge bucket (grey)
- 4.8L bucket (blue)

Procedure / Steps	Safety Points
All discharge cleans are completed with Sodium Hypochlorite 0.125% 1. Ask nursing staff to empty medication drawer and patient locker 2. Place wet floor / hazard signs in highly visible locations 3. Perform hand hygiene 4. Apply gloves 5. Carry out high and low damp dusting using Sodium Hypochlorite 0.125. See Microfibre cloths 8 sided fold method and Damp dusting procedures 6. Move bed out from wall and damp wipe the wall and fittings, using Sodium Hypochlorite 0.125%. See Wall procedure 7. Check for any unsealed pillows or torn mattresses (including the inside) If pillow is torn or soiled: Place in plastic bag, seal and discard to rubbish. Inform Nurse Unit Manager to order replacement Inspect inside the mattress cover. If mattress is torn or soiled: Remove it immediately and advise the Nurse Unit Manager/shift coordinator to arrange a replacement 8. If visibly dirty, remove curtains from the bed area. Linen curtains are placed in a linen bag. Antimicrobial curtains are placed in the waste. See Curtains (bed screens) procedure 9. Wipe patient remote control handsets and telephone with 70% Alcohol Wipes	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 

10. See [Microfibre cloth preparation](#) procedure and [Microfibre cloth 8 sided fold method](#) to prepare blue cleaning cloth with Sodium Hypochlorite 0.125%
11. With a damp, not wet, blue cleaning cloth, wipe bedside furniture including the inside of the bedside locker, wardrobe, patient bed table, hand basin, soap dispenser, paper towel dispenser and oxygen and suction outlets
12. Suction canister can be wiped. Only dispose if used
13. Blood pressure cuffs:
 - the majority used in the hospital are single patient use. They are labelled with the patient sticker and are disposed of when visibly soiled or on discharge
 - Re-useable cuffs are cleaned with multipurpose cleaner or detergent wipes and dried after each patient use. They are discarded if the Velcro becomes soiled with body fluids, hair etc.
14. Clean the bed using Sodium Hypochlorite 0.125%
 - a) Raise the trolley/bed to waist height to avoid stooping or stretching
 - b) Remove and wipe pillows and place on a clean surface
 - c) Wipe the top surface of the mattress. Use a forward backward lunge, flip mattress onto raised rails on opposite side. This exposes the bottom side up and allows what was the top side (now resting on the rails) to dry



- d) With the underside now facing up to the ceiling, wipe the underside of the mattress allow mattress to dry before placing back to normal position. The area between the mattress and the base of the bed may need to be dried by hand with a cloth
- e) Wipe bed rails
- f) Wipe bed frame



Sodium Hypochlorite
0.125%



70% Alcohol Wipes



Asset JF



Exchange gloves and perform hand hygiene in-between cloths

- g) Extend the foot of the bed and wipe all exposed surfaces including underneath of bed and bed wheels

Check that the trolley/bed is in good working order. If faulty, ensure faulty equipment is always cleaned, removed from service and tagged with an yellow “do not use” tag. See [Safety checklist for bed Inspection SOP](#)

15. Damp mop patient’s room with Asset JF, not Sodium Hypochlorite 0.125%. See [Damp mopping](#) procedure
16. If there is an ensuite, clean the shower recess and toilet. See [Shower recess](#) and [Toilet](#) procedures
17. Dispose of cleaning cloth after use and send mop pad for laundering. Launderable (blue) mop pads and (blue) cleaning cloths placed in clear plastic bag
18. Clear plastic bag placed in blue linen bag. Send for laundering at the end of each shift. Blue and red cloths are launderable, yellow disposed
19. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
20. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry
21. Remove and dispose of gloves
22. Perform hand hygiene
23. Remove wet floor / hazard signs when floor is dry
24. Perform hand hygiene

NOTE:

This procedure is generally, completed by:

- PCAs in patient rooms that were not isolation rooms
- Cleaners in patient rooms that were isolation rooms

Patient’s basin:



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

• When removing water jugs and cups etc. fluids can be disposed in patient's basin • Cleaning solutions are not to be disposed of in a patient's basin • Dusting (Dolly) mops are not wrung out or washed in a patient's basin PCAs strip and remake beds. Nursing can assist: • Ensure all areas are dry before remaking the bed • Generally, cleaners do not strip and remake beds Ensure antimicrobial hand gel is placed in bracket at the foot end of the bed	
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Door Daily

Equipment:

- Gloves
- Cleaning cloth (blue)
- 4.8L bucket (blue)
- Platform Safety Step Ladder
- Asset JF
- Extendable dolly mop

Procedure / Steps	Safety Points
Shorter stature staff members to consider use of Platform Safety Step Ladder – See Ladders – Safety Alert 1. Perform hand hygiene 2. Apply gloves 3. See Microfibre cloths 8 sided fold method, Damp dusting and Glass and mirror procedures 4. With a damp, not wet, blue cleaning cloth, wipe over all surfaces of the door, door frame, handles and glass panel 5. Once finished, dispose of cleaning cloths appropriately. Blue and red cloths are launderable, yellow disposed 6. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets 7. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry 8. Remove and dispose of gloves	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture

9. Perform hand hygiene



Asset JF



Exchange gloves and
perform hand hygiene
in-between cloths

High Touch Points

Daily

Equipment:

- Gloves
- Cleaning cloth (blue)
- Asset JF
- 4.8L Bucket (blue)

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply gloves 3. See Microfibre cloths 8 sided fold method and Damp dusting procedures 4. With a damp, not wet, blue cleaning cloth, wipe over all surfaces • Dispose of cleaning cloth between bed spaces • Once the cloth has been used to wipe surfaces, do not return the cloth into cleaning solution; otherwise the cleaning solution will need to be changed prior to cleaning the next patient bed area. i.e. don't double dip 5. Ensure all high touch points are wiped at least once, every 24 hours PCAs will wipe: • Call bell/button	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture

- Bed remote
- Bed rail
- Bed ends
- Patient tray table
- Bedside locker
- Chair
- Television remote
- Telephone
- Overhead examination light
- Suction canister

Cleaners will wipe:

- Basin
- Door handles
- Light switches
- Paper towel holder
- Toilet tissue dispenser – pay attention to underside
- Shelving
- Soap dispenser
- Toilet
- Shower taps and head
- Corridor hand rails
- Patient waiting area

6. Once finished, dispose of cleaning cloths appropriately.
7. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
8. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry
9. Remove and dispose of gloves
10. Perform hand hygiene



Asset JF



Exchange gloves and perform hand hygiene in-between cloths

Intensive ward cleaning & disinfection

As required

Equipment:

- Safety Glasses
- N95/P2 Particulate Respirator
- Mop pads (yellow)
- Wet floor / hazard sign
- 70% Alcohol Wipes
- Convacare adhesive remover
- Gown
- Asset JF
- Sodium Hypochlorite 0.125%
- Cleaning cloths (yellow)
- Step ladder
- Gloves
- 4.8L bucket (yellow)
- Charge bucket (yellow)
- Mop pad (yellow)
- Plastic Bag

Procedure / Steps	Safety Points
This procedure is completed by both PCAs and Cleaners and MUST be done in the following sequence PREPARATION BEFORE CLEANING 1. Remove all curtains (linen and disposable) from the bed area to be cleaned and place in linen carrier or rubbish bin as appropriate. Remove all shower curtains. Alternate the staff who remove (and replace) curtains to minimise repetitive strain. See Curtains (bed screens) procedure 2. Remove all rubbish. See Rubbish procedure 3. Check for any unsealed pillows or torn mattresses (including the inside) If pillow is torn - • Place in plastic bag, seal and discard to rubbish. • Inform Nurse Unit Manager to order replacement If mattress is torn - • Remove it immediately and advise the Nurse Unit Manager/shift coordinator to arrange a replacement	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture   

4. Remove wall posters, sticky tape etc from all surfaces of walls and furniture with Convacare adhesive remover wipes

Liaise with Nurse Unit Manager shift coordinator as to whether the patients can sit out of bed outside the room as this will make cleaning easier:

- If the patient can sit out of bed, strip the bed linen into the linen skip. See [Linen](#) procedure
- If a patient cannot be moved from the bed, then clean as much of the bed frame as possible

CLEANING PROCEDURE

1st STEP OF 2 STEP CLEAN PROCESS – use Asset JF

2nd STEP OF 2 STEP CLEAN PROCESS – use sodium hypochlorite 0.125%, unless otherwise instructed.

Important points to note:

- Replace the cleaning solution when the liquid looks dirty and between patient bed areas.
- Rinse and replace cleaning cloths regularly
- After completing each step of the cleaning process, empty buckets into sluice (preferred) or toilet (then flush) and thoroughly wash out mop, dusting mop and buckets with Asset JF, then rinse with water
- Apply PPE, change PPE and perform hand hygiene in between patient bed areas and between the first and second step cleaning process.
- Patient Care Areas to be covered in the clean include:
 - Bed frame and mattress. See [Discharge/transfer](#) procedure
 - Inside and outside of the locker, telephone, patient handset
 - Television (1st step detergent clean only – do not use sodium hypochlorite to clean the television. See [Television](#) procedure
 - Over bed table



**1st STEP OF
2 STEP CLEAN
PROCESS –
use Asset JF**



- Oxygen and suction points
- Outside surface of the suction container
- Suction canister can be wiped. Only dispose if used
- Inside and outside of the wardrobe (if applicable)
- Shelves beside and above the bed (if applicable) See [Countertop, benchtop and desk](#) procedure
- Overhead wall light (if applicable)
- Any other patient equipment in the room. See [Patient equipment - hoist, dressing trolley, IV poles etc](#) procedure

Patients' Belongings

- Wipe down patient's belongings where possible
- Place in a plastic bag labelled with the patient's name
- Return to the locker once all cleaning in the room has been completed

Common Patient Areas/Bathroom/Showers/Toilets

- Screen tracks
- Hand basin/s. See [Basin – hand](#) procedure
- Outside and underside of the paper towel and soap dispensers
- Both sides of the window blinds
- Shower. See [Shower recess](#) procedure
- Toilets. See [Toilet](#) procedure:
 - Clean outside and underside of toilet paper dispenser
 - Use a separate 'daily' cloth to clean toilet area
 - Clean cistern and flush buttons with daily cloth and discard
 - Clean toilet lid and seat with daily cloth and discard
 - Clean outside of toilet bowl with daily cloth and discard
 - Clean inside of toilet bowl with toilet brush
 - If the toilet brush looks 'shaggy', discard and replace
- Damp mop the floor last. See [Damp mopping](#) procedure

Sodium hypochlorite
0.125%,

2nd STEP CLEAN
PROCESS use sodium
hypochlorite 0.125%,
unless otherwise
instructed.



Convacare Adhesive
Remover Wipes



70% Alcohol Wipe

Exchange gloves and
perform hand hygiene
in-between cloths

Equipment

5. Blood pressure cuffs:

- the majority used the in hospital are single patient use. They are labelled with the patient sticker and are disposed of when visibly soiled or on discharge
- Re-useable cuffs are cleaned with multipurpose cleaner or detergent wipes and dried after each patient use. They are discarded if the Velcro becomes soiled with body fluids, hair etc.
- Equipment in the corridor, or store areas (including patient and visitor chairs, IV poles and commodes). See [Patient equipment - hoist, dressing trolley, IV poles etc](#) procedure
- Damp mop the corridor floor last. See [Damp mopping](#) procedure

Pan Room

- Wipe over all linen skip frames. See [Linen removal](#) procedure
- Place all pans and urinary bottles through the pan sanitiser.
- Place all washbowls, jugs, kidney dishes into the utensil washer
- Wipe the external surfaces of the pan sanitiser and utensil washer
- Clean the trough and hand basin. See [Basin – hand](#) procedure
- Clean and flush the sluice
- Damp mop the floor last. See [Damp mopping](#) procedure

Storage Cupboards/Lotion Rooms/ Etc:

- Thoroughly clean the shelving / benches, all items and equipment in the above areas. See [Storeroom](#) procedure
- Remove items from storage containers and wipe the inside of the storage box including the item. (DO NOT wipe paper products). Return all items to clean containers
- Damp mop the floor last. See [Damp mopping](#) procedure



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Drug Cupboards: This is the responsibility of Nursing Staff

- Clean shelving inside of cupboards with detergent. Where possible wipe bottles with ProMed 70% Alcohol Wipes. Do not wipe any medication in cardboard boxes. Return items to clean containers

Reception Area, Nursing Station, Offices

- See [Countertop, benchtop and desk](#) procedure
- Do not discard documents, forms. Remove documents and forms from shelving and clean shelves with Asset JF. Return documents and forms to shelving.
- Clean telephone, desk area, chairs. Remove any chair which has torn padding and inform the Area Nurse Unit Manager / shift coordinator to arrange replacement
- Wipe computer screens/keyboards with ProMed 70% Alcohol Wipes
- Damp mop the floor last. See [Damp mopping](#) procedure

COMPLETION OF CLEAN

1. Two hourly check done by PSS Supervisors while cleaning is being carried out
2. Once the whole ward has been cleaned inform the Ward/Nurse Unit Manager
3. It is the responsibility of the PSS Supervisor to ensure any issues are to be actioned within the allocated timeframe and re-inspected as required. See [Intensive ward cleaning & disinfection protocol](#)

Intensive ward cleaning & disinfection protocol



Government of Western Australia
East Metropolitan Health Service



Patient Support Services

Intensive Clean Protocol

Notification is given by Infection Prevention & Management Unit (IPMU) to PSS Assistant Managers and PSS Manager regarding the requirement for an intensive clean, and the method of cleaning to be used.

PSS Assistant Manager(s) to notify all relevant Heads of Departments, PSS Supervisors, PSS Administrative Assistants and Linen Services Officer regarding the date & time the Intensive Clean will take place.

After receiving notification by IPMU and PSS Assistant Manager, PSS Supervisors are to liaise with CNS/NUM of the ward to make the necessary arrangements for the Intensive Clean.

PSS Supervisors need to ensure:

- ❑ Cleaning equipment & chemicals are in stock (the ward is to order additional if required)
- ❑ Intensive clean trolley is stocked & prepared the evening prior to the Intensive Clean
- ❑ Organise staffing resources
 - inform Cleaning & PCA staff allocated to the area of the intensive clean
 - arrange additional staffing resources required to carry out the intensive clean, noting cost centre from the notification sent
 - if ongoing increased cleaning is required ensure that daily staffing sheets are updated accordingly and reviewed on a daily basis
- ❑ Allocate a Team Leader to ensure correct cleaning processes are performed and standards maintained. NB: The Team Leader is to remind the cleaning team of appropriate communication protocols with patients at the start of each new intensive clean.
- ❑ Timesheets must be entered clearly with 'Intensive Clean' & Ward area(s) being cleaned
- ❑ Administrative Assistant to ensure correct cost code, wage code HWGA3/4 & footnote "1" is entered in Roster for the cleaning team. Correct wage code for team leader.
- ❑ Ensure cleaners are trained in correct manual handling techniques for beds/stretchers and cleaning of patient equipment
- ❑ **PSS Supervisors are to check every second hour on the Intensive Clean. Completed areas are to be audited with TopCat to ensure acceptable cleaning standards.**
- ❑ PSS Supervisors need to manage the duty of care to chemical-sensitive staff for the duration of the intensive clean.

Method of Cleaning will be advised by IPMU on a case by case basis:

- One-step Clean and Disinfection using combination 0.125% Sodium Hypochlorite and detergent solution; or
- Two-step Clean and Disinfection:
 - 1st step detergent and water
 - 2nd step 0.125% Sodium Hypochlorite
- Principles of Cleaning (refer PSS Cleaners Manual)
- Ensure correct PPE is used
- Appropriate signage used for wet floors etc

PSS Supervisors are to emphasise with staff the importance of maintaining transmission-based precautions & good hand hygiene standards at all times.

Created
Reviewed
Reviewed
Approved
Next Review

October 2009
September 2021 – Assistant Manager, PSS
September 2021 – Infection Prevention and Management
September 2021 – PSS Manager
September 2023

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Where tradition plus innovation equals excellence >>>

Isolation room

Isolation room daily - including '2 times daily clean' requirements

Daily

Equipment:

- Linen bag
- Gloves
- Static Mop
- Dusting Mop
- Wet floor / hazard sign
- Rubbish bag
- Room Care R2L
- N95/P2 Particulate Respirator
- Asset JF
- Sodium Hypochlorite 0.125%
- Tuffie
- 70% Alcohol Wipes
- Safety Goggles
- Cleaning cloths (yellow)
- 4.8L Bucket (yellow)
- Charge Bucket (yellow)
- Mop pad (yellow)
- Step ladder



Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply PPE as indicated on the coloured card located on the door of the patient's room and according to the chemical being used 3. Place wet floor / hazard signs in highly visible areas 4. Use Sodium Hypochlorite 0.125% for cleaning, unless otherwise specified. 5. If visibly dirty, remove curtains. Linen curtains are placed in a linen bag. Antimicrobial curtains are placed in the waste. See Curtains (bed screens) procedure 6. Empty linen skip, damp wipe and replace with clean linen bag. See Linen procedure 7. Clean Television. See Television procedure 8. Using a dusting mop for high surfaces and yellow cleaning cloth for low surfaces, damp dust and wipe from clean to dirty: overhead railing, shelves, other patient furniture, soap and paper towel dispenser, hand basin, over bed light, over bed table, bed and frame	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture

9. See [Microfibre cloths 8 sided fold method](#) and [Damp dusting](#) procedures
10. Clean outside of electrical fittings with alcohol wipes
11. Spot clean walls if visibly soiled. See [Wall](#) procedure
12. If ensuite attached, clean hand basins, shower recess and toilet. See [Basin – hand](#), [Shower recess](#) and [Toilet](#) procedures
13. Dispose of Dusting Mop head and yellow cleaning cloths when finished and wipe over Dusting Mop handle with Tuffies/Room Care R2L
14. Static mop floor and then dispose of static cloth; wipe over static mop handle with Tuffies/Room Care R2L. See [Static sweeping](#) procedure
15. Mop floor thoroughly after room has been cleaned and then wipe over mop handle. See [Damp mopping](#) procedure
16. Dispose of yellow mop pad
17. Remove and dispose of PPE as required by the coloured isolation card
18. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
19. Once finished empty buckets into sluice (preferred) or toilet (then flush) and dispose of yellow cleaning cloths
20. Clean bucket/s with Asset JF and turn upside down to dry
21. Store cleaning equipment safely in the room until patient is discharged
22. Remove and dispose of PPE as dictated by the Isolation Room coloured card
23. Perform hand hygiene
24. Once floor is dry remove wet floor / hazard signs and return to storage area
25. Perform hand hygiene



Cleaning equipment is stored in the room until patient is discharged



Door needs to be kept shut

If cleaning is done twice daily wear a P2/N95 Particulate Respirator



Don't overstretch when cleaning use mop with extendable handle



Asset JF



Isolation Rooms Requiring 'Twice Daily Cleaning'

(Advised by Infection Prevention Management Team / PSS Supervisors)

Use Sodium Hypochlorite 0.125% for cleaning, unless otherwise specified.

Second 'Daily' Cleans – Target High (frequent) Touch Areas only

- See [High Touch Points](#) procedure

Patient Room and Bathroom high touch areas to be cleaned:

1. Bedside rails
2. Over-bed table
3. Nurse call bell
4. Door handles & push plates
5. Room basin
6. Toilet
7. Toilet flush button
8. Toilet paper dispenser
9. Paper towel dispenser
10. Bathroom basin
11. Bathroom/toilet hand rails
12. Bedside locker/patient storage unit
13. Patient chair
14. Patient phone
15. Television remote

General Ward high touch areas to be cleaned:

16. Shared toilets/bathrooms
17. Hand rails in corridors
18. Fridge and freezer handles

Floors are not required to be routinely cleaned during 2nd clean of the day, unless they are visibly dirty.



Room Care R2L



Sodium Hypochlorite
0.125%



Tuffie

Contact precaution (Orange) card or Single room precaution (White) card rooms only

When sodium hypochlorite 0.125% is being used within a contact precaution (Orange) card or Single Room (White) card room, and the patient is unable to leave the room when Sodium Hypochlorite 0.125% is in use, the door may be left open to allow dispersal of the sodium hypochlorite fumes.

It is the responsibility of the person who has left the door open to ensure that the door is closed immediately after the room has been cleaned. Whilst the door is open staff should not enter the room without the correct PPE



70% Alcohol Wipes

Exchange gloves and perform hand hygiene in-between cloths



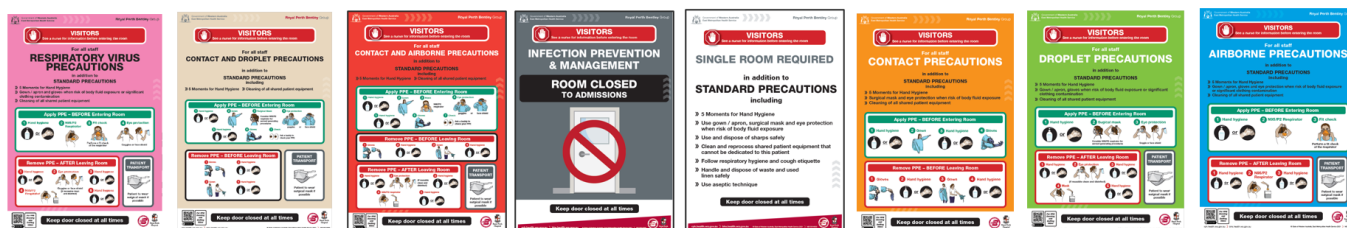
Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Isolation room discharge (1 step)

On discharge

Equipment:

- PPE
- 4.8L bucket (yellow)
- Sodium Hypochlorite 0.125%
- Tuffie
- 70% Alcohol Wipes
- Cleaning cloths (yellow)
- Dusting Mop
- Charge Bucket (yellow)
- Mop pad (yellow)
- Platform Safety Step ladder
- Static Mop
- Room Care R2L
- Wet floor / hazard sign
- Rubbish bag
- Linen skip



Procedure / Steps	Safety Points
Prior to cleaning commencing, Nursing/PCA staff: • Remove all curtains (linen or disposable), including shower curtains. See Curtains (bed screens) procedure • Remove hoist slings and send to laundry for washing • Tie up linen skip and remove from room. See Linen procedure • Check for any unsealed pillows or torn mattresses (including the inside) • If pillow is torn: ○ Place in plastic bag, seal and discard in rubbish ○ Inform Nurse Unit Manager/ shift coordinator to order replacement • If mattress is torn: ○ Place mattress in a large plastic bag, remove it from room and advise the Nurse Unit Manager/shift coordinator to arrange for a replacement	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture

- Remove wall posters and all sticky tape from surfaces of walls and furniture using Convacare adhesive remover wipes if required
- Nursing staff to unlock and empty medication drawers
- Do not dispose of paper towel or toilet paper which is contained in a dispenser
- Dispose of toilet rolls, not contained in a dispenser
- **Gloves stock in a wire frame holder**
 - Perform hand hygiene. Remove boxes of gloves from wire frame holder and dispose. They will be replaced with new stock.
- **Gloves stock in a Perspex frame holder**
 - Perform hand hygiene
 - Remove glove boxes from Perspex frame holder and place on a clean surface while clean is in progress
 - Clean Perspex frame holder
 - After the clean is complete, with clean hands return glove boxes to Perspex frame holder
- Dispose of products that are unable to be cleaned, for example those with paper backing
- Only dispose of suction canister and tubing if used
- Remove patient care equipment such as: bed pans, urinals, wash bowls, infusion pumps, stethoscopes, urinary measuring jugs so they may be appropriately cleaned, as outlined in the Infection Prevention Manual. See also [Patient equipment - hoist, dressing trolley, IV poles etc](#) procedure
 - Infusion pumps and stethoscopes cleaned before removal from room

For airborne precaution (blue carded) rooms wait 1 hour after patient has been discharged before entering



Exchange gloves and perform hand hygiene in-between cloths



Tuffie

ONE-STEP CLEAN AND DISINFECTION – always clean with sodium hypochlorite 0.125%

- Perform hand hygiene
- Put on PPE as indicated on the card located on the door of the patient's room and according to the cleaning products you will be using
- Place wet floor / hazard signs in highly visible areas
- Prepare to commence clean of the room using sodium hypochlorite 0.125% (this is premixed combined detergent and disinfectant solution) and Tuffies and Promed 70% Alcohol Wipes on electrical equipment
- Antelock Room is cleaned first
 - Shelving. See [Countertop, benchtop and desk](#) procedure
 - Basin, including soap and paper towel dispenser (remove a few sheets, but do not refill at this stage). See [Basin – hand](#) procedure
 - Floor area
- Televisions using a damp, not wet, extendable Dusting (Dolly) mop.
The television does not need to be cleaned using Sodium Hypochlorite 0.125%. See [Television](#) procedure
- Overhead railings and over bed light and shelves, suction canister and tubing
- Windows and doors. See [Door](#) procedure
- Suction canister can be wiped. Only dispose if used
- Room hand basin including soap dispenser and paper towel dispenser (remove a few sheets, but do not refill at this stage). See [Basin – hand](#) procedure
- Locker, drawers, over bed table and wardrobe
- Furniture (chairs, stool etc)
- Blood pressure cuffs:
 - the majority used in the hospital are single patient use. They are labelled with the patient sticker and are disposed of when visibly soiled or on discharge
 - Re-useable cuffs are cleaned with multipurpose cleaner or detergent wipes and dried after each patient use. They are



70% Alcohol Wipes



Room Care R2L



Sodium Hypochlorite 0.125%



Convacare Adhesive Remover Wipes

discarded if the Velcro becomes soiled with body fluids, hair etc.

- Spot clean the walls. See [Wall](#) procedure
- Clean the bed mattress and frame
 - Raise the trolley/bed to waist height to avoid stooping or stretching
 - Remove and wipe pillows and place on a clean surface
 - Wipe the top surface of the mattress. Use a forward backward lunge, flip mattress onto raised rails on opposite side. This exposes the bottom side up and allows what was the top side (now resting on the rails) to dry



- With the underside now facing up to the ceiling, wipe the underside of the mattress allow mattress to dry before placing back to normal position. The area between the mattress and the base of the bed may need to be dried by hand with a cloth
- Wipe bed rails
- Wipe bed frame
- Extend the foot of the bed and wipe all exposed surfaces including underneath of bed and bed wheels

Check that the trolley/bed is in good working order. If faulty, ensure faulty equipment is always cleaned, removed from service and tagged with an yellow “do not use” tag. See [Safety checklist for bed Inspection SOP](#)

- Wipe mattress and pillows with water following cleaning with Sodium Hypochlorite 0.125%.



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

• Waste bins emptied and cleaned. See Rubbish procedure • Linen skip emptied and cleaned. See Linen procedure • Bathrooms. If ensuite attached, clean hand basins, shower recess and toilet. See Basin – hand, Shower recess and Toilet procedures. Order of cleaning: ○ door rails, soap dispenser, paper towel dispenser (do not refill at this stage), mirror • shower recess and shower chair. See Shower recess and Patient equipment - hoist, dressing trolley, IV poles etc procedures ○ toilet paper dispenser (remove a few sheets, but do not refill at this stage), toilets and surrounds (as per the toilet cleaning procedure) ○ bathroom / toilet floor ○ dispose of cleaning solution and cloths • Use static mop to pick up loose dirt from floor and dispose of cloth. See Static sweeping procedure • Wipe down the handle with Tuffies and Promed wipes and place it outside the door • Mop floor area with Sodium Hypochlorite 0.125%. See Damp mopping procedure • Remove and dispose of PPE before leaving the room as required by the Isolation Card • Perform hand hygiene • Perform hand hygiene and apply new gloves before cleaning equipment used • Dispose of dusting mop head, disposable yellow microfibre mops and yellow disposable microfibre cloths. Wipe down handles with Tuffies and Promed wipes. Dispose of cleaning wipes. • Empty buckets into sluice or toilet and thoroughly wash out mop, dusting mop and buckets with Sodium hypochlorite 0.125%, and turn upside down to dry. Dispose of cleaning cloths	
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NOTE

PCAs strip and remake beds. Nursing can assist:

- Ensure all areas are dry before remaking the bed
- Generally, cleaners do not strip and remake beds

**The area between the mattress and the base of the bed must be dry.
Elevate mattress on bed frame and may need to be dried by hand with
a cloth.**

Ensure room is dry prior to preparing the room for new admission

- **Ensure all areas are dry before you commence making the bed.**

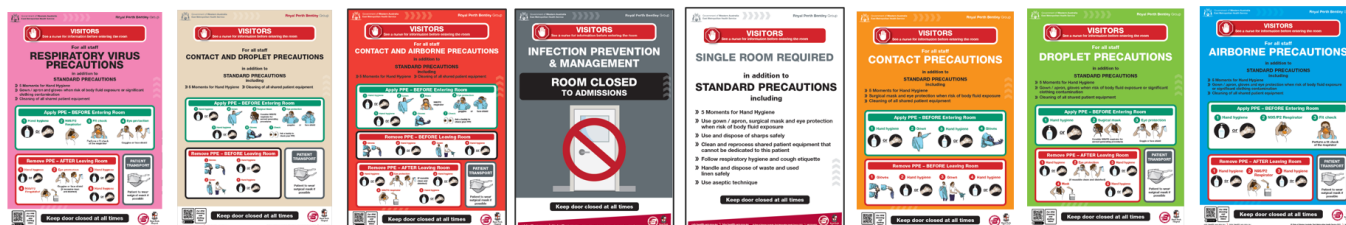
**With clean hands, restock paper towels and toilet paper as required
(when the level has reached the indicator window of dispenser)**

Isolation room discharge (2 step) - if 2nd step is Sodium Hypochlorite 0.125%

On discharge

Equipment:

- PPE
- Asset JF
- Sodium Hypochlorite 0.125%
- Tuffie
- 70% Alcohol Wipes
- Linen skip
- Cleaning cloths (yellow)
- 4.8L bucket (yellow)
- Charge Bucket (yellow)
- Mop pad (yellow)
- Platform Safety Step ladder
- Static Mop
- Dusting Mop
- Wet floor / hazard sign
- Rubbish bag
- Room Care R2L



Procedure / Steps	Safety Points
Prior to cleaning commencing, Nursing/PCA staff: - Remove all curtains (linen or disposable), including shower curtains. See Curtains (bed screens) procedure - Remove hoist slings and send to laundry for washing - Tie up linen skip and remove from room. See Linen procedure - Check for any unsealed pillows or torn mattresses (including the inside) If pillow is torn: - Place in plastic bag, seal and discard in rubbish - Inform Nurse Unit Manager/ shift coordinator to order replacement If mattress is torn:	For airborne precaution (blue carded) rooms wait 1 hour after patient has been discharged before entering  Exchange gloves and perform hand hygiene in-between cloths Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely

○ Place mattress in a large plastic bag, remove it from room and advise the Nurse Unit Manager/shift coordinator to arrange for a replacement • Remove wall posters and all sticky tape from surfaces of walls and furniture using Convacare adhesive remover wipes if required • Nursing staff to unlock and empty medication drawers • Do not dispose of paper towel or toilet paper which is contained in a dispenser • Dispose of toilet rolls, not contained in a dispenser • Gloves stock in a wire frame holder ○ Perform hand hygiene. Remove boxes of gloves from wire frame holder and dispose. They will be replaced with new stock. • Gloves stock in a Perspex frame holder ○ Perform hand hygiene ○ Remove glove boxes from Perspex frame holder and place on a clean surface while two step clean is in progress ○ Clean Perspex frame holder as part of the two step cleaning ○ After two step clean is complete, with clean hands return glove boxes to Perspex frame holder • Dispose of products that are unable to be cleaned, for example those with paper backing • Only dispose of suction canister and tubing if used • Remove patient care equipment such as: bed pans, urinals, wash bowls, infusion pumps, stethoscopes, urinary measuring jugs so they may be appropriately cleaned, as outlined in the Infection Prevention Manual. See also Patient equipment - hoist, dressing trolley, IV poles etc procedure ○ Infusion pumps and stethoscopes cleaned before removal from room	Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture   Asset JF    Tuffie
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FIRST STEP CLEAN – always clean with Asset JF

- Perform hand hygiene
- Put on PPE as indicated on the card located on the door of the patient's room and according to the cleaning products you will be using
- Place wet floor / hazard signs in highly visible areas
- Prepare to commence first clean of the room using multi-purpose cleaner (Asset JF) and Promed 70% Alcohol Wipes on electrical equipment
- Antelock Room is cleaned first
 - Shelving. See [Countertop, benchtop and desk](#) procedure
 - Basin, including soap and paper towel dispenser (remove a few sheets, but do not refill at this stage). See [Basin – hand](#) procedure
 - Floor area
- Televisions using damp, not wet, extendable Dusting (Dolly) mop. See [Television](#) procedure
- Overhead railings and over bed light and shelves, suction canister and tubing
- Suction canister can be wiped. Only dispose if used
- Windows and doors. See [Door](#) procedure
- Room hand basin including soap dispenser and paper towel dispenser (remove a few sheets, but do not refill at this stage). See [Basin – hand](#) procedure
- Locker, drawers, over bed table and wardrobe
- Furniture (chairs, stool etc)
- Blood pressure cuffs:
 - the majority used the in hospital are single patient use. They are labelled with the patient sticker and are disposed of when visibly soiled or on discharge
 - Re-useable cuffs are cleaned with multipurpose cleaner or detergent wipes and dried after each patient use. They are discarded if the Velcro becomes soiled with body fluids, hair etc.
- Spot clean the walls. See [Wall](#) procedure



70% Alcohol Wipes



Room Care R2L



Exchange gloves and perform hand hygiene in-between cloths



Sodium Hypochlorite
0.125%

- Clean the bed mattress and frame
 - Raise the trolley/bed to waist height to avoid stooping or stretching
 - Remove and wipe pillows and place on a clean surface
 - Wipe the top surface of the mattress. Use a forward backward lunge, flip mattress onto raised rails on opposite side. This exposes the bottom side up and allows what was the top side (now resting on the rails) to dry



- With the underside now facing up to the ceiling, wipe the underside of the mattress allow mattress to dry before placing back to normal position. The area between the mattress and the base of the bed may need to be dried by hand with a cloth
- Wipe bed rails
- Wipe bed frame
- Extend the foot of the bed and wipe all exposed surfaces including underneath of bed and bed wheels

Check that the trolley/bed is in good working order. If faulty, ensure faulty equipment is always cleaned, removed from service and tagged with an yellow “do not use” tag. See [Safety checklist for bed Inspection SOP](#)

- Waste bins emptied and cleaned. See [Rubbish](#) procedure
- Linen skip emptied and cleaned. See [Linen](#) procedure
- Bathrooms. If ensuite attached, clean hand basins, shower recess and toilet. See [Basin – hand](#), [Shower recess](#) and [Toilet](#) procedures.



Convacare Adhesive Remover Wipes



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Prepare new cleaning solution (Asset JF) and cleaning cloths. Order of cleaning:

- Door rails, soap dispenser, paper towel dispenser (do not refill at this stage), mirror
- shower recess and shower chair
- toilet paper dispenser (remove a few sheets, but do not refill at this stage), toilets and surrounds
- Bathroom / toilet floor
- dispose of cleaning solution and cloths
- Use static mop to pick up loose dirt from floor and dispose of cloth. See [Static sweeping](#) procedure
- Wipe down the handle with Tuffies/Room care R2L and place the handle outside the door
- With new cleaning solution, damp mop floor area. See [Damp mopping](#) procedure
- Remove and dispose of gloves and perform hand hygiene.
- Perform hand hygiene and apply new gloves before emptying and cleaning buckets
- Empty buckets into sluice or toilet and thoroughly wash out mop, dusting mop and buckets with **Asset JF**, then rinse with water. Dispose of cleaning cloths

SECOND STEP – can be Nocospray or Sodium Hypochlorite 0.125%

- **If using Nocospray for the second step refer to Nocospray instructions OR use Sodium Hypochlorite 0.125% (refer below)**
- Change PPE prior to performing the second clean
- **Wait until all surfaces within the room are dry** then commence SECOND STEP of patient's room using Sodium Hypochlorite 0.125% (undiluted), wiping over all surfaces noted at FIRST STEP - **except the television which does not need to be cleaned using Sodium Hypochlorite 0.125%**
- Wipe mattress and pillows with water following cleaning with Sodium Hypochlorite 0.125%.

- Dispose of dusting mop head. Put mop head into plastic bag for laundering and wipe down handles with Tuffies/Room Care R2L. Dispose of cleaning cloths
- Clean bucket/s with Sodium Hypochlorite 0.125% and turn upside down to dry
- Remove and dispose of PPE before leaving the room as dictated by the Isolation Room card
- Perform hand hygiene

NOTE

PCAs strip and remake beds. Nursing can assist:

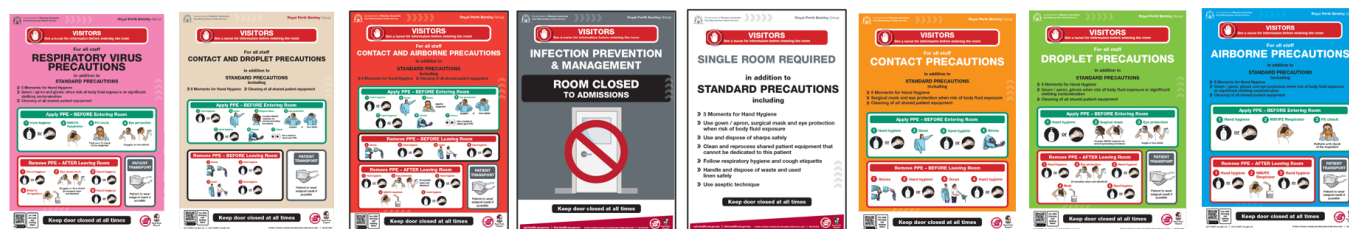
- Ensure all areas are dry before remaking the bed
- Generally, cleaners do not strip and remake beds
- **The area between the mattress and the base of the bed must be dry. Elevate mattress on bed frame and may need to be dried by hand with a cloth**
- **Ensure room is dry prior to preparing the room for new admission**
- **Ensure all areas are dry before you commence making the bed**
- **With clean hands, restock paper towels and toilet paper as required (when the level has reached the indicator window of dispenser)**

Isolation room discharge (2 step) - if 2nd step is NocoSpray

On discharge

Equipment:

- PPE
- Asset JF
- Nocospray machine
- Tuffie
- 70% Alcohol Wipes
- Nocolyse
- Cleaning cloths (yellow)
- 4.8L bucket (yellow)
- Charge Bucket (yellow)
- Mop pad (yellow)
- Platform Safety Step ladder
- Linen skip
- Static Mop
- Dusting Mop
- Wet floor / hazard sign
- Rubbish bag
- Room Care R2L



See also Nocospray Use in Cleaning Isolation Rooms SOP (RPH):

[https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/EMHS/RPH/Nocospray%20Use%20in%20Cleaning%20Isolation%20Rooms%20SOP%20\(RPH\).pdf](https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/EMHS/RPH/Nocospray%20Use%20in%20Cleaning%20Isolation%20Rooms%20SOP%20(RPH).pdf)

Procedure / Steps	Safety Points
Prior to cleaning commencing, Nursing/PCA staff: • Remove all curtains (linen or disposable), including shower curtains. See Curtains (bed screens) procedure • Remove hoist slings and send to laundry for washing • Tie up linen skip and remove from room. See Linen procedure • Check for any unsealed pillows or torn mattresses (including the inside) • If pillow is torn: ○ Place in plastic bag, seal and discard in rubbish ○ Inform Nurse Unit Manager/ shift coordinator to order replacement	For airborne precaution (blue carded) rooms wait 1 hour after patient has been discharged before entering  Cleaning equipment is stored in the room until patient is discharged

- **If mattress is torn:**

- Place mattress in a large plastic bag, remove it from room and advise the Nurse Unit Manager/shift coordinator to arrange for a replacement

- Remove wall posters and all sticky tape from surfaces of walls and furniture using Convacare adhesive remover wipes if required

- Nursing staff to unlock and empty medication drawers

- Do not dispose of paper towel or toilet paper which is contained in a dispenser

- Dispose of toilet rolls, not contained in a dispenser

- **Gloves stock in a wire frame holder**

- Perform hand hygiene. Remove boxes of gloves from wire frame holder and dispose. They will be replaced with new stock.

- **Gloves stock in a Perspex frame holder**

- Perform hand hygiene
- Remove glove boxes from Perspex frame holder and place on a clean surface while two step clean is in progress
- Clean Perspex frame holder as part of the two step cleaning
- After two step clean is complete, with clean hands return glove boxes to Perspex frame holder

- Dispose of products that are unable to be cleaned, for example those with paper backing

- Only dispose of suction canister and tubing if used

- Remove patient care equipment such as: bed pans, urinals, wash bowls, infusion pumps, stethoscopes, urinary measuring jugs so they may be appropriately cleaned, as outlined in the Infection Prevention Manual. See also [Patient equipment - hoist, dressing trolley, IV poles etc](#) procedure



Tuffie



ProMed 70% Alcohol Wipes



Room Care R2L



Door needs to be kept shut

- Infusion pumps and stethoscopes cleaned before removal from room

FIRST STEP CLEAN – always clean with Asset JF

- Perform hand hygiene
- Put on PPE as indicated on the card located on the door of the patient's room and according to the cleaning products you will be using
- Place wet floor / hazard signs in highly visible areas
- Prepare to commence first clean of the room using multi-purpose cleaner (Asset JF) and Promed 70% Alcohol Wipes on electrical equipment
- Antelock Room is cleaned first
 - Shelving. See [Countertop, benchtop and desk](#) procedure
 - Basin, including soap and paper towel dispenser (remove a few sheets, but do not refill at this stage). See [Basin – hand](#) procedure
 - Floor area
- Televisions using damp, not wet, extendable Dusting (Dolly) mop. See [Television](#) procedure
- Overhead railings and over bed light and shelves, suction canister and tubing
- Suction canister can be wiped. Only dispose if used
- Windows and doors. See [Door](#) procedure
- Room hand basin including soap dispenser and paper towel dispenser (remove a few sheets, but do not refill at this stage). See [Basin – hand](#) procedure
- Locker, drawers, over bed table and wardrobe
- Furniture (chairs, stool etc)
- Blood pressure cuffs:
 - the majority used the in hospital are single patient use. They are labelled with the patient sticker and are disposed of when visibly soiled or on discharge

****Covers for smoke detectors on Ward 4A/B**** available from PSS Assistant Manager's office

See Microfibre cloths 8 sided fold method



Exchange gloves and perform hand hygiene in-between cloths

Always consider correct body postures to reduce risk of injury

Maintain normal spinal posture at all times.

Adopt appropriate posture to perform task safely

Maintain wide base of support

Avoid overreaching, over stretching or twisting actions

Keep arms close to body

Avoid stooped posturing

Keep neck in neutral posture

- Re-useable cuffs are cleaned with multipurpose cleaner or detergent wipes and dried after each patient use. They are discarded if the Velcro becomes soiled with body fluids, hair etc.
- Spot cleaning walls. See [Wall](#) procedure
- Bed mattress and frame
 - Raise the trolley/bed to waist height to avoid stooping or stretching
 - Remove and wipe pillows and place on a clean surface
 - Wipe the top surface of the mattress. Use a forward backward lunge, flip mattress onto raised rails on opposite side. This exposes the bottom side up and allows what was the top side (now resting on the rails) to dry



- With the underside now facing up to the ceiling, wipe the underside of the mattress allow mattress to dry before placing back to normal position. The area between the mattress and the base of the bed may need to be dried by hand with a cloth
- Wipe bed rails
- Wipe bed frame
- Extend the foot of the bed and wipe all exposed surfaces including underneath of bed and bed wheels

Check that the trolley/bed is in good working order. If faulty, ensure faulty equipment is always cleaned, removed from service and tagged with an yellow “do not use” tag. See [Safety checklist for bed Inspection SOP](#)



Nocospray Machine



Nocolyse



Do Not Enter sign



Convacare Adhesive Remover Wipes

- Waste bins emptied and cleaned. See [Rubbish](#) procedure
- Linen skip emptied and cleaned. See [Linen](#) procedure
- Bathrooms. If ensuite attached, clean hand basins, shower recess and toilet. See [Basin – hand](#), [Shower recess](#) and [Toilet](#) procedures. Prepare new cleaning solution (Asset JF) and cleaning cloths. Order of cleaning:
 - Door rails, soap dispenser, paper towel dispenser (do not refill at this stage), mirror
 - shower recess and shower chair
 - toilet paper dispenser (remove a few sheets, but do not refill at this stage), toilets and surrounds (as per the toilet cleaning procedure)
 - Bathroom / toilet floor
 - dispose of cleaning solution and cloths
- Use static mop to pick up loose dirt from floor and dispose of cloth. See [Static sweeping](#) procedure
- Wipe down the handle with Tuffies/Room care R2L and place the handle outside the door
- With new cleaning solution, damp mop floor area. See [Damp mopping](#) procedure
- Remove and dispose of gloves and perform hand hygiene.
- Perform hand hygiene and apply new gloves before emptying and cleaning buckets
- Empty bucket/s into sluice or toilet and thoroughly wash out bucket/s with Asset JF, then rinse with water. Dispose of yellow cleaning cloths

SECOND STEP – can be Nocospray or Sodium Hypochlorite 0.125%

- If using Sodium Hypochlorite 0.125% for the second step refer to ISOLATION ROOM DISCHARGE ('2 Step Clean') procedure **OR** Nocospray (refer below)

Only trained staff are to use the Nocospray system. Do not used if untrained.



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

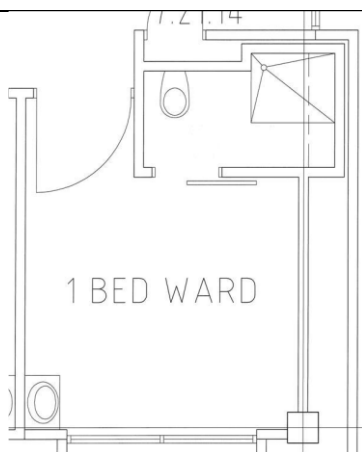
Prepare room for NocoSpray (Refer also to Noco Spray Conversions and Durations and Safe Operating Procedure

- Open all drawers and cupboard doors in patient room and ensuite bathroom.
- Roll window blinds fully down.
- Ensure door to ensuite remains open.
- Lift up toilet seat.
- Place side rails on bed in the up position. Lean the mattress and pillow against one of the raised side rails making sure that both surfaces are exposed.
- Place any extra equipment that requires disinfecting in room taking care to ensure that it has been cleaned previously with detergent.
- Seal the door vent.

Set up NocoSpray Machine

- Wait until all surfaces within the room are dry before commencing the disinfection of patient room using the NocoSpray Machine.
- Perform hand hygiene.
- Apply PPE as required for setting up NocoSpray.
- Push NocoSpray trolley inside the room, in the corner opposite to the ensuite ensuring the NocoSpray is facing towards the ensuite (pointing away from smoke detector, see NocoSpray manual).
 - ****Covers for smoke detectors on Ward 4A/B**** available from PSS Assistant Manager's office





- The NocoSpray must sit on a hard surface so the airflow is not comprised and should not be removed from the trolley. The NocoSpray should never be placed on the floor/chair.
- **DO NOT** move machine or trolley whilst the bottle is inserted in the machine.
- **DO NOT** spray in direction of lights, electricals, smoke detectors or other sources of heat.
- Always lock the brakes on the NocoSpray trolley.

Using the NocoSpray machine

If in doubt re the use of the NocoSpray machine, always contact your PSS Supervisor

- Remove nozzle from NocoSpray machine.
- Take the lid off the 1 litre Nocolyse (disinfectant) bottle and screw the filler tube inside the bottle.
- Replace the nozzle onto the machine, ensuring the Nocolyse bottle rests in the cavity below it.
- **Remove PPE and perform hand hygiene**
- Connect the machine to a power outlet.
- ***Refer to the setting chart (attached on the trolley) or in this Manual; or consult your Supervisor for correct setting.***
- Select the correct room size on the dial, using the selector switch/dial on the side of the machine.
- Switch on the machine using on/off switch at rear of machine.

- Press the start button at the side of machine, following this a buzzer will sound for 15 seconds during which you should leave the room.
- **IMPORTANT:** Always ensure that the door to the room is securely closed behind you when you leave
- Place a **“Warning-Disinfection in Progress. Do Not Enter”** sign over door at entrance to room. Write re-entry time on sign after closing door, refer to Noco Spray Conversions and Durations.
- The machine will begin to operate, making a whistling sound.
- Place a towel on the floor in front of the door outside the room.
- Inform the Nurse Coordinator that the room is under treatment for the specified disinfection time.

Remove the NocoSpray Machine after the appropriate time has elapsed

Never enter a room prior to the Total Time being reached

- Perform hand hygiene.
- Apply gloves and goggles.
- Switch off the machine using the on/off switch at rear of machine.
- Disconnect machine from power outlet.
- Remove nozzle from machine and unscrew Nocolyse bottle, replacing the lid. Place nozzle back on the machine.
- Using Tuffies, damp wipe over the machine particularly the nozzle area.
- Remove PPE and perform hand hygiene.

Prepare Room for Patient Use

- Remove the towel from the front of the door
- **Remove cover from room smoke detector**
- Remove the cover that sealed the door vents
- Close all drawers and cupboards that were opened.
- Put toilet seat down.
- Replace mattress correctly on bed.
- Replace consumables as required Toilet Paper, Hand Towel, Rubbish Bags, Linen bags, Infectious Waste Bags.

• Move any equipment that does not belong in the room to designated areas. • Remove “Warning - Disinfection in Progress. Do Not Enter” from door. • Remove machine and place back in storage PCAs strip and remake beds. Nursing can assist: • Ensure all areas are dry before remaking the bed • Generally, cleaners do not strip and remake beds	
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NocoSpray durations

Ward/Area	Room Description	Size m ³	Spray Time mins/secs	Total time mins/secs
Q BLOCK				
CCU Coronary Care Unit	Single with ensuite B - F	50 m ³	3:00	33:00
	2 bed with ensuite	99 m ³	6:00	36:00
4F	Single with ensuite K - N	60 m ³	3:45	33:45
	4 bed with ensuite I, J, O & P	124 m ³	7:30	37:30
AMU Acute Medical Unit	2 bed with ensuite A	94 m ³	6:00	36:00
	Single with ensuite B - H	48 m ³	3:00	33:00
	4 bed with ensuite I - M	118 m ³	7:30	37:30
	6 bed with ensuite	110 m ³	6:45	36:45
	Room P	223 m ³	13:30	43:30
A BLOCK				
9C	Single with ensuite	70 m ³	4:30	34:30
	4 bed with toilet & shower	203 m ³	13:30	43:30
	2 bed with ensuite	93 m ³	6:00	36:00
A Block – All other wards (excluding 9C)	Male/Female Toilet & Shower	63 - 74 m ³	4:30	34:30
	4 bed	101 – 112 m ³	6:45	36:45
	4 bed with toilet & shower	157 m ³	9:45	39:45
	2 bed	51 – 63 m ³	3:45	33:45
	Single	44 m ³	2:42	32:42
	Single with ensuite	51 – 63 m ³	3:45	33:45
Public Toilets – Level 3	Disabled Toilet	22 m ³	1:30	31:30
	Male Toilet	46 m ³	3:00	33:00
	Female Toilet	88 m ³	5:15	35:15
R BLOCK				
SMTU State Major Trauma Unit 3G	Single with ensuite B, C & D	73 m ³	4:30	34:30
	Single with ensuite A, J, L & S	86 m ³	5:15	35:15
	2 bed with ensuite I & P	95 m ³	6:00	36:00
	2 bed with ensuite H & R	110 m ³	6:45	36:45
	2 bed with ensuite F, G, N, O, K & Q	48 m ³	7:30	37:30
	2 bed with ensuite M	133 m ³	8:15	38:15
SMTU State Major Trauma Unit 3H	Single with ensuite	84 m ³	5:15	35:15
	2 bed with ensuite F	69 m ³	4:30	34:30
6G/6H/5G/5H	Single with ensuite	63 – 75 m ³	4:30	34:30
	2 bed	58 m ³	3:45	33:45
	4 bed with sep toilet & shower	133 m ³	7:30	37:30

	6H Bariatric Room with ensuite	110 m ³	6:45	36:45
ICU Surgical side	Single isolation room 1 - 3	54 m ³	3:45	33:45
	Single isolation room 9 - 12	89 m ³	6:00	36:00
ICU General side	Negative pressure rooms 1 - 4	51 m ³	3:45	33:45

Microfibre

Microfibre cloth preparation

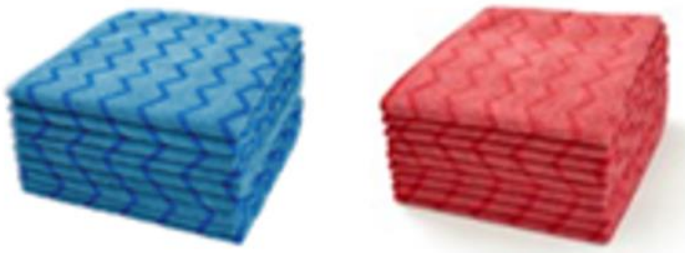
As required

Equipment:

- Cleaning cloths (colours as needed)
- Gloves
- Asset JF
- 4.8L buckets (colours as needed)
- Measuring jug

Procedure / Steps	Safety Points
Perform hand hygiene. Gloves are not required to fold cloths.  1. BEGIN Begin with open, clean Microfibre cloth.  2. FOLD Fold Microfibre cloth in half.  3. FOLD Fold Microfibre cloth in quarters, resulting in a square-shaped pad with four layers of cloth.  4. PLACE Place folded cloths into the appropriate coloured pail, with cloths standing upright.  5. POUR Pour diluted chemical solution to the correct ratios above, slowly over the top of the cloths to dampen evenly.  6. ABSORB Allow 5-10 minutes for the cloths to absorb diluted chemical solution evenly. Cloths are now ready for use. Note - At Step 5 perform hand hygiene and apply gloves before dispensing and pouring Asset J-Fill solution 7. Remove and dispose of gloves 8. Perform hand hygiene	    Asset JF

See [Microfibre 8 sided folding](#) and [Microfibre Ratios](#) procedures



- Blue for general cleaning
- Red for bathrooms, toilets and showers



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Microfibre cloth 8 sided fold method

As required

Equipment:

- Cleaning cloths (colours as needed)

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Begin with open clean Microfibre cloth 3. Fold Microfibre cloth in half 4. Fold Microfibre cloth in quarters resulting in a square-shaped pad with four layers of cloth	Perform hand hygiene in-between cloths Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

5. **Clean surfaces** with the two exposed sides of the Microfibre cloth. Track the side used for each surface while cleaning



6. **Open** Microfibre cloth to change sides



7. **Refold** to expose two fresh cleaning surfaces. Track the side used for each surface while cleaning



8. **Open** the cloth fully once four surfaces have been used and expose four fresh surfaces.



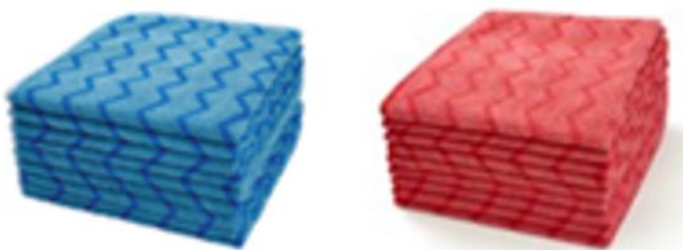
9. **Repeat** Steps 3 through 8 to use all eight sides



10. Dispose of cleaning cloth after use. Launderable (blue) mop pads and (blue) cleaning cloths placed in clear plastic bag. Clear plastic bag placed in blue linen bag. Send for laundering at the end of each shift. Blue and red cloths are launderable, yellow disposed

11. Perform hand hygiene

See [Microfibre Ratios](#) and [Microfibre cloth preparation](#) procedures



- Blue for general cleaning
- Red for bathrooms, toilets and showers

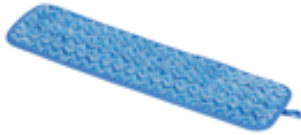
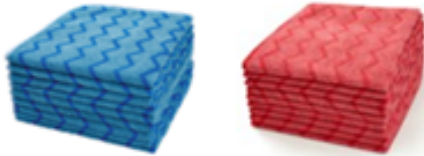


Microfibre ratios general cleaning

As required

Equipment:



MICROFIBRE PREMIUM HYGEN™ 45CM WET PAD, BLUE LAUNDERABALE  <i>Always prepare 5 pads at a time only. i.e.</i> <i>Place 5 pads then pour 800 mL</i> <i>Place 5 pads then pour 800 mL</i> <i>Place 5 pads then pour 800 mL</i> <i>...and so on</i>	Number required	Mils of Asset J-Fill 	Absorb time 
	5 mops	800 mL	Approx. 10 – 15 minutes
	10 mops	1.6 L	
	15 mops	2.4 L	
	20 mops	3.2 L	
	25 mops	4 L	
	30 mops	4.8 L	
MICROFIBRE BLUE & RED CLOTH – LAUNDERABALE  <i>Blue for general cleaning</i> <i>Red for bathrooms, toilets & showers</i>	6 cloths	450 mL	Approx. 5 – 10 minutes
	8 cloths	600 mL	
	10 cloths	750 mL	
	12 cloths	900 mL	
Cloths for use on grey mattresses require more solution. Approximately 100 – 125 mL per cloth			
Exchange gloves and perform hand hygiene in-between cloths 			

See [Microfibre cloth preparation](#) and [Microfibre 8 sided folding](#) procedures

Microfibre ratios isolation cleaning

As required

Equipment:



OATES HYPRO MOP – DISPOSABLE	Number required	Mils of Sodium Hypochlorite 0.125% 	Absorb time 
	1 mop	80 mL	2 minutes
	2 mops	160 mL	
	3 mops	240 mL	
	4 mops	320 mL	
	5 mops	400 mL	
OATES YELLOW CLOTH – DISPOSABLE	1 cloth	40 mL	2 minutes
2 cloths	80 mL		
3 cloths	120 mL		
4 cloths	160 mL		
5 cloths	200 mL		
6 cloths	240 mL		
7 cloths	280 mL		
8 cloths	320 mL		
9 cloths	360 mL		
10 cloths	400 mL		
<i>Exchange gloves and perform hand hygiene in-between cloths</i> 			

See [Microfibre cloth preparation](#) and [Microfibre 8 sided folding](#) procedures

Patient room and surrounds

Daily

Equipment:

- Gloves
- Mop pad (blue)
- Brown paper rubbish bag
- Convacare adhesive remover
- Bulldog clip
- Goggles
- 4.8L bucket (blue)
- Cleaning cloth (blue)
- Room Care R2L
- 70% Alcohol Wipes
- Platform Safety Step ladder
- Charge bucket (grey)
- Plastic rubbish bag
- Asset JF
- Wet floor / hazard signs
- Extendable dusting mop

Procedure / Steps	Safety Points
This procedure is completed by PCAs and Cleaners and MUST be done in the correct sequence Includes high touch points, overhead lights, bedside lockers, over bed tables, curtain rails, high and low dusting 1. Perform hand hygiene 2. Apply gloves 3. Half fill blue bucket with Asset JF 4. Place wet floor/hazard signs where visible 5. Remove used brown paper bag from patient's bedside locker or patient bed tables as required or when maximum $\frac{3}{4}$ full and dispose of in ward bin Note: • Brown paper bags are to be replaced at least once a day, but may need to be changed more frequently • Brown paper bags preferably secured by a bulldog clip, not sticky tape 6. Remove any sticky tape residue with Convacare adhesive remover wipes 7. With a extendable dusting mop and or damp, not wet, blue cleaning cloth, wipe high touch points. See High Touch Points procedure 8. Clean Television. See Television procedure 9. When finished <u>each bed area</u>, dispose of cleaning cloth appropriately. Blue and red cloths are launderable, yellow disposed 10. Empty cleaning solution in sluice	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 

11. Remove and dispose of gloves and perform hand hygiene.
12. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
13. Clean bucket/s with Asset JF.
14. Remove gloves and perform hand hygiene
- 15. Start procedure again for next bed area.**
16. Empty rubbish bin and replace liner leave in room (after hours and in ICU). See [Rubbish](#) procedure
17. Remove and dispose of gloves
18. Perform hand hygiene
19. Place new brown paper bag onto patient's bedside locker or patient over bed tables with bulldog clip and make sure bag is secure
20. Ensure the bag can be reached by the patient
21. Perform hand hygiene
22. Damp mop room. See [Damp mopping](#) procedure
23. Once floor is dry remove wet floor / hazard signs and return to storage area
24. Perform hand hygiene
25. Replenish soap and paper towel dispensers when empty (after hours and in ICU)

This procedure is completed by PCAs and Cleaners and MUST be done in the correct sequence



Convacare Adhesive Remover Wipes

Do not over extend yourself use extendable dusting mop.



When cleaning lights, turn off first and report any faulty lights to Nurse Coordinator.

Use Risk Assessment and correct Manual Handling techniques when moving furniture.



Asset JF



Exchange gloves and perform hand hygiene in-between cloths



70% Alcohol Wipes



Room Care R2L





Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Television – general

Daily

Equipment:

- Gloves
- 4.8L bucket (blue)
- Asset JF
- Extendable Dusting (Dolly) Mop
- Safety Goggles

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply gloves and goggles 3. Half fill blue bucket with Asset JF 4. Display wet floor/hazard signs 5. Use Dusting (Dolly) Mop (extendable) in Asset JF detergent solution and gently squeeze/wring until damp (almost dry), with no water dripping  6. Dust all external surfaces of the Television set as required 7. PSS staff are not to wipe connections and wiring 8. When all dust has been removed, dispose of Dusting (Dolly) mop head 9. Do not dispose reusable mop handle 10. Remove gloves, then perform hand hygiene 11. Remove goggles and perform hand hygiene	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture   

12. Apply new gloves before wiping Dusting (Dolly) mop handle and emptying and cleaning buckets
13. Wipe the Dusting (Dolly) mop handle and hang to dry
14. Empty bucket/s, clean with Asset JF and turn bucket/s upside down to dry
15. Remove goggles and dispose of gloves
16. Perform hand hygiene

PSS staff are not to wipe connections and wiring



Asset JF



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Television - Health In a Virtual Environment (HIVE) room specific Daily

Equipment:

- Gloves
- 4.8L bucket (blue)
- Asset JF
- Extendable Dusting (Dolly) Mop
- Goggles

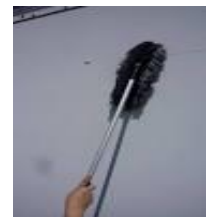
Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply gloves and goggles 3. Half fill blue bucket with Asset JF 4. Display wet floor/hazard signs 5. Use Dusting (Dolly) Mop (extendable) in Asset JF detergent solution and gently squeeze/wring until damp (almost dry), with no water dripping 6. Dust all external surfaces of the Television set as required	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture
 7. PSS staff clean flat surfaces and the protective cover 8. PSS staff are not to wipe connections and wiring 9. HIVE Technicians are responsible for the cleaning and maintenance where connections and wiring are exposed	

10. When all dust has been removed, dispose of Dusting (Dolly) mop head
11. Do not dispose reusable mop handle
12. Remove gloves, then perform hand hygiene
13. Remove goggles and perform hand hygiene
14. Apply new gloves before wiping Dusting (Dolly) mop handle and emptying and cleaning buckets
15. Wipe the Dusting (Dolly) mop handle and hang to dry
16. Empty bucket/s, clean with Asset JF and turn bucket/s upside down to dry
17. Remove goggles and dispose of gloves
18. Perform hand hygiene

PSS staff are not to wipe connections and wiring



Asset JF



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Vent - door/ceiling

Weekly & as required

Equipment:

- Gloves
- Cleaning cloth (blue)
- 4.8L bucket (blue)
- Extendable Dusting mop
- Asset JF

Procedure / Steps	Safety Points
1 Perform hand hygiene 2 Apply gloves 3 Half fill blue bucket with Asset JF 4 See Damp dusting procedure <u>Dusting mop</u> 5 Wet dusting (dolly) mop and gently squeeze until only damp 6 The dusting (dolly) mop head must be in contact with surfaces at all times  7 Rinse and gently squeeze out frequently, to remove dirt / dust 8 Dispose of dusting (dolly) mop <u>Cleaning cloth</u> 5. With a damp, not wet, blue cloth wipe over all surfaces. See Microfibre cloths 8 sided fold method procedure	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 

6. Dispose of cleaning cloth/s appropriately. Blue cloths are Launderable
7. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
8. Empty bucket/s in sluice and clean with Asset JF detergent, turn bucket/s upside down to dry
9. Remove and dispose of gloves
10. Perform hand hygiene



Asset JF



Exchange gloves and perform hand hygiene in-between cloths



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WARD AREA

Key safety principles:

When cleaning in a hospital ward area, it's essential to use proper body mechanics to minimise the risk of musculoskeletal injury. Always stand with your feet shoulder-width apart to maintain a stable base, and avoid twisting your torso when reaching or moving heavy equipment. Bend your knees, not your back, when picking up cleaning supplies or when mopping or sweeping. If you need to reach high or low areas, adjust your body position by squatting or using a step stool, instead of stretching awkwardly, which could strain your back, shoulders, or neck. Keep the cleaning tools close to your body, and use long-handled mops or brooms to reduce bending and awkward postures.

In addition to posture, regular breaks and ergonomic tools are key to preventing injury during cleaning in hospital wards. Use equipment like adjustable vacuum cleaners or floor polishers to ensure you're not overexerting your muscles, and make sure cleaning carts are at waist height to prevent constant bending or lifting. For repetitive tasks like dusting or wiping, alternate tasks and take breaks to stretch your arms, shoulders, and back. Stretching helps release muscle tension and improves flexibility, reducing the likelihood of strain. By practicing these techniques, healthcare workers can significantly reduce the risk of musculoskeletal injuries while maintaining a clean and safe environment in hospital wards.



Countertop, benchtop and desk

Daily

Equipment:

- 4.8L bucket (blue)
- Asset JF
- Cleaning cloth (blue)
- Gloves

Procedure / Steps	Safety Points
    1. Perform hand hygiene 2. In clinical areas remove items off the tops, and place on a cleaned surface such as a trolley 3. Perform hand hygiene and apply gloves 4. See Microfibre cloths 8 sided fold method and Damp dusting procedures 5. Remove items of counter top and place on a clean surface 6. With a damp, not wet, blue cleaning cloth, wipe over counter top and surrounds such as walls / shelves. See Wall procedure 7. Do not use a scourer to remove marks as this may scratch the bench top	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 

8. Once finished, dispose of cleaning cloth/s appropriately. Blue cloths are launderable
9. Remove and dispose of gloves and perform hand hygiene
10. Replace items back that were removed, back in the same place, neatly.
11. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
12. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry.
13. Remove and dispose of gloves
14. Perform hand hygiene



Asset JF



Exchange gloves and perform hand hygiene in-between cloths



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Drinking fountain

Daily

Equipment:

- Gloves
- Scourer
- Asset JF
- 4.8L bucket (blue)
- Cleaning cloth (blue)

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply gloves 3. With a damp, not wet, blue cleaning cloth, wipe over drinking fountain including handle and spout of fountain  4. Using scourer ensure all joins and crevices are free from dust, calcium and dirt build up 5. Once finished, dispose of scourer 6. Dispose of cleaning cloths appropriately. Blue cloths are launderable 7. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets 8. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry 9. Remove and dispose of gloves 10. Perform hand hygiene	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 



Asset JF



Exchange gloves and perform hand hygiene in-between cloths



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Fridges

Medicine and treatment room fridge Weekly

Equipment:

- Gloves
- Cleaning cloth (blue)
- 4.8L bucket (blue)
- Asset JF

Procedure / Steps	Safety Points
While cleaning in a medication room, a clinical (nursing) staff member must remain present at all times 1. Ask nursing staff to remove contents from fridge 2. Turn off power supply to the fridge 3. Perform hand hygiene 4. Apply gloves 5. Open fridge door 6. With a damp, not wet, blue cleaning cloth, wipe over all surfaces inside the fridge from top to bottom. See Microfibre cloths 8 sided fold method and Damp dusting procedures 7. Wipe seal and outer surface of the fridge 8. Close fridge door 9. Dispose of cleaning cloths appropriately. Blue cloths are launderable 10. Remove and dispose of gloves and perform hand hygiene. 11. <u>Turn on power supply to fridge</u> 12. <u>Advise nursing staff that the fridge is now clean and that they can return contents to fridge</u> 13. Perform hand hygiene and apply new gloves before emptying and cleaning buckets 14. Empty and clean bucket/s with Asset JF. Turn upside bucket/s down to dry	Check electrical cord to ensure there are no breaks or splits Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 

15. Remove and dispose of gloves

16. Perform hand hygiene



Asset JF



Exchange gloves and perform hand hygiene in-between cloths



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Ward fridge & staff fridge daily

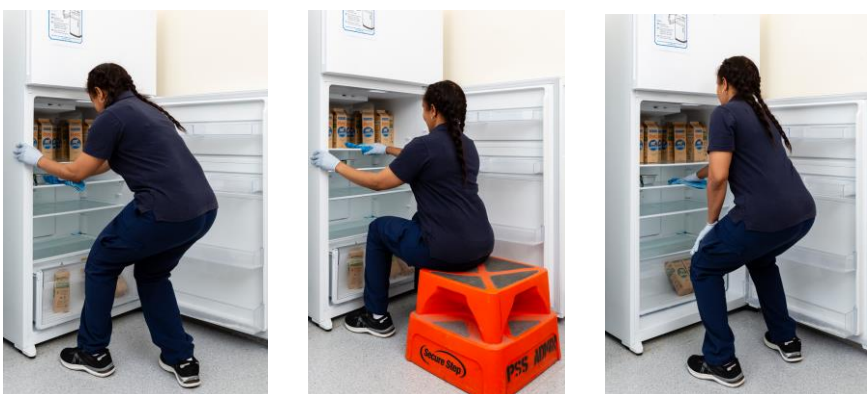
Daily

Equipment:

- Fridge
- Gloves
- Cleaning cloth (blue)
- 4.8L bucket (green)
- Asset JF

See also Ward Refrigerator Management SOP:

<https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/EMHS/RPH/Ward%20Refrigerator%20Management%20SOP.pdf>

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Open fridge door 3. Inspect fridge and freezer before 10.00am daily for expired items and storage requirements per the notice below 4. Check with nursing staff if it is safe to dispose of any contents 5. Dispose of contents as required 6. Perform hand hygiene and apply gloves 7. With a damp, not wet, blue cleaning cloth, spot clean any spills. See Microfibre cloths 8 sided fold method and Damp dusting procedures	 Inspect fridge and freezer before 10.00am daily Check with nursing staff before disposing of any contents Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions
 8. Remove gloves and perform hand hygiene. 9. Rectify storage of contents as required, per the notice below 10. Perform hand hygiene and apply new gloves before emptying and cleaning buckets	

11. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry 12. Remove and dispose of gloves 13. Perform hand hygiene	Keep arms close to body Avoid stooped posturing Keep neck in neutral posture  Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.
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Appendix I: Staff refrigerator poster

Staff Refrigerator

Can be stored in refrigerator

- ✓ Staff food and beverages

Do not store in refrigerator

- ✗ Patient food and beverages
- ✗ Catering food and beverages

As per PCA Duty Statement:

Refer to PCA Manual

Daily (before 10am) - Refrigerator / freezer checked for expired items and spills (to be cleaned immediately)

Sunday – Refrigerator cleaned, and freezer defrosted

EMPAC required for fridge maintenance

Refer to **Ward Refrigerator Management SOP** for additional information

Appendix II: Ward refrigerator poster

Ward Refrigerator

Staff food, patient water jugs and patient food are preferably located in dedicated refrigerators; this may not always be possible

Acceptable, with dedicated shelves for:

- ✓ Patient water jugs
- ✓ Sealed food, directly from catering distribution area
- ✓ Unopened, cold, catering food / beverages from patient tray -
Labelled with patient sticker, visible use by date
- ✓ Patient food and beverages, brought from home - Labelled with
patient sticker and date placed in refrigerator. Consume within 3
days [Day 1 = date item placed in refrigerator]
- ✓ Cold packs stored in a dedicated, sealed container

Do not store in refrigerator

- ✗ Opened catering food and beverages
- ✗ Hot catering food
- ✗ Frozen food
- ✗ Raw proteins (for example, raw eggs)

As per PCA Duty Statement:

Refer to PCA Manual

Daily (before 10am) - Refrigerator / freezer checked for expired items and spills (to be cleaned immediately)

Sunday – Refrigerator cleaned, freezer defrosted, and cold pack container decontaminated in dishwasher

EMPAC required for fridge maintenance

Refer to **Ward Refrigerator Management SOP** for additional information

Ward Refrigerator Management SOP

Version: 2.0

Endorsed by: Coordinator of Nursing, IPM 08/04/2024

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Ward fridge & staff fridge weekly

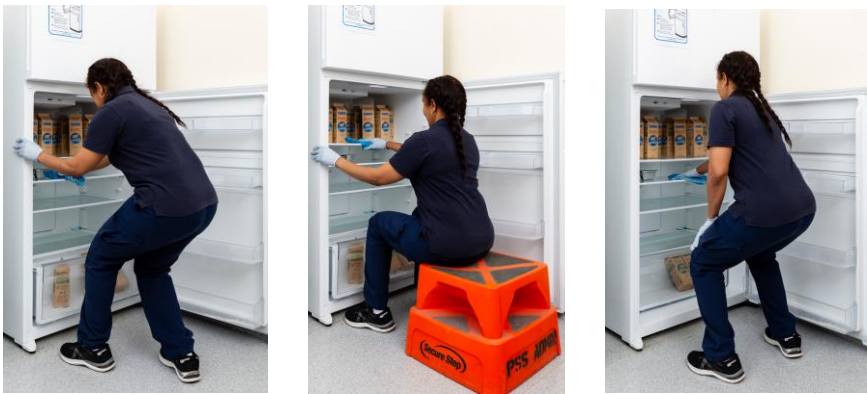
Weekly

Equipment:

- Cleaning cloth (blue)
- Cleaning gloves
- 4.8L bucket (green)
- Asset JF
- Rubbish bag

See also Ward Refrigerator Management SOP:

<https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/EMHS/RPH/Ward%20Refrigerator%20Management%20SOP.pdf>

Procedure / Steps	Safety Points
1. Turn off power supply to the fridge 2. Perform hand hygiene 3. Apply gloves 4. Open fridge door 5. Check with nursing staff if it is safe to dispose of any mouldy or out of date contents from fridge 6. Remove the rest of the contents of the fridge. <i>Note: cold pack storage containers – container and lid must be washed in dishwasher.</i> 7. With a damp, not wet, blue cleaning cloth, wipe over all surfaces inside the fridge	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture
	
8. See Microfibre cloths 8 sided fold method and Damp dusting procedures 9. Ensure freezer area and any ice is defrosted	

10. Wipe over seal and outer surface of the fridge ensuring seals are clean of any loose debris
11. Once all surfaces are dry return contents to fridge
12. Close fridge door
- 13. Turn power supply back on**
14. Once finished, dispose of cleaning cloth/s appropriately. Blue cloths are launderable
15. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
16. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry
17. Remove and dispose of gloves
18. Perform hand hygiene



Asset JF

Exchange gloves and perform hand hygiene in-between cloths



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Lamson chute - Pneumatic Tube System

As required

Equipment:

- Lamson carrier tube
- Foam padding

See also Pneumatic Tube System: Use of Policy (RPH):

<https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/EMHS/RPH/Use%20of%20Pneumatic%20Tube%20System%20Policy.pdf>

Procedure / Steps	Safety Points
All relevant staff who work in a ward/unit where a station is located will be trained on the Pneumatic Tube System (PTS) guidelines The designated Station Owner is responsible to ensure that all new staff are trained in the system. - Staff are to have consideration for other department/wards needs as there will be times when an urgent specimen or an urgent request for blood is required. - Staff must follow the operating instructions for the PTS at all times. - Operators at each station are to return empty carriers belonging to other stations to their origin as soon as possible. All carriers have station numbers engraved on the carrier e.g. number 45 equates to station 45 which is the Intensive Care Unit (ICU). - Any carrier sent to the wrong station must be returned to the home station as quickly as possible. - Operation of the system uses a “traffic light” method. Ask Nurse Coordinator what type of specimen or item is being sent and whether it can be sent via the Lamson Pneumatic Tube System (See notes below – what can and can not be sent via PTS) - Perform hand hygiene - Collect an empty carrier tube and sandwich the specimen or item inside so that each end of the tube has foam padding inserted - Use only Lamson™ supplied carriers	 Do not use unless trained by the designated station owner See Extra notes in this procedure 

9. Pathology specimens must first be placed in approved specimen container(s) with lids and tubing firmly attached and then into the biohazard Ziploc bag (ensure properly closed /sealed). Appropriate request form(s) are to accompany the specimen(s)
10. Pad the delicate items to avoid breakage. Foam inserts required at both ends when transporting clinical samples
11. Inspect the velcro seals on carriers weekly and prior to use - before sending. A carrier with worn seals is **not** to be used. This reduces the speed of travel and can block the system. If the seals are worn out, contact the Shift Engineers for replacement
12. Slide carrier cap sideways to open the lid
13. Load items carefully into the carrier ensuring the items are sealed and foam inserts are placed at both ends as required; being mindful not to overload the carrier
14. Close the lid on the carrier securely, until it snaps shut. Ensure that the bag is not caught in the cap as it would interfere with the efficiency of transport and perhaps result in blockages
15. Before loading the carrier into the system dial the desired destination address on the keypad, as the last address used remains on the station
16. See the directory on the front of the station
17. Place the carrier to be sent in the send platform. Depending on the traffic in the carrier system the relevant LED's will light up. When 2 lights (yellow and green light up) it indicates that the carrier is now ready to be sent
18. Sending will commence automatically as soon as the system is available. Depending on line queue, the station may take a few minutes before sending the device
19. When the carrier has been selected, the green light turns off. The yellow light remains laminated until the carrier arrives at its destination
20. When the carrier arrives at its destination all lights turn off
21. Arrived carriers must be collected from the collection baskets at regular intervals or the system will jam
22. Empty carriers must be returned as soon as possible to the home station as identified on the carrier.



Ensure item to be sent is compatible with Pneumatic Tube System (PTS)



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

23. Perform hand hygiene



Extra notes:

Items that CAN be sent via the PTS

- **Clinical Samples for Pathology** All patient samples with the exception of those listed as below in “Pathology”.
- **Blood for transfusion** Red cells, plasma, platelets and cryoprecipitate.
- **Pharmacy** Prescriptions to Pharmacy with the exception of those listed as below in “Pharmacy”

Items that CANNOT be sent via the PTS

- Items exceeding **750g** in weight
- Glass items not able to be padded adequately to avoid breakage
- Personal property and/or valuables including food or sweets
- Any items which require packaging in ice / dry ice
- “Sharps” for example, needles.

Pathology items that CANNOT be sent via the PTS

- Histopathology samples in formalin
- Blood collection tubes that have not been fully drawn or have been filled using a needle and syringe (these items are prone to lysis, that is dissolution or decomposition, if transported via the Lamson carrier)
- A blood specimen if the collection tube top has been removed (thus losing its vacuum).
- Glass blood culture bottles
- Specimens to be maintained at 37° C
- All blood products in glass containers (vials) for example albumen, intravenous immuno-globulins etc
- Used blood packs

Pharmacy items that CANNOT be sent via the PTS – (this list is not exhaustive). As per **R PBG Medication Administration Clinical Practice Standard:**

- Items requiring refrigeration
- Items that cannot be shaken vigorously e.g. carbonated substances, proteins such as monoclonal antibodies ('mabs'), peptides such as insulins
- Fragile items e.g. lanthanum, effervescent tablets. Glass items must be padded adequately to avoid breakage
- Items manufactured by pharmacy, e.g. sterile and chemotherapy items
- Hazardous items e.g. inhalation anaesthetics, cytotoxic items
- Enoxaparin (as per manufacturer)
- Items with limited/restricted supply e.g. clinical trial items, Special Access Scheme items, S4R / S8 items
- Expensive items
- Heavy / large size items – more than 750g (as per Shift Engineer)

DO

- Ensure accompanying documentation is included in the delivery

• Ensure biological material/clinical samples comply with triple packaging regulations • <i>Return the carrier to the sending station as soon as possible.</i> DO NOT • Place items in the system unless they are within a carrier • Use a damaged carrier • Overload the carrier, use two carriers sent separately where required	
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Lift

Daily & as required

Equipment:

- Gloves
- Charge bucket (grey)
- 4.8L bucket (blue)
- Asset JF
- Cleaning cloth (blue)
- Wet floor / hazard signs
- Mop pad (blue)
- Suma Inox D7.1 polish
- Extendable dolly mop

Procedure / Steps	Safety Points
1. Ask the housekeeping supervisor for lift lock key # 45 2. Place wet floor / hazard signs in lift and lock lift off when cleaning to ensure safety of yourself and others 3. Perform hand hygiene 4. Apply gloves 5. With a damp, not wet, blue cleaning cloth, wipe over lift walls. See Wall, Microfibre cloths 8 sided fold method and Damp dusting procedures 6. Wipe over stainless-steel handrails with Suma Inox D7.1 polish. Suma Inox D7.1 polish is for the stainless-steel handrails only. 7. Damp mop floor of lift. See Damp mopping procedure 8. If applicable, clean mirrors. See Glass and mirror procedure 9. Once finished, dispose of cleaning cloths appropriately. Blue cloths are launderable 10. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets 11. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry. 12. Send blue mop pad for laundering 13. Remove and dispose of gloves 14. Perform hand hygiene 15. When floor is dry remove wet floor / hazard signs and unlock lift 16. Perform hand hygiene	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 



Asset JF



Suma Inox D7.1



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Spills – general and body fluids

As required

Equipment:

- Sodium Hypochlorite 0.125%
- Wet floor / hazard signs
- 'Bluey'
- Gloves
- Asset JF
- Mop pad (yellow)
- Cleaning cloth (yellow)
- Apron/gown
- Bucket
- Safe Sorb
- Charge bucket (yellow)
- Safety goggles

See also Occupational Exposure to Blood/Body Fluids Management SOP:

<https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/EMHS/RPH/Occupational%20Exposure%20to%20Blood%20Body%20Fluids%20Management%20SOP.pdf>

Procedure / Steps	Safety Points
Body fluids are described as any fluid (for example urine, blood, faeces, vomit) excreted by the body In the clinical areas, it is the nurse's responsibility for the initial clean up of any body fluids unless there is an emergency situation. After the nurse has carried out the first clean, it is then the responsibility of the cleaner / PCA to carry out the final clean after the bulk has been removed In non-clinical areas, where a nurse or a mid wife is not in attendance, it is the responsibility of the Patient Support Services staff member to clean up any body fluids. The affected area is to be cordoned off for safety reasons 1. Perform hand hygiene 2. Apply gloves and other PPE as required by standard precautions. 3. Place wet floor / hazard signs around the spill in prominent areas 4. If required Sprinkle Safe Sorb onto fluids, wipe up gel residue with a disposable yellow cleaning cloth and gather with 'bluey' and dispose 5. The location of the spill will determine if a cloth or mop should be used 6. Half fill yellow bucket with Asset JF	 Safe Sorb

7. Wet mop/cloth in Asset JF solution and gently squeeze until only damp
8. See [Microfibre cloths 8 sided fold method](#) and [Damp dusting](#) procedures
9. With a damp, not wet, yellow mop or yellow cleaning cloth, wipe up the spill.
10. Starting at the outer edge working your way towards the centre of the spill
11. Exchange gloves and perform hand hygiene in-between cloths
12. Dispose of yellow cloths and mop pads as needed in waste bag
13. Clean area again using 0.125% Sodium Hypochlorite
14. When floor / surface is clean and dry empty cleaning solution into sluice
15. Once finished, dispose of yellow cleaning cloths, empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry.
16. Remove and dispose of PPE appropriately
17. Perform hand hygiene
18. When floor is dry remove wet floor / hazard signs
19. Perform hand hygiene



Sodium Hypochlorite
0.125%



Asset JF



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Stairwell

Daily

Equipment:

- Asset JF
- Dust pan & brush
- Gloves
- Wet floor / hazard signs
- Static mop (dry cloth)
- Charge bucket (grey)
- Mop pad (blue)

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Place wet floor / hazard signs in highly visible areas 3. Static sweep stairs starting from top and working your way down to the bottom. See Static sweeping procedure 4. Pick up rubbish and debris with dust pan and brush 5. With a damp, not wet, blue cleaning cloth, wipe handrails and bannisters. See Damp dusting procedure. Starting from top and working your way down to the bottom 6. Damp mop stairs and landing with blue mop pad. Starting from top and working your way down to the bottom. See Damp mopping procedure 7. Once finished, dispose of cleaning cloths. Blue cloths are launderable 8. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets 9. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry. 10. Send blue mop pads for laundering at the end of each shift 11. Return equipment to cleaners' cupboard 12. Perform hand hygiene 13. Once floor is dry remove wet floor / hazard signs and return to storage area 14. Perform hand hygiene	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 
Vent cleaning, weekly or as required in stairwells, see Vent - door/ceiling procedure	



Asset JF



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Storeroom

Storeroom with shelving Weekly

Equipment:

- Gloves
- Platform Safety Step Ladder
- 4.8L bucket (blue)
- Cleaning cloth (blue)
- Trolley
- Tuffie wipes

See also **Storage of Sterile & Non-Sterile Stock in Clinical Areas SOP:**

<https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/EMHS/RPH/Storage%20of%20Sterile%20and%20Non-Sterile%20Stock%20in%20Clinical%20Areas%20SOP.pdf>

Procedure / Steps	Safety Points
On Saturday afternoons, ward PCA's are to prioritise this task unless required for a Hospital Emergency e.g. Code Blue. Nursing teams on wards are aware and can utilise CARPS for support where required: • Generally completed Saturday afternoon 1600 – 1800 hours • Complete approximately one quarter of the stock weekly, which equates to all stock monthly • Ensure stock rotations principles are adhered to – items should be returned to clean locations with oldest stock at the front / on top, newest stock at the back 1. Write name as “unavailable – storeroom cleaning” on the whiteboard whilst storeroom cleaning 2. Perform hand hygiene 3. Collect platform safety step ladder and a trolley. Inspect ladder and trolley to ensure they are safe to use. See Ladders - Safety Alert 4. Ensure trolley is cleaned before use 5. Perform hand hygiene 6. In the storeroom work in small sections from top to bottom and side to side. Use platform safety step ladder, avoid overreaching. 7. Remove pigeon boxes and products from shelf to the trolley as required to create adequate working space	 Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture

8. Perform hand hygiene
9. Apply gloves
10. With a damp, not wet, blue cleaning cloth, wipe over surfaces and remove all dust and dirt
11. See [Microfibre cloths 8 sided fold method](#) and [Damp dusting](#) procedures
 - Tuffie wipes can be used as an alternative to Microfibre cloths in order to access the inner corners of the pigeon boxes
 - Do not use Oxivir wipes or Isowipes
12. Empty pigeon boxes, placing products on a clean surface. Wipe pigeon boxes thoroughly and place them on a clean surface



13. Wipe the shelves. Remove and dispose of gloves. Perform hand hygiene
14. When pigeon boxes are dry, with clean hands replace products in the box
15. Return dried pigeon boxes with products neatly to their original location on shelf. Stock should not be on left the floor. If there is excess stock that cannot be accommodated off the floor, advise Nursing staff
16. Once finished, dispose of cleaning cloth/s appropriately. Blue cloths are launderable
17. Perform hand hygiene and apply new gloves before emptying and cleaning buckets



Asset JF



Exchange gloves and perform hand hygiene in-between cloths



Tuffie wipes can be used as an alternative to Microfibre cloths in order to access the inner corners of the pigeon boxes.

Do not use Oxivir wipes or Isowipes

18. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry.
19. Remove and dispose of gloves
20. Perform hand hygiene

Make a note of any items that are out of stock and present it to the Nurse Coordinator for ordering

Complete approximately one quarter of the stock weekly. Equates to all stock monthly



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Storeroom with wire racking

Weekly

Equipment:

- Gloves
- 4.8L bucket (blue)
- Cleaning cloth (blue)
- Asset JF
- Platform Safety Step ladder

See also Storage of Sterile & Non-Sterile Stock in Clinical Areas SOP:

<https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/EMHS/RPH/Storage%20of%20Sterile%20and%20Non-Sterile%20Stock%20in%20Clinical%20Areas%20SOP.pdf>

Procedure / Steps	Safety Points
On Saturday afternoons, ward PCA's are to prioritise this task unless required for a Hospital Emergency e.g. Code Blue. Nursing teams on wards are aware and can utilise CARPS for support where required: • Generally completed Saturday afternoon 1600 – 1800 hours • Complete approximately one quarter of the stock weekly, which equates to all stock monthly • Ensure stock rotations principles are adhered to – items should be returned to clean locations with oldest stock at the front / on top, newest stock at the back 1. Write name as “unavailable – storeroom cleaning” on the whiteboard whilst storeroom cleaning 2. Perform hand hygiene 3. Collect platform safety step ladder and a trolley. Inspect ladder and trolley to ensure they are safe to use. See Ladders - Safety Alert. 4. Perform hand hygiene and apply gloves 5. With a damp, not wet, blue cleaning cloth, wipe over surfaces of wire racking and wire baskets thoroughly to remove all dust and dirt. 6. Work in small sections from top to bottom and side to side. Use platform safety step ladder and avoid overreaching. See Microfibre cloths 8 sided fold method and Damp dusting procedures	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture   Asset JF

- Stock should not be on left the floor. If there is excess stock that cannot be accommodated off the floor please advise Nursing staff.
7. Once finished, dispose of cleaning cloth/s appropriately. Blue cloths are launderable
 8. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
 9. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry.
 10. Remove and dispose of gloves
 11. Perform hand hygiene

Make a note of any items that are out of stock and present it to the Nurse Coordinator for ordering



Exchange gloves and perform hand hygiene in-between cloths

Complete approximately one quarter of the stock weekly. Equates to all stock monthly



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Storeroom with wire racking – intensive clean

As required

Equipment:

- Gloves
- 4.8L bucket (blue)
- Cleaning cloth (blue)
- Asset JF
- Platform Safety Step ladder
- Trolley

See also Storage of Sterile & Non-Sterile Stock in Clinical Areas SOP:

<https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/EMHS/RPH/Storage%20of%20Sterile%20and%20Non-Sterile%20Stock%20in%20Clinical%20Areas%20SOP.pdf>

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Collect platform safety step ladder and a trolley. Inspect ladder and trolley to ensure they are safe to use. See Ladders - Safety Alert 3. Ensure trolley is cleaned before use 4. Perform hand hygiene 5. In the storeroom work in small sections from top to bottom and side to side. Use platform safety step ladder, avoid overreaching. 6. Remove wire baskets (including products) from wire racking to the trolley as required to create adequate working space 7. Perform hand hygiene and apply gloves 8. Empty wire baskets, placing products on a clean surface. 9. See Microfibre cloths 8 sided fold method and Damp dusting procedures 10. Wipe thoroughly to remove all dust and dirt, with a damp, not wet, blue cleaning cloth: 11. Wipe the wire racking 12. Wipe the wire baskets and place them on a clean surface 13. Remove gloves and perform hand hygiene 14. When wire baskets are dry, with clean hands replace products in the basket 15. When wire racking is dried, return dried wire baskets with products neatly to their original location on the wire racking	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 

16. Continue working in small sections from top to bottom and side to side
17. Once finished, dispose of cleaning cloth/s appropriately. Blue cloths are launderable
18. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
19. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry.
20. Remove and dispose of gloves
21. Perform hand hygiene

Make a note of any items that are out of stock and present it to the Nurse Coordinator for ordering



Asset JF



Exchange gloves and perform hand hygiene in-between cloths



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Tearoom

Daily

Equipment:

- Gloves
- Asset JF
- Charge bucket (grey)
- Cleaning cloth (blue)
- Scourer
- Mop pad (blue)
- 4.8L bucket (green)
- Cream Cleanser R7
- Wet floor / hazard signs

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply gloves 3. Half fill green bucket from Asset JF dispenser 4. With a damp, not wet, blue cleaning cloth, wipe over counter top and surrounds such as basin/sink, refrigerator, walls and shelving. See Microfibre cloths 8 sided fold method, Damp dusting, Basin - hand, Wall and Countertop, benchtop and desk procedures  5. Use a small amount of Cream Cleanser R7 and scourers remove any calcium build up or coffee stains around taps and plug hole in the sink 6. Then rinse with clean warm water 7. Sweep the floor to remove all loose debris. See Static sweeping procedure 8. Mop floor. See Damp mopping procedure 9. Once finished, dispose of scourer and cleaning cloths appropriately. Blue cloths are launderable 10. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets	 Exchange gloves and perform hand hygiene in-between cloths  Asset JF Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions

11. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry.
12. Remove and dispose of gloves
13. Perform hand hygiene

Keep arms close to body

Avoid stooped posturing

Keep neck in neutral posture



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Urinal – public

3 – 4 times daily

Equipment:

- Gloves
- 4.8L bucket (red)
- Safety Goggles
- Cleaning cloth (red)
- Mop pad (red)
- Wet floor/hazard signs
- Asset JF
- Room Care R2L

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Display wet floor/hazard signs 3. Perform hand hygiene and apply goggles 4. Perform hand hygiene and apply gloves 5. Flush urinal and using toilet brush scrub all internal surfaces with Go Getter  6. Flush and rinse away 7. With a damp, not wet, red cleaning cloth wipe all external surfaces of the urinal and spot clean walls as required. See Microfibre cloths 8 sided fold method, Wall and Damp dusting procedures 8. Remove and dispose of gloves, perform hand hygiene 9. Remove goggles and perform hand hygiene 10. Clean goggles	 Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 

11. Mop the area around the toilet. Mop floor using grey charging bucket, red mop handle & red mop pads. See [Damp mopping](#) procedure
12. Send mop pad for laundering
13. Dispose of cleaning cloth/s appropriately. Red cleaning cloths are launderable
14. Perform hand hygiene and apply new gloves before emptying and cleaning buckets
15. Empty bucket/s, clean with Asset JF and turn bucket/s upside down to dry
16. Remove and dispose of gloves
17. Perform hand hygiene



Asset JF



Go Getter



Exchange gloves and perform hand hygiene in-between cloths



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Utensil washer Miele model G7762

As required

Equipment:

- Gloves
- Safety Goggles
- Asepti-Clean
- Asepti-Rinse

Procedure / Steps	Safety Points
PERSONNEL OPERATING THIS MACHINE MUST FIRST BE TRAINED BY A PSS SUPERVISOR OR DELEGATED AUTHORITY Please note: If your machine is fitted with a water softener see your supervisor for checking and filling instructions as the settings must correspond to the water supply hardness. 1. Perform hand hygiene and apply goggles 2. Perform hand hygiene and apply gloves 3. Load machine with utensils so that water can access all surfaces. I.e. hollow items are upside down with the tall ones in the middle of the machine 4. Put deep based items at an angle so that water can run off 5. Do not obstruct the rotation of the spray arm 6. Put small lightweight and delicate items in the top basket 7. Put large heavy items in the bottom basket 8. Put cutlery in a cutlery basket with handles pointing downwards 9. Select wash program: • Intensive program: For heavily soiled items • Normal program: For general cleaning needs • Delicate program: For more fragile items 10. Press the "Start" button once the program is selected. Do not open the door mid-cycle: Avoid opening the washer door while a cycle is running, as this could cause water leakage or interrupt the cycle 11. Remove and dispose of gloves 12. Perform hand hygiene	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture  Do not wash bedpans and urinals in this washer

13. Remove goggles and reprocess appropriately (goggles could be reusable or disposable)
14. Perform hand hygiene

DETERGENT and RINSE AID

1. Perform hand hygiene
2. Check sight glass for detergent level. Add Asepti - Clean when indicator light comes on
3. Check Rinse – Aid level. Add Asepti – Rinse Clear if needed. If level is not reducing notify your supervisor or phone Miele 9478 0469
4. Perform hand hygiene

WEEKLY:

Remove the filter in base of machine, clean and ensure it is replaced correctly



Asepti-Clean



Asepti-Rinse



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Utensil washer Miele model G7862

As required

Equipment:

- Gloves
- Safety Goggles
- Asepti-Clean
- Asepti-Rinse

Procedure / Steps	Safety Points
PERSONNEL OPERATING THIS MACHINE MUST FIRST BE TRAINED BY A PSS SUPERVISOR OR DELEGATED AUTHORITY Please note: If your machine is fitted with a water softener see your supervisor for checking and filling instructions as the settings must correspond to the water supply hardness. 1. Perform hand hygiene and apply goggles 2. Perform hand hygiene and apply gloves 3. Load machine with utensils so that water can access all surfaces. i.e. hollow items are upside down with the tall ones in the middle of the machine 4. Put deep based items at an angle so that water can run off 5. Do not obstruct the rotation of the spray arm 6. Put small lightweight and delicate items in the top basket 7. Put large heavy items in the bottom basket 8. Put cutlery in a cutlery basket with handles pointing downwards 9. Select wash program: • Intensive program: For heavily soiled items • Normal program: For general cleaning needs • Delicate program: For more fragile items 10. Press the "Start" button once the program is selected. Do not open the door mid-cycle: Avoid opening the washer door while a cycle is running, as this could cause water leakage or interrupt the cycle 11. Remove and dispose of gloves 12. Perform hand hygiene	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture  Do not wash bedpans or urinal bottles in machine

13. Remove goggles and reprocess appropriately (goggles could be reusable or disposable)
14. Perform hand hygiene

LIQUID DETERGENT and RINSE AID

1. If Rinse Aid is used, check the level and add until indicators are dark brown. Use a funnel if necessary and screw the caps back on.
2. Check indicator light for detergent level. Add Asepti - Clean when indicator light comes on or if FDO appears in display
3. Close door
4. Reset program dial to 12 o'clock
5. Reselect Vario TD
6. Push start button
7. Disinfection light will be on at the end of the program
8. Perform hand hygiene and apply gloves
9. Open door and remove utensils – **CAUTION, MAY BE HOT**
10. Remove and dispose of gloves and perform hand hygiene

WEEKLY:

Remove the filter in base of machine, clean and ensure it is replaced correctly



Asepti-Clean



Asepti-Rinse



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Wall

As required

Equipment:

- Gloves
- 4.8L Bucket (blue)
- Asset JF
- Cleaning cloth (blue)
- Cream R7

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply gloves 3. With a damp, not wet, blue cleaning cloth spot wipe marks on walls. See Microfibre cloths 8 sided fold method and Damp dusting procedures  4. If there are marks or stubborn soils on walls apply a small amount of Cream R7 to cleaning cloth and clean soiled area. 5. Wipe away any excess Cream R7 6. Once finished, dispose of cleaning cloth/s appropriately. Blue and red cloths are launderable, yellow disposed 7. Remove and dispose of gloves. Perform hand hygiene and apply new gloves before emptying and cleaning buckets 8. Empty and clean bucket/s with Asset JF. Turn bucket/s upside down to dry. 9. Remove and dispose of gloves 10. Perform hand hygiene	 Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture  Avoid over reaching, over stretching or twisting actions



Asset JF



Cream R7

Report any graffiti to
the Supervisor



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Water jugs

Twice daily – morning & afternoon

Equipment:

- Trolley
- Stride Citrus Neutral cleaner
- Water jugs
- Dishwasher

Procedure / Steps	Safety Points
At no times are used and clean jugs to be on the same trolley 1. Collection of used jugs • In ward areas, attend to non-isolation rooms first 1. Perform hand hygiene 2. With clean hands collect used jugs and cups from each patient in the room and place on a clean trolley. Repeat for each room 3. Perform hand hygiene 4. Attend to isolation rooms after non isolation rooms 5. Apply PPE as per isolation card. Collect used water jug and cup 6. To bring used jug out of isolation room: ○ Staff in room to remove their PPE as per isolation card (keep mask & eye protection on if applicable) ○ Perform hand hygiene before picking up jug and exiting isolation room OR ○ Staff inside isolation room may pass used jug(s) to PCA outside room ○ Place used jug on shelf of trolley outside room. ○ Perform hand hygiene • Repeat steps 5 & 6 for each isolation room • Perform hand hygiene and wheel trolley to pantry	Wherever possible, nursing staff should be consulted before giving or taking water jugs from patients. This will assist the nurse in measuring fluid intake. At no times are used and clean jugs to be on the same trolley Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture

2. Dishwasher

See *Dishwasher Operational Instructions in PSS Procedure Manual*

- Check if dishwasher is filled with water and temperature programmed to WASH <60°C & RINSE <82°C. Run the dishwasher twice whilst loading the basket (to ensure correct temperature is reached prior to washing)
- Empty all remaining content of jugs into sink
- Prepare basket with used jugs, lids and glasses. Also include the ice scoop, ice scoop container & lid.
- DO NOT overcrowd. Space lids to allow effective washing.
- Slide basket into dishwasher and close dishwasher door
- Perform hand hygiene
- Select required program on dishwasher and activate wash (where applicable). Ensure full wash and rinse cycles completed.
- Clean/wipe down benches and dishwasher handles with Catering Stride Citrus Neutral cleaner before leaving pantry
 - Perform hand hygiene, apply new gloves and clean trolley with Stride Citrus neutral cleaner and single use paper towel
- When cycle is finished place water jugs and glasses on to a clean trolley on the top shelf
- Inspect jugs and lids for cracks/damages and inform PSS Supervisor of damages / discards. PSS Supervisor to advise Catering so the discarded jugs and lids can be replaced. Catering Management Team ext 42146 (0600-2030 hours)

Place the ice scoop & ice scoop container & lid through dishwasher prior to use



Stride Citrus Neutral cleaner

Inspect the jugs for cracks/damage

At no times are used and clean jugs to be on the same trolley



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

3. Filling jugs

- Perform hand hygiene
- Fill jugs with ice using ice scoop (3/4 filled)
 - See "Ice Machines"



- For pantries that have a second sink, utilise the sink near the window for filling water jugs. - Prepare tap by wiping the tap nozzle with alcohol wipes and let the tap run for 30 seconds before starting to fill jugs to clear the line - Scrub and rinse the sink - Fill each water jug with water - Place clean filled water jugs and cups on trolley - Fit lid onto jug, ensure effective and secure seal - Place a clean cup on top of each lid, upside down - Perform hand hygiene	At no times are used and clean jugs to be on the same trolley
4. Prior to leaving catering pantry - Clean/wipe down benches and dishwasher handles with Catering Stride Citrus Neural cleaner before leaving pantry - Ensure floors are dry (i.e. no ice and/or splash on floor). Use a CAUTION sign as needed. Note: Do Not use any external ward chemicals in the catering areas. Refer to Catering Management Team if any queries on ext 42146 (0600-2030 hours)	 DO NOT line trolley with 'Roar' paper towel or other lining. No coverings on top or underneath the water jugs At no times are used and clean jugs to be on the same trolley
5. Distribution of clean jugs - In ward areas, attend to non-isolation rooms first. - Perform hand hygiene. With clean hands deliver a clean jug to each patient in the room Do not give water jugs to fasting patients (nil by mouth) <u>Check with the nurse if unsure or patient on thickened fluids</u> - Attend to isolation rooms after non isolation rooms - Apply PPE as per Isolation Room card. Take in clean water jug and cup - Exit room, removing PPE as per Isolation Room card	 If possible, nursing staff should be consulted before giving or taking water jugs from patients. This will assist the nurse in measuring fluid intake At no times are used and clean jugs to be on the same trolley

Dishwasher instructions

Procedure / Steps

TO FILL – check inside dishwasher that it is empty & filter trays are in correct position.

– Close door – press  (do not hold down this button-just touch to start)
(filling machine takes approx 20 mins)

– Open door to check that water has filled in dishwasher before using.

TO WASH – Close open door – Machine will start automatically
(washing is completed when there is no sound from the dishwasher)

DO NOT OPEN dishwasher **door** until **full cycle** (wash & rinse) is **completed**

TO EMPTY – press  and hold for **3 seconds**
(Button will flash & stops flashing when machine is empty)

– Leave door open to air dry

– Clean filter trays & inside of dishwasher

IMPORTANT NOTES

- Wash cycle-60°C & above Rinse cycle-82°C & above
- Record temperature on dishwasher temperature sheet
- If dishwasher chemicals (under bench) are running too low phone 42146 & advise Catering supervisor
- Change water in dishwasher after Morning Tea, Afternoon Tea & Supper
- **DO NOT** line trolleys with 'roar' (paper towel) or use to cover crockery or jugs (use trays as detailed in pictures below)



Ice machine

HAZARD ICE SPILL ON FLOOR!

Ensure all ice has been picked up from the floor
Use of mop may be required
Place caution sign

Procedure / Steps

ALL STAFF

1. Perform HAND HYGIENE before using ice scoop to collect ice (**do not use gloves**)
2. Hold the ice scoop by the handle. Do not touch other parts of the scoop.
3. **DO NOT USE Jugs, cups or bare hands to scoop ice from machine.**
4. Ensure ice scoop is placed back inside container without touching any other part of the scoop. Close the lid of the container .
5. Unused ice must not be returned to the ice holding compartment.

FILLING UP WATER JUGS

Please follow WATER JUGS procedure from PCA Manual

Wash all jugs via the pantry dishwasher (wash scoop & container as well)

HAND HYGIENE before using ice scoop to collect ice

USE SINK TAP not HAND BASIN TAP to fill water jugs



Cleaning of ice machines is performed by Catering Department staff

WASTE

Key safety principles:

Handling waste in a hospital setting, including linen, rubbish, infectious waste, and sharps containers, requires careful attention to body mechanics to avoid musculoskeletal injuries. When lifting or moving waste, always bend at the knees, not the waist, and engage your leg muscles to perform the lift rather than relying on your back. Keep the load close to your body to reduce strain on your spine and shoulders, and avoid twisting while lifting or carrying waste. When handling heavy or awkwardly shaped items, such as large linen bags or sharps containers, seek assistance from a colleague whenever possible to share the load and reduce individual strain. Additionally, ensure that waste containers are at an appropriate height to avoid bending or over-reaching, and always use proper lifting equipment, such as trolleys or carts, when transporting larger loads.

To further reduce the risk of musculoskeletal injury, make use of ergonomic tools and techniques. Use waste bins or carts with adjustable heights to ensure that lifting and depositing waste can be done comfortably without excessive bending. When pushing or pulling carts with waste, engage your core muscles and keep your back straight, using your legs to initiate the movement. If you need to handle sharps containers, wear proper gloves and be cautious of the weight and shape to prevent awkward handling. Take regular breaks to stretch, particularly your back, shoulders, and wrists, to alleviate muscle tension from repetitive lifting and moving. By practicing safe lifting techniques, using ergonomic equipment, and incorporating breaks into your routine, you can significantly reduce the risk of musculoskeletal injury while handling hospital waste.



Glass recycling box

Check daily and dispose when maximum 2/3 full

Equipment:

- Glass recycling box
- Clear plastic liner
- Tape

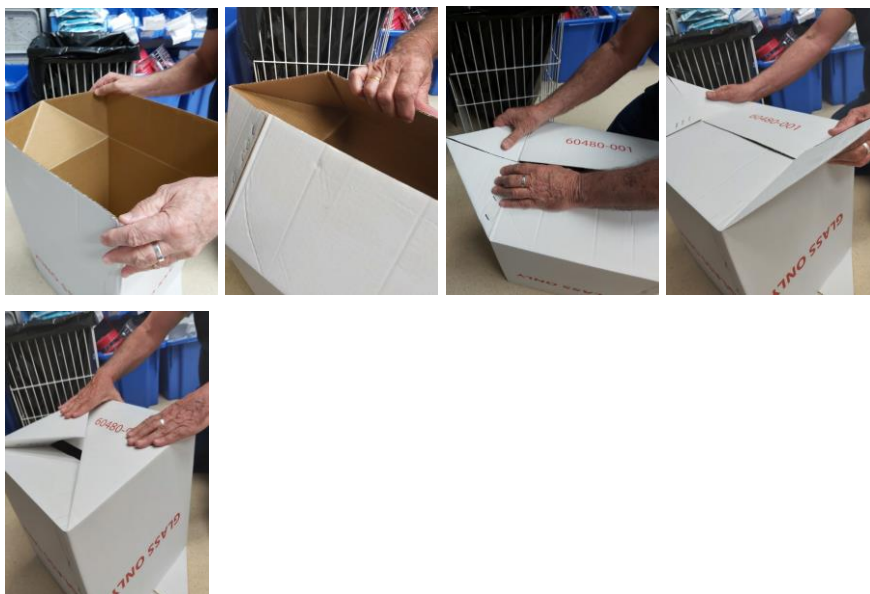
Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Visually inspect the contents to confirm only glass will be recycled. Should there be any clinical items present: • <u>DO NOT attempt to decant clinical items</u> • The whole box should be disposed into a clinical waste bin • Advise a senior member of Nursing staff that clinical items were present 3. Double tie both ends of the plastic box liner together 4. Fold down box lid along the marked grooves 5. Ensure lid closed with tabs secure  6. Use handles to carry the box to appropriate disposal area (if you are unsure ask your supervisor where it is)  7. Perform hand hygiene	 Check daily and dispose when maximum 2/3 full Before handling the box, gently tap the box with your foot to check how heavy it is. If box is too heavy, ask for help when handling Use a trolley to transport the box Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing

8. Make up a new glass box

9. With box upright, fold what will become the lid and carry handles



10. Turn the box upside down and fold what will become the base



11. Tape the base to secure it

Keep neck in neutral posture



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.



12. After box is made up and taped securely, turn back upright

13. Place a clear box liner in the box and return the box to the treatment room, with Glass Only signage on the box visible



14. Perform hand hygiene

Infectious waste - including cytotoxic waste

As required or when $\frac{3}{4}$ maximum full

Equipment:

- Gloves
- Cable ties
- Replacement bags (yellow)
- Dusting Mop
- Safety Goggles
- Room Care R2L

Procedure / Steps	Safety Points
1. Report incorrectly disposed rubbish to the Ward Coordinator or Patient Support Services Supervisor 2. Perform hand hygiene and apply goggles 3. Perform hand hygiene and apply gloves 4. Remove existing bag 5. Use a cable tie to secure the opening of the bag 6. Double bag the rubbish (put the waste bag into an empty waste bag to reduce the risk of leaks and spills) 7. Walk to allocated disposal point and dispose of rubbish bags	All clinical and cytotoxic waste bins must remain locked when in public areas. Keys are kept in each ward for PSS staff to use. Ensure that rubbish is held away from your body, to reduce the risk of injury or contamination Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture
  8. Squirt bin outside with Room Care R2L and wipe first 9. Wipe inside of bin with Room Care R2L and dusting mop 10. Dispose of dusting mop 11. Remove and dispose of gloves 12. Perform hand hygiene 13. Remove goggles and reprocess appropriately (goggles could be reusable or disposable) 14. Perform hand hygiene 15. Replace liner and perform hand hygiene	

All clinical and **cytotoxic** waste bins must remain locked when in public areas. Keys are kept in each ward for PSS staff to use.

Cytotoxic waste is colour coded purple and for PSS staff is treated the same as infectious waste - except it is placed in the **purple bin**



Room Care R2L



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Linen

Linen removal

As required when maximum $\frac{3}{4}$ full

Equipment:

- Gloves
- Replacement linen bag
- Plastic bags

Procedure / Steps	Safety Points
 Blue bags with slide sheets or slings MUST be sent to laundry daily General linen is sent to laundry when linen bags are maximum $\frac{3}{4}$ full 1. Perform hand hygiene 2. Gloves are required if the linen bag is soiled 3. DO NOT over fill linen bags 4. Linen bags must be maximum $\frac{3}{4}$ full 5. Take linen skip to designated collection area 6. Double tie drawstring to ensure it is secure 7. Remove through the trolley horizontally. DO NOT lift the bag up and out of the trolley. 	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture  Gloves are required if the linen bag is soiled Soiled or wet linen should be placed in plastic bags before being placed into linen skip bag

8. Place linen bag on designated linen trolley, stacking linen bags neatly



9. Return linen skip back to ward area
10. Tie clean linen bag on linen skip
11. Remove and dispose of gloves if used
12. Perform hand hygiene

DO NOT pick up a linen bag that is more than 3/4 full. Use another skip if there is an overflow.

PCAs do not decant linen. Nursing staff are to decant as needed. If overfilled raise with Supervisor and or Shift Coordinator. If issue continues, submit a CHoIR.

Remain upright and forward facing and bend knees when removing linen bags from skips

Blue bags **MUST** go to laundry daily



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Linen removal do's and don'ts

As required when maximum $\frac{3}{4}$ full

DO



Ensure all bags are tied off correctly

DON'T



No loose items are to be placed on trolleys



- Ensure that blue and pink bags are on top
- All cords are to the back and not the ground
- Fill one trolley before moving on to the next



- Do not allow cords to hang off the trolley or drag on the ground
- Placing bags on top of the trolley puts staff at risk of over extending
- Do not place blue and pink bags on the bottom of the trolley

Using the correct linen bag

- **Pink linen bags** – reject linen only mending / stained (no soiled linen) Must be tagged with site name
- **Grey linen bags** – general soiled Spotless linen
- **Lilac linen bags** – general soiled Spotless linen for Mental Health customers
- **Blue linen bags** – customer owned goods, uniforms

Ensure NO SHARPS or NON-LINEN items are put in the linen bags



Linen tie off - bag stand

As required when maximum $\frac{3}{4}$ full

How to Tie Off a Linen Bag fitted to a Bag Stand

1. Hold the string and insert underneath



2. Pull the string.



3. Insert the right side going to left side and bring down.



4. Pull the string.



5. To undo the knot – hold the bag and pull the tail.



Linen tie off - slip knot

As required when maximum $\frac{3}{4}$ full

How to Tie Off a Full Linen Bag using a Slip Knot

1. Pull the string while pushing the bag closed.



2. Loop the string around your hand



3. Insert the tail of the string inside the loop



4. Pull the tail through half way to complete the slip knot.



5. A correctly tied off linen bag



Transportation & handling of soiled linen - Hygiene Orderly

As required or when linen trolley full

Equipment:

- Rigger Gloves (optional)
- Empty Linen Trolley
- Safety Shoes
- High Visibility Vests

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply rigger gloves (optional), safety shoes and high visibility vests 3. Check trolley is in good working order then transport the empty Linen Trolley from Level 1, North Block, Linen Services 4. Inspect full Linen Trolley. See Linen procedure: • All linen bags must be maximum $\frac{3}{4}$ full • Drawstring on linen bag must be double tied to ensure it is secure • Linen trolley must be appropriately stacked, each linen bag on top of the other • The opening of each linen bag is to be placed towards the back of the trolley • Linen trolley not to be overloaded (i.e. not protruding above or out of trolley) 5. Remove full linen trolley from allocated central collection area on ward and place empty in same location 6. Transport linen trolley by PUSHING with both hands 7. Repeat process for each allocated area 8. Remove gloves 9. Perform hand hygiene	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture   See Supervisor for replacement of gloves. For example, when soiled with body fluids



TOWING TUG available for use on Level 1 & 4 ONLY for STAFF that have completed the Tug Training Program

NEVER PUSH AND PULL 2 TROLLEYS AT ONCE

Linen trolleys should never be pulled during transportation.

Only one linen trolley to be transported at a time.



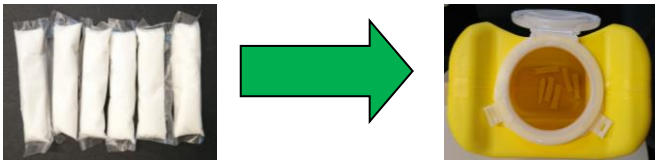
Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Medication disposal bin

As required when maximum $\frac{3}{4}$ full

Equipment:

- Gloves
- Required number of med-gel sachets
- Medication disposal bin

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Add required number of med-gel sachets to empty bin: DO NOT open med-gel sachets  3. Place TICK on the bin to indicate med-gel sachets have been added  4. The required number of med-gel sachets are:  1.4L = 1 sachet  8L = 4 sachets  5.1L = 3 sachets  17L = 6 sachets	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture Replace existing bins as required or when fill level reaches base of handle  Regularly check contents level in sharps containers

5. Once fill level reaches base of handle...



6. ...Perform hand hygiene and apply gloves. Seal bin



7. Discard bin into large clinical waste bin on ward areas



8. Remove and dispose of gloves
9. Perform hand hygiene
10. Replace bin

Only use the number of sachets as required

DO NOT open med-gel sachets



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Rubbish

Daily

Equipment:

- Tuffie Wipes/Room Care R2L
- Gloves
- Replacement bags

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply gloves 3. Check replacement bag is the right size 4. Replace existing rubbish bag when required or when no greater than $\frac{3}{4}$ full 5. Remove existing bag  6. Double tie the open end of the bag into a double knot 7. Wipe inside of bin with Tuffies / Room Care R2L 8. Replace bin liner 9. Do not place rubbish bins underneath sharps containers 10. Walk bags to and place in central area away from traffic flow to minimize hazard 11. When you have finished emptying the bins collect gathered rubbish and place in bin at allocated disposal point 12. Remove and dispose of gloves 13. Perform hand hygiene	Do not place rubbish bins underneath sharps containers Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture  Report incorrectly disposed rubbish to the Ward Coordinator or Patient Support Services Supervisor



Room Care R2L



Tuffie Wipes



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Sharp container

As required when maximum $\frac{3}{4}$ full

Equipment:

- Sharps container
- Key for sharps container is located on the ward PCA trolley
- Gloves

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply gloves 3. Lock flap of container into place: <u>Ensuring that it is securely sealed</u> 4. Unlock sharps container (if wall mounted) 5. Place sealed containers in lockable yellow (clinical waste) bins in designated disposal area 6. Remove and dispose of gloves 7. Perform hand hygiene 8. Replace with empty sharps container 9. Perform hand hygiene Additional safety points: • All clinical and cytotoxic waste bins must remain locked when in public areas. Keys are kept in each ward for PSS staff to use • Ensure waste bins are not placed under sharps containers • For everyone's safety, always dispose of sharps containers correctly	Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture Replace existing sharps container as required or when no greater than $\frac{3}{4}$ full  Regularly check contents level in sharps containers





Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Waste transportation and handling - Hygiene Orderly

As required or when waste skip full

Equipment:

- Rigger Gloves (optional)
- Safety Shoes
- High Visibility Vests
- Tuffie Wipes

Procedure / Steps	Safety Points
1. Perform hand hygiene 2. Apply rigger gloves (optional), high visibility vests and safety shoes 3. Empty waste skips are stored in A or R Block Courtyards. Wipe over top ends of an empty Waste Skip using Tuffie detergent wipes 4. Transport the empty Waste Skip to central collection area on Ward 5. Prior to removing the full waste skip ensure that the skip is not overloaded to allow for clear visibility over the skip during transportation 6. Remove full waste skip from allocated central collection area on Ward and place empty skip in same location 7. Transport Waste Skip by PUSHING with both hands. NEVER PUSH AND PULL 2 BINS AT ONCE 8. Repeat process for each allocated area 9. Remove gloves if used 10. Perform hand hygiene	PUSHING with both hands. NEVER PUSH AND PULL 2 BINS AT ONCE Always consider correct body postures to reduce risk of injury Maintain normal spinal posture at all times. Adopt appropriate posture to perform task safely Maintain wide base of support Avoid overreaching, over stretching or twisting actions Keep arms close to body Avoid stooped posturing Keep neck in neutral posture 



NEVER PUSH AND PULL 2 BINS AT ONCE



See Supervisor for replacement of gloves. For example, when soiled with body fluids



Tuffie

Different waste types should never be mixed

Waste skips should never be pulled during transportation.

Only one waste skip to be transported at a time.

Nothing other than waste should be transported in waste skip



Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

OTHER TASKS

Blood collection for transfusion

As directed by nursing

Equipment:

- Documentation

Procedure / Steps	Safety Points
1. Nursing staff will initiate the collection of blood / blood products from the Transfusion Medicine Unit (TMU) which is located on level 2 North block 2. The ward nursing staff will provide a form to be delivered to the TMU 3. It is the Nurse's responsibility to ensure the form has a: • patient identification sticker • ward (location) • product required If any of this information is missing, please advise the nursing staff to complete it before leaving the ward 4. Take the form directly to TMU 5. Hand the form to staff at the front desk and wait until you receive the blood / blood product 6. If the desk is unattended or it is after hours, press the buzzer that is located on the right-hand side of the window 7. If no response, follow the instructions on the wall next to the telephone 8. Perform hand hygiene 9. Take blood product <u>directly back to ward</u> and deliver it to the requesting nurse or nurse coordinator. Do not perform other errands when transporting blood / blood products 10. If the nurse advises you that the transfusion cannot commence immediately, the blood / blood product must be returned to TMU 11. Once delivery has been completed perform hand hygiene	 Do not perform other errands when transporting blood / blood products Return immediately to ward area with blood / blood products Do not put any blood / blood products in the ward fridge – always hand the blood / blood product to the nurse  Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

Specimen urgent delivery

As required

Equipment:

- Specimen inside a clear snap lock bio – hazard bag

Procedure / Steps	Safety Points
1. If an urgent specimen requires delivery to the lab, the nursing staff will give it to you and provide instructions 2. The specimen must be inside a clear snap lock bio – hazard bag 3. In the outside pocket of the snap lock clear bio hazard bag there should be a form. It is the Nurse or Doctor's responsibility to fill in the form correctly. 4. On that form it must have a: • Patient identification sticker • Ward (location) • Specimen details If the form is missing any of the above details, advise the nursing staff to complete it before leaving the ward 5. Perform hand hygiene 6. Take the specimen directly to central specimen collection area located on level 2 North Block 7. Hand the specimen to the staff member at the front desk and indicate it is urgent 8. “Blood Gas” specimens are always urgent and must be delivered within 10 minutes of collection. Always inform Laboratory staff that it is a “blood gas” specimen when delivered 9. If there is no one at the front desk ring the hand held bell which is located in front of the window 10. Immediately once the specimen has been delivered perform hand hygiene 11. Return to your ward area	 If you are not sure if the specimen is urgent and requires immediate delivery, check with the Nurse Do not perform other duties or errands on the way. Go directly to the lab. ASK the nurse if the specimen is a “Blood Gas” sample. These are URGENT  Report any equipment damage or malfunction to a Supervisor / shift coordinator / Nurse Unit Manager immediately. If unsafe to use the equipment do not use. Tag it out.

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